2026 Kentucky Commercial Provider Manual

Effective September 1, 2026



Anthem Blue Cross and Blue Shield is the trade name of Anthem Health Plans of Kentucky, Inc., an independent licensee of the Blue Cross Blue Shield Association. ®ANTHEM is a registered trademark of Anthem Insurance Companies, Inc. The Blue Cross and Blue Shield names and symbols are registered marks of the Blue Cross Blue Shield Association. Additional information about Anthem Blue Cross and Blue Shield in Kentucky is available at www.anthem.com. ©2026 Anthem Health Plans of Kentucky, Inc.

KY-BCBS-CM-PM-010987-26-S3103 | v1a 06012026

Table of Contents

Introduction **5**
Professional and Respectful Communication Standards **7**
Legal and Administrative Requirements **8**
Affiliates 8
Appointment Access and Geographic Availability 8
Artificial Intelligence (AI) – Restrictions on AI Use in Performing a Health Plan Function 12
Clinical Data Sharing 12
Coordination of Benefits 12
Data Notification Responsibilities 13
Data Verification Requirements 14
Digital Engagement 14
Dispute Resolution, Mediation, and Arbitration 14
Financial Institution/Merchant Fees 17
Insurance Requirements 17
Misrouted Protected Health Information (PHI) 17
Open Practice 17
Patient Safety Standards Attestation Required by Hospitals 17
Privacy Policy Statement 18
Risk Adjustments 18
Substance Use Disorder (SUD) Confidentiality Protections for Members 20
Use of a Non-Participating Provider 20
Digital Applications **21**
Anthem Provider Website 21
Email Communications 21
Online Provider Directory and Demographic Data Integrity 21
Availity Essentials 23
Provider and Facility Digital Guidelines 26
Provider Participation **33**
Credentialing 33
Non-Credentialed Providers and Facilities Participation Standards 33
Claims Submission **35**
Electronic Claims Submissions 35

Electronic Data Interchange (EDI) 35
Paper Claims Submission 37
Claim Submission Filing Tips 38
Claim Inquiry/Adjustment Filing Tips 42
National Drug Codes (NDC) 45
Medical Records Submission 49
Overpayments 52
Medicare Crossover 54
Claim Payment Disputes 58
Provider and Facility Claim Payment Dispute Process 58
Required Documentation for Claims Payment Disputes 60
Clinical Appeals 61
Member Quality of Care/Quality of Service Investigations 64
Corrective Action Plan (CAP) 64
Reporting 65
Reimbursement Policies and Requirements 66
Reimbursement Audit and Review Process 67
Medical Policies and Clinical Guidelines 91
Clinical Practice Guidelines 91
Preventive Health Guidelines 91
Medical Policies and Clinical Utilization Management (UM) Guidelines 91
Utilization Management 94
Utilization Management Program 94
Case Management 100
Carelon Medical Benefits Management 100
Quality Improvement Program 102
Quality Improvement (QI) Program Overview 102
Member Rights and Responsibilities 104
Continuity and Coordination of Care 104
Continuity of Care/Transition of Care Program 104
Quality-In-Sights®: Hospital Incentive Program (Q-HIP®) 105
Performance Data 106
Overview of HEDIS® 106
Overview of CAHPS 107

Culturally & Linguistically Appropriate Services **108**
Centers of Medical Excellence (CME) **110**
Audit and Review **115**
Anthem Claims Audit and Review Policy 115
Claims Audit Appeal Policy 119
Fraud, Waste, and Abuse Detection **120**
Reporting Fraud, Waste, and Abuse 120
Investigation Process 121
SIU Prepayment Review 122
Acting on Investigative Findings 123
Pharmacy & Prescriber Home Program **124**
Federal Employees Health Benefits Program and Postal Service Health Benefits Program **127**
Health Insurance Marketplace® (Exchanges) **131**
BlueCard Program Overview **131**
Medicare Advantage **131**
Workers' Compensation **132**
Provider Guidelines 132
Utilization Management Guidelines 132
The Workers' Compensation Act of Kentucky Standards 132
Other Payers Accessing the Anthem Workers' Compensation Network 133
Rules for Calculating Permanent Disability 133
Grievances 134
Important Links **135**

Introduction

Anthem and our health plan affiliates are committed to collaborating with care providers to make a meaningful impact on the health of their patients – our Members. That's why we continue to focus on streamlining our processes to help make it easier for care provider partners to find and use the information they need for their business interactions with us. With this collaboration, it's one more way that we're working to ensure Members have access to high-quality, affordable healthcare.

This Provider Manual (Manual) contains important information regarding key administrative requirements, policies, and procedures. While the Manual covers a wide array of policies, procedures, forms, and other useful information that can be found and maintained on the Anthem website, a few key topics are:

- Digital guidelines
- Claims submission
- Reimbursement and administrative policies and requirements
- Utilization management
- Quality improvement

As a participant in our diverse Anthem network, care providers (Providers and Facilities) agree to comply with Anthem policies and procedures, including those contained in this Manual. Payment may be denied, in full or in part, should Providers or Facilities fail to comply with the Manual. However, in the event of an inconsistency between the Agreement and this Manual, the Agreement will govern.

The policies and procedures outlined in this Manual apply unless otherwise specified in the Agreement.

Provider and Facility

This Manual is intended to support all entities and individuals who have executed a Provider or Facility agreement with Anthem.

The use of "Provider" within this Manual refers to entities and individuals contracted with Anthem who submit professional Claims.

The use of "Facility" within this manual refers to entities contracted with Anthem who submit institutional Claims.

General references to "Provider website" and similar terms apply to both Providers and Facilities.

Capitalization

Capitalized terminology used in this Manual is defined in the Anthem Facility Agreement or Anthem Provider Agreement, otherwise referred to in this Manual as "Agreement".

Updates to the Provider Manual

This Manual may be updated at any time and is subject to change. If there is a material change to this Manual, then Anthem will make reasonable efforts to notify Providers and Facilities in advance of such change through web-posted newsletters or email communications. In such cases, the most recently published information will supersede all previous information and be considered the current directive.

Important disclaimer

This Manual is not intended to be a complete catalog of all Anthem policies and procedures. Other policies and procedures not included in this Manual may be posted on the Anthem website or published in specially targeted communications, including but not limited to bulletins and newsletters. This Manual does not contain legal, tax, or medical advice. Providers and Facilities should consult their advisors for advice on these topics.

Professional and Respectful Communication Standards

Anthem maintains an ongoing commitment to fostering a respectful, collaborative, and professional environment, recognizing that effective communication is an integral component. Your Participating Provider Agreement (the “Agreement”) with Anthem outlines your obligations as a Participating Provider with Anthem regarding conduct and professionalism. Providers, including those who represent them, such as office staff, billing entities, etc., are expected to conduct themselves in a professional and respectful manner in all interactions with Anthem members, employees, and representatives. Professional and respectful communication is not just a courtesy, but a fundamental responsibility that supports collaboration, builds trust, and enhances the quality of service we offer to our members. By upholding these standards, Providers contribute to a positive and inclusive atmosphere where every individual feels valued and respected.

In addition to the standard policies and guidelines outlined in this manual and your participating Provider Agreement, Anthem maintains a zero-tolerance policy for abusive or disruptive behavior, whether physical or verbal, from Providers or those representing Providers during the course of business. Violent acts and/or continued abusive or disruptive behavior will result in the termination of your participation in Anthem’s provider network.

Examples of behavior that will not be tolerated include, but are not limited to, any act of violence, threats, harassment, intimidation, and other disruptive behavior. Such behavior can include actual physical injury, direct or indirect verbal or written statements, disruptive behavior, suggestions of self-harm, threats of retaliation to others, or gestures that communicate a threat of physical harm.

Legal and Administrative Requirements

Affiliates

Affiliates are an important concept in Anthem's Provider and Facility Agreements, as these entities access the rates, terms, or conditions of the agreements. To view a current listing of Anthem affiliates, visit the **Anthem Provider Home page**. Under the **Resources** menu, select **Forms, Guides and Administrative Policies,** then scroll down to the **Contracting & Updates** section and select **Provider Agreement Affiliates List**.

Appointment Access and Geographic Availability

Anthem uses these standards and regulations to assess access to services and experiences of Members for Kentucky Commercial and Healthcare Exchange Networks. Offices are expected to provide access in accordance with the Member's needs and expectations for their medical and behavioral health circumstances.

Provider Data Maintenance: Providers contracted with Anthem should utilize Availity's Provider Demographic Management (PDM) application on **Availity.com** to request changes to existing practice information.

Office Appointment Accessibility:
Assessment of appointment timeliness to meet Members' needs

Provider offices have the opportunity to be selected for a review of scheduling of appointments by a vendor, CSS (Center for the Study of Services), and the response to their inquiries is required as part of this contract with Anthem. Please assist the surveyor during the phone call and participate in this quality program for your patients.

Medical Appointment Access	Compliance
Emergency	Immediate access 24/7/365 or refer to ER or 911.
Urgent / Acute Care	Within 24 hours • Patients can be seen in the office by their doctor, covering doctor or another practitioner in the practice within the timeframe. • Patient is directed to Urgent Care Center, ER, or 911, as appropriate.
Non-Urgent (Symptomatic or chronic)	Within 72 hours Patients can be seen in the office by their doctor, another participating practitioner in the practice, or a covering practitioner within the timeframe.
Routine / Check-up	Within 10 business days Patients can be seen in the office by their doctor, another participating practitioner in the practice, or a covering practitioner within the timeframe.
Routine follow-up – evaluate since previous visit	Within 30 calendar days Patient can meet with their doctor, another participating practitioner in the practice, or a covering practitioner within the timeframe.

Medical Appointment Access	Compliance
Preventive Care	Within 30 calendar days Patients can be seen in the office by their doctor, another participating practitioner in the practice, or a covering practitioner within the timeframe.
Routine, Specialist	Within 30 calendar days Patient can meet with their doctor, another participating practitioner in the practice, or a covering practitioner within the timeframe.
Office Wait Time	Recommended not to exceed 30 minutes or less before being taken to the exam room.
After Hours Urgent Care (Required arrangements)	24/7/365 phone access All Members shall have phone access to urgent medical help or instructions after regular business hours through their primary care physicians via: • Live person connects the caller to their available doctor or on-call doctor. • Recording or live person directs the patient to Urgent Care, 911, or ER as appropriate. In addition to, but not in place of the above, the caller may be directed to contact a live health care practitioner (via cell, pager, beeper, transfer system) or get a call back for urgent instructions. **Having no provision is non-compliant and will require rectification.**

Behavioral Health Appointment Access	Compliance
Emergency	Immediate access 24/7/365 or refer to 911, ER, or crisis center.
Discharge Follow-up BH Appointment	Within 7 days New or existing patients can be seen in the office by a designated BH practitioner within the timeframe after discharge from an inpatient psychiatric hospitalization.
Emergent – Non-Life Threatening	Within 6 hours • Patients can be seen in the office by their BH practitioner, another participating practitioner in the practice, or a covering practitioner within the timeframe. • Patient is directed to 24-hour crisis services, 911, or ER as appropriate.
Urgent Care	Within 48 hours • Patients can be seen in the office by their BH practitioner, another participating practitioner in the practice, or a covering practitioner within the timeframe. • Patient is directed to 24-hour crisis services, 911, or ER as appropriate.

Behavioral Health Appointment Access	Compliance
Routine – Initial Visit Appointment	Within 10 business days New patients can be seen in the office by a designated BH practitioner or another equivalent participating practitioner within the timeframe. (After the intake assessment or referral.)
Routine – Regular Appointment	Within 10 business days Patient can meet with their BH practitioner, another participating practitioner in the practice, or a covering practitioner within the timeframe.
Routine – Follow-up Appointment	Within 30 calendar days New or existing patients can be seen in the office by their BH practitioner, another participating practitioner in the practice or a covering practitioner within the timeframe.
After Hours Urgent Care (Required arrangements)	24/7/365 phone access All Members shall have phone access to emergent/urgent instruction/consultation after regular business hours through their BH practitioner via: • Recording or live person directs patient to 24-hour crisis services, 911, or ER, as appropriate. • Caller is directed to contact a BH practitioner (via cell, pager, beeper, transfer system) or get a call back for instructions or consultation. ***Having no provision is non-compliant and will require rectification.***
Out of Office Coverage	Arrangement for coverage when the practitioner is unavailable (vacation, illness, holiday, etc.) via: • Cell phone, pager, etc. • Patient is directed to another BH practitioner in the practice, on call, or covering practitioner. • Prior arrangement with patients.

Provider Availability:

Assessment of numbers and types to meet Member needs

It is your responsibility to keep the status of your office updated for the online directory and patient selection. Is our phone number correct? Has a physician moved, retired, deceased, resigned their contract or is no longer in practice? Is your practice accepting new patients? Providers are required to notify Anthem to update our database of active providers used for the online directory.

Medical Network Adequacy	
Open Practice Keep Anthem updated on the status for the web directory.	At least 90% of Primary Care Physicians' practices will be open for new patient selection.
Geographic Availability of Medical Providers Mileage is based upon Member and provider zip code coordinates and locality definitions per Quest Analytics Suites™ software.	

Medical Network Adequacy	
Medical Geographics	**Measure**
Primary Care Physicians: Family Medicine, Internal Medicine, and Pediatrics	• 2 of each type within 5 miles (urban) • 2 of each type within 12 miles (suburban) • 2 of each type within 30 miles (rural)
OB/GYN	• 1 within 15 miles (urban) • 1 within 30 miles (suburban) • 1 within 40 miles (rural)
Specialists	• Minimum of 1 of each major specialty within 30 miles
Hospitals	• 1 within 30 miles
Skilled Nursing Facility	• 1 within 30 miles
Behavioral Health Network Adequacy	
Geographic Availability of Behavioral Health Providers Mileage is based upon Member and provider coordinates and locality definitions per Quest Analytics Suites™ software.	
BH Geographics	**Measure**
Psychiatrist (MD/DO) (Include Sub-Abuse)	• 2 within 10 miles (urban) • 2 within 25 miles (suburban) • 2 within 60 miles (rural)
Non-MD Professionals: **Psychologists and Master's Level Therapists** (Include Sub-Abuse)	• 2 of each type within 10 miles (urban) • 2 of each type within 25 miles (suburban) • 2 of each type within 60 miles (rural)
BH Treatment Facilities (Facilities offering IP BH services)	• 1 within 35 miles (urban /suburban combined) • 1 within 75 miles (rural)

Multicultural Health:
Practitioners meeting the needs and preferences of their patients

- Doctors are expected to identify their patients' needs by explaining things in a way they can understand, listening carefully, showing respect for what they have to say, and spending enough time with the patient.
- Anthem has provided offices with a tool (link below) that provides ideas, resources, and tools that can help doctors and their staff better understand and communicate with select patient groups with specific needs. This allows patients to fully understand their medical situation and get the maximum benefit from their time with their doctor. **mydiversepatients.com**

  Please see the *Culturally and Linguistically Appropriate Services* section of this manual for more information on resources for Providers to help support addressing racial and ethnic disparities in health and healthcare. Features include CME learning experiences, real-life stories about unique challenges faced by diverse patients, tips, and techniques to promote improved health outcomes.
- Practitioners can provide Anthem with their gender, language spoken, and race/ethnicity for the provider directory. Providers contracted with Anthem should utilize Availity's Provider Demographic Management (PDM) application hosted on **Availity.com** to request changes

This information will be utilized in online provider directories available to your customers to locate a doctor who meets their cultural, racial, ethnic, gender, and language needs and preferences.

Artificial Intelligence (AI) – Restrictions on AI Use in Performing a Health Plan Function

Providers who have been delegated to perform a health plan function (e.g., Utilization Management, credentialing, and/or claims payment) shall not use AI in the performance of any delegated health plan function without the advance written consent of the health plan. Providers shall request such approval in accordance with the notice provisions of the Provider Agreement, and the health plan shall provide its written response in accordance with the notice provisions of the Provider Agreement.

Clinical Data Sharing

When requested by Anthem, providers are required to submit clinical data (such as discharge summaries, consult notes, and medication lists) and admission, discharge, and transfer (ADT) data to Anthem for certain healthcare operations functions. We collect this data to improve the quality and efficiency of healthcare delivery to our members. Providers are required to submit:

- Facilities must provide Anthem with, at a minimum, Health Level Seven International (HL7) Admission, Discharge and Transfer (ADT) messaging data for all Members on a near real-time basis, including all standard HL7 message events pertaining to ADT as published by HL7. The Facility will transfer the required message data segments according to the standard HL7 format, or as requested by Anthem. For purposes of this section, “near real-time basis” means no later than twenty-four (24) hours from admission, discharge, or transfer of any Members.
- Clinical data for a Member on a daily, weekly, or monthly basis, in a mutually agreeable format and method based on the provider's electronic medical record (EMR) or other electronic data sharing capabilities, e.g., industry-standard CCDA clinical data format.

Anthem’s permitted uses of the data with respect to clinical data requests include utilization management, case management, identification of gaps in care, conducting clinical quality improvement, risk adjustment, documentation in support of HEDIS® and other regulatory and accrediting reporting requirements, and for any other purpose permitted under HIPAA.

Anthem has determined that the data requested is the minimum necessary for Anthem to accomplish its intended purposes. The data will be provided in accordance with data layout and format requirements defined by Anthem.

For details on how to submit clinical data, review the administrative policy by visiting the **Anthem Provider Home page**. Under the **Resources** menu, select **Forms, Guides and Administrative Policies,** then scroll down to the **Administrative Policies** section and select **Clinical Data Sharing**.

In the event of a conflict between this Policy and the Provider Agreement, the Provider Agreement shall prevail.

Coordination of Benefits

If a Member or eligible dependent is covered by more than one Health Benefit Plan, the carriers involved work together to prevent duplicate payments for any services. This cooperative effort is called Coordination of Benefits (COB), a provision in most Health Benefit Plans.

If a Plan is other than the primary payer, any further compensation to the Provider or Facility from the Plan or the Member will be determined in accordance with the Agreement, the applicable Health Benefit Plan, and any applicable Plan written policies and procedures for coordinating benefits.

The secondary plan payment will be the lesser of the patient's remaining liability after primary payment or the secondary plan's allowable.

Notwithstanding the foregoing, in no event shall Plan or the Member be required to pay more than they would have paid had the Plan been the primary payer. Providers and Facilities will not collect any amount from the Member if such amount, when added to the amounts collected from the primary and secondary payers, would cause total reimbursement to the Provider or Facility for the Covered Service to exceed the amount allowed for the Covered Service under the Agreement. Further, this provision shall not be construed to require Providers or Facilities to waive Cost Share in contravention of any Medicare rule or regulation, nor shall this provision be construed to supersede any other Medicare rule or regulation. If, under this Section, Providers and Facilities are permitted to seek payment from other sources by reason of the existence of other group coverage in addition to the Plan's Health Benefit Plan. Providers and Facilities may seek payment from other sources on a basis other than the Plan rate.

Make the Most of Electronic Coordination of Benefits (COB) Submissions

Availity is Anthem's designated electronic data interchange (EDI) gateway. The **Anthem Companion Guide** contains the required segments to bill Coordination of Benefit Claims electronically. To learn more, contact the EDI vendor.

When filing Coordination of Benefits Claims on paper submission

Include the Explanation of Benefit. (EOB) from the primary insurance carrier with coordination of benefits (COB), Claims submitted for secondary payment.

Data Notification Responsibilities

Providers and Facilities are responsible for notifying Anthem when changes occur within the Provider practice or Facility. Providers and Facilities should reference their Agreement for specific timeframes associated with change notifications.

Examples of these changes include, but are not limited to:

- Adding new practitioners to or removing practitioners from a group
- Change in ownership
- Change in tax identification number
- Making changes to demographic information or adding new locations
- Selling or transferring control to any third party
- Acquiring, or merging with, another medical practice or entity
- Change in accreditation
- Change in affiliation
- Change in licensure or eligibility status
- Change in operations, business, or corporation

Data Verification Requirements

The Consolidated Appropriations Act (CAA) of 2021 is a federal act containing legal and regulatory requirements for health plans and Providers and Facilities to improve the accuracy of Provider directory information.

Providers and Facilities are required to review and verify the accuracy of this information in the online Provider directory every ninety (90) days:

- Provider/facility name
- Address
- Specialty
- Phone number
- Digital contact information

Providers who fail to verify their information every ninety (90) days may be removed from the online Provider directory.

Providers will be reinstated to the online Provider directory once verification is completed.

To review, verify, and update your online directory information, Anthem uses the Provider data management (PDM) capability available on **Availity.com** to update Provider or Facility data. Using the Availity PDM capability meets the verification requirement to validate the Provider demographic data set by the CAA.

For details on Availity PDM, refer to the *Online Provider Directory and Demographic Data Integrity* subsection of this manual.

Digital Engagement

Anthem expects Providers and Facilities will utilize digital tools unless otherwise prohibited by law or other legal requirements for transactions such as filing Claims, prior authorizations, verifying eligibility and benefits, paperless payments, etc. Providers and Facilities should refer to the guidance included throughout this Manual where digital tools are available. For a complete list of digital tools, refer to the *Digital Applications* section and *Provider and Facility Digital Guidelines* subsection in this Manual.

Dispute Resolution, Mediation, and Arbitration

The substantive rights and obligations of Anthem Providers and Facilities with respect to resolving disputes are set forth in the Anthem Provider or Facility Agreement (the "Agreement). All administrative remedies set forth in the Agreement shall be exhausted prior to filing an arbitration demand. The following provisions set forth the procedures and processes that must be followed during the exercise of the Dispute Resolution and Arbitration Provisions in the Agreement. To the extent possible, the language of the Agreement and the Provider Manual should be read together and harmonized if there are details in one not addressed in the other.

A. Fees and Costs

   All fees and costs associated with neutrals, logistics, and administration of confidential non-binding mediation and confidential binding arbitration (i.e., mediator travel and fee, arbitrator(s) travel and fee(s), arbitration association administrative costs, etc.) shall be shared equally between the parties. Each party shall be responsible for the payment of its own fees and costs

that the party incurs (i.e., attorney fees, experts, depositions, document production, e-discovery, etc.). Notwithstanding this provision, the arbitrator or panel of arbitrators may issue an order in accordance with Federal Rule of Civil Procedure Rule 11 or the respective state rule counterpart, awarding a party its fees if that party requested fees under Rule 11, or the respective state court counterpart rules in its initial pleadings. Notwithstanding this provision, the arbitrator or panel of arbitrators may issue an order in conjunction with a party's offer of judgment in accordance with Federal Rule of Civil Procedure Rule 68.

B. Location of the Arbitration

The arbitration hearing will be held in the city and state in which the Anthem office, identified in the address block on the signature page to the Agreement, is located, except that if there is no address block on the signature page, then the arbitration hearing will be held in the city and state in which the Anthem Plan identified in the Agreement has its principal place of business. Notwithstanding the foregoing, both parties can agree in writing to hold the arbitration hearing in some other location.

C. Pre-Arbitration Mediation and Selection and Replacement of Arbitrator(s)

Refer to the Agreement for invoking dispute resolution requirements, monetary thresholds of disputes (exclusive of interest, costs, or attorney fees) that require a meeting to discuss and in an effort to resolve, or that require pre-arbitration mediation and selection of the mediator. In the event of a dispute where the dispute resolution provision is invoked, the first step is for the complaining entity to provide written notice containing a detailed description of the dispute, all relevant underlying facts, a detailed description of the amount(s) in dispute and how they have been calculated and any other information in this Provider Manual describing the policy, procedure or process that is being disputed.

Refer to the Agreement for governing arbitration rules, monetary thresholds (exclusive of interest, costs, or attorney fees) as applicable, selection of a single arbitrator or panel of three arbitrators, and replacement of an arbitrator.

D. Consolidation

The arbitrator or panel of arbitrators does not have the authority to consolidate separately filed arbitrations, for discovery or otherwise, without the written consent and agreement of the parties. The arbitrator or panel of arbitrators does not have the authority to permit Providers or Facilities under separate Agreements with Anthem to bring one arbitration action without written consent and agreement by the parties. Rather, each Provider or Facility with separate Agreements should file for separate arbitration in its own name, unless there is written consent and agreement by the parties to consolidate the action, in some fashion.

E. Discovery

The parties recognize that litigation in state and federal courts can be costly and burdensome. One of the parties' goals in providing for disputes to be mediated and arbitrated instead of litigated is to reduce the costs and burdens associated with resolving disputes. Accordingly, the parties expressly agree that discovery shall be conducted with strict adherence to the rules and procedures established by the mediation or arbitration administrator identified in the Agreement, except that the parties will be entitled to serve requests for production of documents and data, which shall be governed by Federal Rules of Civil Procedure 26 and 34. The parties shall confer and draft an Order Regarding Procedures for Production Format and Electronic Discovery, which shall be presented to the arbitrator or panel of arbitrators for review, approval, and entry.

F. Decision of Arbitrator(s)

The decision of the arbitrator, if a single arbitrator is used, or the majority decision of the arbitrators, if a panel is used, shall be binding upon the parties. The arbitrator(s) may construe or interpret, but shall not vary or ignore, the provisions of the Agreement and shall be bound by and follow controlling law. The arbitrator(s) shall not toll or modify any applicable statute of limitations set forth in the Agreement or controlling law if the Agreement is silent. If there is a dispute regarding the applicability or enforcement of the class waiver provisions found in the Agreement, that dispute shall only be decided by a court of competent jurisdiction and shall not be decided by the arbitrator(s). Either party may request either a reasoned award or decision, or findings of fact and conclusions of law, and if either party makes such a request, the arbitrator(s) shall issue such an award or decision setting forth the factual and legal basis for the decision.

The arbitrator(s) may consider and decide the merits of the dispute or any issue in the dispute on a motion for summary disposition. In ruling on a motion for summary disposition, the arbitrator(s) shall apply the standards applicable to motions for summary judgment under Federal Rule of Civil Procedure 56.

Judgment upon the award rendered by the arbitrator(s) may be confirmed and enforced in any court of competent jurisdiction. Without limiting the foregoing, the parties hereby consent to the jurisdiction of the courts in the State(s) in which Anthem is located, as identified in the address block on the signature page to the Agreement, and of the United States District Courts sitting in the State(s) in which Anthem is located, as identified in the address block on the signature page to the Agreement, for confirmation, specific enforcement, or other relief in furtherance of the arbitration proceedings or to enforce judgment of the award in such arbitration proceeding.

If a party files an interim award, award, or judgment with a state or federal district court, then all documents must be filed under seal to ensure confidentiality as outlined below, and only the portions outlining the specific relief or specific enforcement or performance shall be filed, and the remainder of the opinion or decision shall be redacted.

Refer to the Agreement for monetary thresholds (inclusive of interest, costs, and attorney fees) as applicable for the right to appeal the decision of the arbitrator or panel of arbitrators. A decision that has been appealed shall not be enforceable while the appeal is pending.

G. Interest

Providers or Facilities agree that the state’s statutory pre-judgment interest statute is inapplicable to Dispute Resolution and Arbitration. Should the arbitrator(s) determine that pre-judgment interest is appropriate and issue an award including it, pre-judgment interest shall be simple, not compounded, at an annual percentage rate no more than five percent (5%) or the interest applied for “clean claims”, whichever is less. If an award is issued and it includes post-judgment interest, it will not begin accruing until thirty (30) business days after the date of the award to allow time for payment. If an appeal is taken by either side, the obligation to pay any damages and/or interest awarded shall be tolled until a decision is reached as a result of the appeal.

H. Confidentiality

Subject to any disclosures that may be required or requested under state or federal law, all statements made, materials generated or exchanged, and conduct occurring during the arbitration process including, but not limited to, materials produced during discovery, arbitration statements filed with the arbitrator(s), and the decision of the arbitrator(s), are confidential and shall not be disclosed in any manner to any person who is not a director, officer, or employee of a party or an arbitrator or used for any purpose outside the arbitration. If either party files an action in federal or

state court arising from or relating to a mediation or arbitration, all documents must be filed under seal to ensure that confidentiality is maintained. Nothing in this provision, however, shall preclude Anthem or its parent company from disclosing any such details regarding the arbitration to its accountants, auditors, brokers, insurers, reinsurers, retrocessionaires, or affiliates and Other Payers whose Claims have been at issue in the arbitration, including Administrative Services Only (ASO) groups and other Blue Plans.

Financial Institution/Merchant Fees

Providers and Facilities are responsible for any fees or expenses charged to them by their own financial institution or payment service provider.

Insurance Requirements

Providers and Facilities shall self-insure or maintain insurance in types and amounts reasonably determined by Providers and Facilities, or as required under applicable licensing or regulatory requirements.

Misrouted Protected Health Information (PHI)

Providers and Facilities are required to review all Member information received from Anthem to ensure no misrouted PHI is included. Misrouted PHI includes information about Members that a Provider or Facility is not currently treating. PHI can be misrouted to Providers and Facilities by mail, fax, email, or electronic remittance. Providers and Facilities are required to immediately destroy any misrouted PHI or safeguard the PHI for as long as it is retained. In no event are Providers or Facilities permitted to misuse or re-disclose misrouted PHI. If Providers or Facilities cannot destroy or safeguard misrouted PHI, Providers and Facilities must contact Provider Services to report receipt of misrouted PHI.

Open Practice

Providers shall give Anthem sixty (60) days' prior written notice when the Provider no longer accepts new patients. Providers contracted with Anthem should utilize Availity's Provider Demographic Management (PDM) application hosted on **Availity.com** to request changes to existing practice information.

Patient Safety Standards Attestation Required by Hospitals

As a qualified health plan (QHP) issuer, Anthem must ensure compliance with federal regulation 42 CFR 156.1110 when contracting with hospitals with more than fifty (50) beds. This regulation mandates QHPs to verify that hospitals meet these specific patient safety standards:

- The hospital must use a patient safety evaluation system as defined in 42 CFR 3.20 by:
    - Collecting and reporting data to patient safety organizations (PSOs)
    - Implementing a mechanism for comprehensive person-centered hospital discharge
    - Improving care coordination and healthcare quality for each patient
- If the hospital is not working with a PSO, the hospital must implement an evidence-based initiative to improve healthcare quality, reduce preventable harm, prevent readmission, and improve care coordination.

To help us ensure compliance, we require participating hospitals to attest to meeting these patient safety standards by completing the Patient Safety Standards form annually. To access the form, go to the **Anthem Provider Home page**, and under the **Resources** menu, select **Forms, Guides and Administrative Policies.** Under the **Affordable Care Act** section, select **Patient Safety Standards Form**.

Privacy Policy Statement

Information regarding Anthem's Corporate Privacy Policy Statement, which sets forth guidelines regarding a Member's right to access and amend information in Anthem's possession, is available online. Go to the **Anthem Provider Home page**, scroll to the bottom footer, and select **Privacy**.

Risk Adjustments

Compliance with Federal Laws, Audits, and Record Retention Requirements

Medical records and other health and enrollment information of Members must be handled under established procedures that:

- Safeguard the privacy of any information that identifies a particular Member;
- Maintain such records and information in a manner that is accurate and timely; and
- Identify when and to whom Member information may be disclosed.

In addition to the obligation to safeguard the privacy of any information that identifies a Member, Anthem, Providers, and Facilities are obligated to abide by all Federal and state laws regarding confidentiality and disclosure for medical health records (including mental health records) and enrollee information.

Encounter Data for Risk Adjustment Purposes

Commercial Risk Adjustment and Data Submission: Risk adjustment is the process used by Health and Human Services (HHS) to adjust the payment made to health plans under the Affordable Care Act (ACA) based on the health status of the Members who are insured under small group or individual health benefit plans compliant with the ACA (aka ACA Compliant Plans). Risk adjustment was implemented to pay health plans more accurately for the predicted health cost expenditures of Members by adjusting payments based on demographics (age and gender) as well as health status. Anthem, as a qualifying health plan, is required to submit diagnosis data collected from encounters and Claim data to HHS for purposes of risk adjustment. Because HHS requires that health plans submit "all ICD-10 codes for each beneficiary", Anthem also collects diagnosis data from the Members' medical records created and maintained by the Provider or Facility.

Under the HHS risk adjustment model, the health plan is permitted to submit diagnosis data from inpatient hospital, outpatient hospital, and physician/qualified non-physician, e.g., nurse practitioner encounters only.

Maintaining documentation of Members' visits and of Members' diagnoses and chronic conditions helps Anthem fulfill its requirements under the Affordable Care Act. Those requirements relate to the risk adjustment, reinsurance, and risk corridor, or "3Rs" provision in the ACA. To ensure that Anthem is reporting current and accurate Member diagnoses, Providers and Facilities may be asked to complete an Encounter Facilitation Form (also known as a SOAP note) for Members insured under small group or individual health benefit plans suspected of having unreported or out-of-date condition information in their records. Anthem's goal is to have this information confirmed and/or updated no less than annually. As a condition of the Facility or Provider's Agreement with Anthem, the Provider or

Facility shall comply with Anthem's requests to submit complete and accurate medical records, Encounter Facilitation Forms, or other similar encounter or risk adjustment data in a timely manner to Anthem, Plan, or designee upon request. Providers and Facilities also agree to cooperate with Anthem's, or its designee's, requests to reach out to patients to request appointments or encounters so additional information can be collected to resolve any gaps in care (example, blood tests in certain instances) and to provide the updated and complete Member health information to Anthem to help it fulfill its requirements under the Affordable Care Act.

In addition to the above ACA-related commercial risk adjustment requirements, Providers and Facilities also may be required to produce certain documentation for Members enrolled in Medicare Advantage or Medicaid.

RADV Audits

As part of the risk adjustment process, HHS will perform a risk adjustment data validation (RADV) audit in order to validate the Members' diagnosis data that was previously submitted by health plans. These audits are typically performed once a year. If the health plan is selected by HHS to participate in a RADV audit, the health plan and the Providers or Facilities that treated the Members included in the audit will be required to submit medical records to validate the diagnosis data previously submitted.

ICD-10-CM Codes

HHS requires that physicians use the ICD-10-CM Codes (ICD-10 Codes) or successor codes and coding practice services under ACA-compliant plans. In all cases, the medical record documentation must support the ICD-9 Codes or successor codes selected and substantiate that proper coding guidelines were followed by the Provider or Facility. For example, in accordance with the guidelines, it is important for Providers and Facilities to code all conditions that co-exist at the time of an encounter and that require or affect patient care, treatment, or management. In addition, coding guidelines require that the Provider or Facility code to the highest level of specificity, which includes fully documenting the patient's diagnosis.

Medical Record Documentation Requirements

Medical records significantly impact risk adjustment because:

- They are a valuable source of diagnosis data;
- They dictate what ICD-10 code or successor code is assigned; and
- They are used to validate diagnosis data that was previously provided to HHS by the health plans.

Because of this, the Provider and Facility play an extremely important role in ensuring that the best documentation practices are established.

HHS record documentation requirements include:

- Patient's name and date of birth should appear on all pages of the record.
- Patient's condition(s) should be clearly documented in the record.
- The documentation must show that the condition was monitored, evaluated, assessed/addressed or treated (MEAT), or there is evidence of treatment, assessment, monitoring or medicate, plan, evaluate, referral (TAMPER).
- The documentation describing the condition and MEAT OR TAMPER must be legible.
- The documentation must be clear, concise, complete, and specific.

- When using abbreviations, use standard and appropriate abbreviations. Because some abbreviations have different meanings, use the abbreviation that is appropriate for the context in which it is being used.
- Physicians /Qualified Non-Physicians' signature, credentials, and date must appear on record and must be legible.

Substance Use Disorder (SUD) Confidentiality Protections for Members

Providers and Facilities subject to federal regulation 42 CFR Part 2 as "Part 2 Programs" must obtain compliant consent from their patients allowing for the use of records for treatment, payment, and healthcare operations (TPO). Without this consent, the Part 2 Program should not submit Claims for payment.

Part 2 programs are federally assisted programs that offer substance use disorder (SUD) diagnosis, treatment, or referral for treatment, and are subject to specific confidentiality regulations outlined in 42 CFR Part 2.

Use of a Non-Participating Provider

Anthem's mission is to provide affordable, quality healthcare benefits to its Members. Members access their highest level of healthcare benefits when receiving services from Network/Participating Providers and Facilities. Providers and Facilities put Members at risk of higher out-of-pocket expenses when they refer to, utilize, or transfer Members to non-participating Providers or Facilities in non-emergent situations, without Anthem's prior approval.

Providers and Facilities are reminded that, pursuant to their Agreement with Anthem, they are required to refer to, utilize, or transfer Members to other Network/Participating Providers and Facilities.

Providers and Facilities that establish a pattern of referring to, utilizing, or transferring Members to Non-participating Providers may be subject to disciplinary action, up to and including financial penalties, or termination from the Anthem Network(s). Anthem understands there may be instances in which Providers and Facilities must utilize Non-participating Providers. In such cases, Providers and Facilities must present evidence of extenuating circumstances if they choose to dispute any resulting disciplinary action.

Providers and Facilities can contact their Anthem contract manager for additional information.

Digital Applications

Anthem Provider Website

Anthem.com is a public website. **anthem.com**

Anthem designed the provider public website to make navigation easy and useful for Providers and Facilities. The website holds timely and important information to assist Providers when working with Anthem. To access the Provider website:

- Go to **anthem.com** and select **For Providers** at the top of the webpage
- From the **Provider Welcome page**, select **Kentucky** to go to the **Anthem Provider Home page**

Email Communications

Anthem produces a monthly newsletter, *Provider News*, to notify Providers, Facilities, and their staff about updates, events, and changes in processes and policies. Providers and Facilities can sign up for email communications to be notified when a newsletter is published and receive other notifications from Anthem. To sign up for our email communications, go to the ***Provider News* website** and select the **Subscribe to Email** button.

Note: *Provider News* emails will come from **ProviderCommunications@email.anthem.com**. Add this to your browser's sender/recipient list to ensure our emails are received.

Online Provider Directory and Demographic Data Integrity

Providers and Facilities can confirm their Network participation status by using the **Find Care** tool. A search can be done on a specific provider name, or by viewing a list of local in-network Providers and Facilities using search features such as provider specialty, zip code, and plan type.

Online Provider Directory

Accessing the Online Provider Directory:

- Go to **anthem.com**
- Select the **Find Care** link at the top right of the page.

Before directing a Member to another Provider or Facility, verify that the Provider or Facility is participating in the Member's specific network. ***Note:*** The Member's Network Name should be on the lower right corner of the front of the Member's ID card.

To help ensure Members are directed to Providers and Facilities within their specific Network, utilize the Online Provider Directory in one of the following ways:

- **Use Member ID for Basic Search**: Search by entering the Member's ID number (including the three-character prefix), or simply enter the three-character prefix by itself.
- Select **Basic Search as Guest**.

Providers and Facilities with questions about their participation status listed in the online directory should contact the number on the back of the Member's ID Card.

Updating Demographic Data with Anthem

It is critical that Members receive accurate and current data related to Provider availability. Providers and Facilities must notify Anthem of any demographic changes. All requests must be received forty-five (45) days **prior** to the change/update. Any requests received within less than forty-five (45) days' notice may be assigned a future effective date. Contractual terms may supersede the effective date request.

IMPORTANT: If updates are not submitted forty-five (45) days prior to the change, Claims submitted for Members may be the responsibility of the Provider or Facility.

Types of demographic data updates can include, but are not limited to:

- Accepting new patients
- Address – additions, terminations, updates (including physical and billing locations)
- Areas of expertise (behavioral health only)
- Email address
- Handicapped accessibility
- Hospital affiliation and admitting privileges
- Languages spoken
- License number
- Name change (Provider/organization or practice)
- National provider identifier (NPI)
- Network participation
- Office hours/days of operation
- Patient age/gender preference
- Phone/fax number
- Provider leaving group, retiring, or joining another practice*
- Specialty
- Tax identification number (TIN) (must be accompanied by a W-9 to be valid)
- Termination of Provider participation agreement **
- Web address

* To request participation for a new Provider or practitioner, even if joining an existing practice, providers or practitioners must first begin the Application process. Go to the **Anthem Provider Home page** and under the **Provider Enrollment and Maintenance** menu, select **Join our Network**.

** For notices of termination from an Anthem network, Providers and Facilities should refer to the termination clause in the Agreement for specific notification requirements. Allow the number of days' notice of termination from Anthem's network as required by the Agreement.

Submitting Provider demographic data requests and roster submissions through Roster Automation

Use the Provider Data Management (PDM) application on Availity Essentials to verify and initiate Provider demographic change requests for all Providers and Facilities. **The PDM application is the preferred intake tool for Providers and Facilities to submit demographic change requests,**

including roster uploads. If preferred, Providers may continue to utilize the Provider Enrollment application in Availity Essentials to submit requests to add new practitioners under existing groups for available provider types.

Within the PDM application, Providers and Facilities have the choice and flexibility to request data updates via the standard PDM experience or by submitting a spreadsheet via a roster upload.

Roster Automation is our technology solution designed to streamline and automate provider data additions, changes, and terminations that are submitted using a standardized Microsoft Excel document. Any Provider, whether an individual provider/practitioner, group, or facility, can use Roster Automation today. If any roster data updates require credentialing, your submission will be routed appropriately for further action.

The resources for this process are listed below and available on our website. Go to the **Anthem Provider Home page**, and under the **Resources** menu select **Forms, Guides and Administrative Policies**. The **Roster Automation Rules of Engagement** and **Roster Automation Standard Template** appear under the **Digital Tools** category.

- **Roster Automation Rules of Engagement:** This is a reference document available to ensure error-free submissions, driving accurate and more timely updates through automation.
- **Roster Automation Standard Template:** Use this template to submit your information. More detailed instructions on formatting and submission requirements can also be found on the first tab of the Roster Automation Standard Template (*User Reference Guide*).
- Upload your completed roster via the Availity PDM application.

Accessing PDM Application

Log onto **Availity.com** and select My Providers > Provider Data Management to begin the attestation process. If submitting a roster, locate the TIN/business name to be verified and update the information accordingly. Before selecting the TIN/business name, select the three-bar menu option on the right side of the window, and select **Upload Rosters,** and follow the prompts.

Availity Administrators will automatically be granted access to PDM. The Administrator may grant additional staff access to Provider Data Management. To find your administrator, go to **My Account Dashboard > My Account > Organization(s) > Administrator Information**.

Exclusions:

- Behavioral Health Providers contracted with Carelon Behavioral Health, who will continue to follow the process for demographic requests and/or roster submissions, as outlined by Carelon Behavioral Health.
- Any specific state mandates or requirements for provider demographic updates.

Availity Essentials

We offer digital solutions to enhance collaboration and streamline interactions with Anthem, helping to eliminate complexities and improve transparency, traceability, and the overall experience for Providers and Facilities.

Availity Essentials is available to all Providers and Facilities:

- **Multi-payer access:** Users can access data from Anthem Medicare, Medicaid, and other Commercial insurers. See **Availity.com** for a full list of payers.

- **No charge:** Anthem transactions are available at no charge to Providers and Facilities.
- **Standard responses:** Responses from multiple payers returned in the same format and screen layout, providing users with consistency across payers.
- **Compliance:** Availity Essentials is compliant with all Health Insurance Portability and Accountability Act (HIPAA) regulations.
- **Accessibility:** Availity Essentials functions are available 24/7 from any computer with internet access.

Availity Essentials simplifies the way we work together through these applications and processes:

- **Eligibility and Benefits application:** Access current Member coverage, benefits information, and Member's digital ID cards. Use the Patient Registration tab to access Eligibility and Benefits.
- **Submit Claims:** Use either the Claims & Payments application or EDI gateway.
- **Claims Status application:** Monitor claim status, submit documents, and file claims disputes online. Access Claims Status from the Claims & Payments tab.
- **Authorizations:** Submit for medical or behavioral health inpatient or outpatient services, file appeals, and track authorization cases. Access the Authorization from the Patient Registration tab.
- **Provider Data Management:** Update demographic information digitally. Access the Provider Data Management application through the My Providers tab.
- **Roster Automation:** Use standardized forms, identify necessary changes, and update the demographic system seamlessly.
- **Remittance Advice:** View, print, or save a copy of remittance advice through the Claims Status application or through Remittance Inquiry in Payer Spaces
- **Clinical Documentation Lookup Application:** Search our Medical Policies by CPT code to view a list of documents needed to process your Claim.

Additional digital methods of engagement include:

- **Carelon Medical Benefits Management:** Access link to precertification requests and inquiries for specific services and access the OptiNet® Survey when applicable at **providerportal.com**.
- **Medical Attachments:** Submit supporting documentation, including medical records, for initial, pended, or denied claims through Availity.com. From the Claims & Payments tab, select Claim Status, submit a claim status inquiry, and use the Submit Attachments link from a successful response. Use the Medical Attachments functions to submit an itemized bill electronically through the EDI 275 transaction. For Providers registered in Medical Attachments through Availity.com, receive digital notifications about additional documents needed for claims processing through Digital RFAI.
- **Member Certificate Booklet:** View a local plan Member's certificate of coverage online, where available. From Availity.com select the Patient Registration tab to access Eligibility and Benefits. The Certificate of Coverage link will be at the top of the page of a successful eligibility and benefits transaction if available in your Anthem market.

- **Secure Messaging:** Claim status is available through the Claims & Payments application. If you have claims questions that require additional clarification, Secure Message may be available. From a successful claim status transaction, select the Secure Messaging link to submit a question on the claim. From Availity.com, go to Payer Spaces, select the payer, then use the Resources Tab to access Secure Messaging responses.

Payer Spaces

To access Anthem-specific applications, use Payer Spaces from Availity.com:

- **Alerts Hub:** Primary Care Providers (PCPs) can receive timely information about their patients, including admission, discharge, and transfer (ADT) and against medical advice discharge notifications.
- **Authorization Look Up Tool:** Determine if an authorization is needed for a commercial Member for a specific outpatient medical or behavioral health service.
- **Chat with Payer:** When the information is not available through self-service on Availity.com, Providers and Facilities can chat with an online representative about prior authorizations, appeals, Claims, eligibility, benefits, and more.
- **Clear Claim Connection:** Research procedure code edits and receive edit rationale.
- **Custom Learning Center:** Access payer-specific educational materials.
- **Fee Schedule:** Retrieves professional office-based contracted price information for patient services.
- **Preference Center:** A resource for Providers and Facilities to share correspondence preferences related to specific transactions, for example, prior authorization decision letters and PCPs' patient event notifications.
- **Provider Digital RFAI Progress Dashboard:** For Providers and Facilities enrolled in Medical Attachments and using the Attachments Dashboard to receive digital notifications when additional documentation is needed to process Claims, use this Dashboard to show your organization's attachment performance.
- **Provider Online Reporting:** Access proprietary Provider-specific reports such as Member rosters and Provider Contract and Fee Schedule notifications.
- **Remittance Inquiry:** View an imaged copy of the paper Anthem remits up to twenty-four (24) months in the past.
- **Total Member View (TMV)**: A robust picture of a Member's health and treatment history, including gaps in care and care reminders.

Getting Started and Availity Essentials Training.

To register for access to Availity Essentials, go to **Availity.com/providers/registration-details/**. For additional assistance with registration, contact Availity Client Services at 1-800-AVAILITY (282-4548).

After logging into Availity Essentials, Providers and Facilities have access to many resources to help jumpstart learning, including free and on-demand training, frequently asked questions, comprehensive help topics, and other resources. Availity Essentials also offers onboarding modules for new Administrators and Users.

From **Availity.com**, select **Help & Training** (from the top navigation menu on the Availity Essentials home page), then select **Get Trained**, and type "*onboarding*" in the search catalog field.

Availity Essentials Training for Anthem-specific tools

Learn about Anthem-specific applications through the Custom Learning Center. From **Payer Spaces,** select **Applications** to access the Custom Learning Center for presentations and reference guides. Find additional learning opportunities through the Provider Learning Hub. To visit the Anthem version of the Provider Learning Hub, go to your public provider site and select the Provider Learning Hub link located with Availity information.

Organization Maintenance

To update Administrator or Organization information:

- To replace the Administrator currently on record with Availity Essentials, call Availity Client Services at **1-800-AVAILITY (282-4548)**.
- An Administrator can use the Maintain Organization feature on Availity.com to maintain the organization's demographic information, including address, phone number, tax ID, and NPI updates. Any changes made to this information automatically apply to all users associated with the organization and affect only the registration information on Availity Essentials.

Support

Submit a support ticket for additional help or technical difficulties through Availity Essentials:

1. Log onto Availity at **Availity.com**
2. Select **Help & Training,** then access **Availity Support**
3. Select the organizations, then select **Continue**
4. Select **Contact Support** from the top menu bar, then **Create Case.**

Provider and Facility Digital Guidelines

Anthem understands that working together digitally streamlines processes and optimizes efficiency. We developed the Provider and Facility Digital Guidelines to outline our expectations and to fully inform Providers and Facilities about our digital platforms.

Anthem expects Providers and Facilities will utilize digital tools unless otherwise mandated by law or other legal requirements.

The Digital Guidelines establish the standards for using secure digital Provider platforms (websites) and applications when transacting business with Anthem. These platforms and applications are accessible to both participating and nonparticipating Providers and Facilities and encompass Availity.com, electronic data interchange (EDI), electronic medical records (EMR) connections, and business-to-business (B2B) desktop integration.

Digital and/or electronic transaction applications are accessed through these platforms:

- Availity EDI Clearinghouse
- B2B application programming interfaces (APIs)
- EMR connections

Digital functionality available through Availity Essentials includes:

- Acceptance of digital ID cards
- Eligibility and benefit inquiry and response

- Prior authorization submissions, including updates, clinical attachments, authorization status, and clinical appeals
- Claim submission, including attachments, and claims status
- Remittances and payments
- Provider enrollment and network management
- Demographic updates

Additional digital applications available to Providers and Facilities include:

- Pharmacy prior authorization drug requests
- Services through Carelon Medical Benefits Management
- Services through Carelon Behavioral Health

Anthem expects Providers and Facilities transacting any functions and processes above will use available digital and/or electronic self-service applications in lieu of manual channels (paper, mail, fax, call, chat, etc.). All channels are consistent with industry standards. All EDI transactions use version 5010.

Note: *As a mandatory requirement, all trading partners must currently transmit directly to the Availity EDI gateway and have an active Availity Trading Partner Agreement in place. This includes Providers and Facilities using their practice management software and clearinghouse billing vendors.*

Providers who do not transition to digital applications may experience delays when using non-digital methods, such as mail, phone, and fax, for transactions that can be conducted using digital applications.

Section 1: Accepting digital ID cards

As our members transition to digital Member ID cards, Providers and Facilities may need to implement changes in their processes to accept this new format. Anthem expects that Providers and Facilities will accept the digital version of the Member identification card in lieu of a physical card when presented. If Providers and Facilities require a copy of a physical ID card, Members can email a copy of their digital card from their smartphone application, or Providers and Facilities may access it directly from Availity Essentials through the Eligibility and Benefits Inquiry application.

Section 2: Eligibility and benefits inquiry and response

Providers and Facilities should leverage these Availity clearinghouse-hosted channels for electronic eligibility and benefit inquiry and response:

- EDI transaction: X12 270/271 – eligibility inquiry and response:
    - Anthem supports the industry standard X12 270/271 transaction set for eligibility and benefit inquiry and response as mandated by HIPAA.
- Availity Essentials:
    - The Eligibility and Benefits Inquiry verification application allows Providers and Facilities to key an inquiry directly into an online eligibility and benefit look-up form with real-time responses.
- Provider desktop integration via B2B APIs:

- Anthem has also enabled real-time access to eligibility and benefit verification APIs that can be directly integrated within participating vendors' practice management software, revenue cycle management software, and some EMR software. Contact Availity for available vendor integration opportunities.

Section 3: Prior authorization submission, attachment, status, and clinical appeals.

Providers and Facilities should leverage these channels for prior authorization submission, status inquiries, and submit electronic attachments related to prior authorization submissions:

- EDI transaction: X12 278 – prior authorization and referral:
  - Anthem supports the industry standard X12 278 transaction for prior authorization submission and status inquiry as mandated per HIPAA.
- EDI transaction: X12 275 – patient information, including HL7 payload for authorization attachments:
  - Anthem supports the industry standard X12 275 transaction for electronic transmission of supporting authorization documentation, including medical records via the HL7 payload.
- Availity Essentials:
  - The Availity Essentials multi-payer Authorization application facilitates prior authorization submission, authorization status inquiry, and the ability to review previously submitted authorizations.
- Provider desktop integration via B2B APIs:
  - Anthem has enabled real-time access to prior authorization APIs, which can be directly integrated within participating vendors' practice management software, revenue cycle management software, and some EMR software. Contact Availity for available vendor integration.

Section 4: Claims: submissions, Claims payment disputes, attachments, and status

<u>Claim submissions status and Claims payment disputes</u>

Providers and Facilities should leverage these channels for electronic Claim submission, attachments (for both pre- and post-payment), and status:

- EDI transaction: X12 837 – professional, institutional, and dental Claim submission (version 5010):
  - Anthem supports the industry standard X12 837 transactions for all fee-for-service and encounter billing as mandated per HIPAA.
  - 837 Claim batch upload through EDI allows Providers and Facilities to upload a batch/file of Claims (must be in X12 837 standard format).
- EDI transaction: X12 276/277 – Claim status inquiry and response:
  - Anthem supports the industry standard X12 276/277 transaction set for Claim status inquiry and response as mandated by HIPAA.
- Availity Essentials – Claims & Payments application
  - The Claims & Payments application enables Providers and Facilities to enter a Claim directly into an online Claim form and upload supporting documentation for a defined Claim.

- The Claim Status application enables Providers and Facilities to access online Claim status. Access the Claim payment dispute tool from Claim Status. Claims Status also enables online Claim payment disputes in most markets and for most Claims. It is the expectation of Anthem that electronic Claim payment disputes are adopted when and where they are integrated.

- Provider desktop integration via B2B APIs:
  - Anthem has also enabled real-time access to Claim status via APIs, which can be directly integrated within participating vendors' practice management software, revenue cycle management software, and some EMR software. Contact Availity for available vendor integration.

Claim attachments

Providers and Facilities should leverage these channels for electronic Claim attachments from Availity.com:

- EDI transaction: X12 275 – patient information, including HL7 payload attachment:
  - Anthem supports the industry standard X12 275 transaction for electronic transmission of supporting Claims documentation, including medical records via the HL7 payload.

Availity Essentials – Claim Status application

- The Claim Status application enables Providers and Facilities to digitally submit supporting Claims documentation, including medical records, directly to the Claim.

- Digital Request for Additional Information (Digital RFAI) – the Medical Attachments application on Availity Essentials enables the transmission of digital notifications when additional documentation, including medical records, is needed to process a Claim.

Section 5: Electronic remittance advice and electronic Claims payment

Electronic remittance advice

Electronic remittance advice (ERA) is an electronic data interchange (EDI) transaction of the explanation of payment of your Claims. Anthem supports the industry-standard X12 835 transaction as mandated by HIPAA.

Providers and Facilities can register, enroll, and manage their ERA preference through **Availity.com**. Printing and mailing remittances will automatically stop thirty (30) days after the ERA enrollment date.

- Viewing an ERA on Availity Essentials is under Claims & Payments, Remittance Viewer. Features of remittance viewer, include the ability to search a two (2) year history of remittances and access the paper image.

- Viewing a portable document format (PDF) version of a remit is under Payer Spaces, which provides a downloadable PDF of the remittance.

To stop receiving ERAs for Claims, contact Availity Client Services at **1-800-AVAILITY** (282-4548). To re-enable receiving paper remittances, contact Provider Services.

Electronic Claims payment

Electronic Claims payment is a secure and fast way to receive payment by reducing administrative processes. There are several options to receive Claims payments electronically.

- **Electronic Funds Transfer (EFT)**

  Electronic funds transfer (EFT) uses the automated clearinghouse (ACH) network to transmit healthcare payments from a health plan to a Provider's or Facility's bank account at no charge for the deposit. Health plans can use a Provider's or Facility's banking information only to deposit funds, not to withdraw funds. The EFT deposit is assigned a trace number (TRN) to help match the payment to the correct 835 electronic remittance advice (ERA), a process called reassociation.

  To enroll in EFT: Providers and Facilities can register, enroll, and manage account changes for EFT through EnrollSafe at **enrollsafe.payeehub.org**. EnrollSafe enrollment eliminates the need for paper registration. EFT payments are deposited faster and are generally the lowest-cost payment method. For help with enrollment, **use this convenient EnrollSafe User Reference Manual**.

  To disenroll from EFT: Providers and Facilities are entitled to disenroll from EFT. Disenroll from EFT payments through EnrollSafe at **enrollsafe.payeehub.org**.

- **Virtual Credit Card (VCC)**

  For Providers and Facilities who don't enroll in EFT, and in lieu of paper checks, Anthem is shifting some reimbursements to a virtual credit card (VCC). VCC allows Providers and Facilities to process payments as credit card transactions. Check with your merchant processor regarding standard transaction fees that will apply.
  Note that Anthem may receive revenue for issuing a VCC.
  Opting out of virtual credit card payment. Providers and Facilities are entitled to opt out of electronic payment. To opt out of virtual credit card payment, there are two (2) options:

  - Enrolling for EFT: To receive EFT payments instead of virtual credit card payments, enroll for EFT through EnrollSafe at **enrollsafe.payeehub.org**.

    OR

  - To opt out of virtual credit card payments, call **800-833-7130** and provide your taxpayer identification number.

- **Zelis Payment Network (ZPN) electronic payment and remittance combination**

  The Zelis Payment Network (ZPN) is an option for Providers and Facilities looking for the additional services Zelis can offer. Electronic payment (ACH or VCC) and Electronic Remittance Advice (ERA) via the Zelis website are included, together with additional services. For more information, go to **Zelis.com**. Zelis may charge fees for its services.

  Note that Anthem may receive revenue for issuing ZPN.

  ERA through Availity is not available for Providers and Facilities using ZPN.

  To disenroll from ZPN payment, there are two (2) options:

  - Enrolling for EFT: To receive EFT payments instead of ZPN payments, enroll for EFT through EnrollSafe at **enrollsafe.payeehub.org**.

    OR

  - To disenroll from ZPN payments, update your Zelis registration on the Zelis provider portal or contact Zelis at **877-828-8770**.

Not being enrolled in a digital payment method such as EFT, VCC, or ZPN will result in payment via paper checks, which may be shifted to the VCC reimbursement method.

Section 6: Provider Enrollment and Network Management

Provider Enrollment

- **Simplified enrollment process:** Providers and Facilities can enroll as a new provider in our network for professional, ancillary, institutional, and facility provider types through Availity Essentials.
- **Real-Time Tracking:** Providers and Facilities can track the status of their requests in the **My Dashboard** section of the Provider Enrollment and Network Management application.

Contract Changes

- **Streamlined Contract Change Requests**: Providers and Facilities can easily submit certain requests for contract changes through Availity Essentials:
  - Amendment requests to add a network or line of business
  - Change of Ownership notice
  - Contract, line of business, or Network Termination requests
  - TIN Change
- **Real-Time Tracking**: Providers and Facilities can track the status of their requests in the **My Dashboard** section of the Provider Enrollment and Network Management application.

My Roster

Providers and Facilities can download roster data through Availity Essentials. This allows the review of data within our system and updates using the formatted file to provide changes.

- To request a roster, go to Availity.com > Payer Spaces > Select Payer Tile > Provider Enrollment and Network Management > Request Current Roster.
- Providers and Facilities will be prompted to select the organization name and TIN they would like included in the roster. Multiple TINs can be included in one request.
- A Roster File Submission confirmation message displays. When the roster is ready to download, click Download Requested Roster. Rosters are usually available within 4 hours. Rosters cannot be requested more than once a day.

Providers and Facilities can edit the downloaded roster and upload the updated version via Availity's Provider Data Management *Upload Roster File* screen to easily update their data. Because the download is correctly formatted, it should enable automatic processing.

See *Section 7: Demographic Updates* section below for more information about Provider Data Management.

Provider and Facilities access for Provider Enrollment and Network Management features:

- To access these features, go to **Availity.com** > Payer Spaces > Select Payer Tile > Provider Enrollment and Network Management application.
- For organizations already using Availity Essentials, the organization's Availity Essentials administrator should go to **My Account Dashboard** from the Availity Essentials home page to register new users and update or unlock accounts for existing users. Staff who need access to

the Provider Enrollment and Network Management application need to be granted the role of Provider Enrollment.

Section 7: Demographic updates

Provider Data Management (PDM)

Availity Essentials Provider Data Management (PDM) is the digital intake application for Providers and Facilities to submit demographic change requests – it is also where Providers can upload a roster with demographic changes. If submitting a roster, find the TIN/business name for which you want to verify and update information. Before you select the TIN/business name, select the three-bar menu option on the right side of the window, select Upload Rosters, and follow the prompts.

For Providers and Facilities using the roster upload option, additional resources are available:

- **Error Report:**
  - Providers and Facilities can use this Error Report to understand where errors occurred (specifically which sheet, tab, and row), the cause of the issue, and how to fix it.
  - Providers and Facilities are responsible for using the Error Report to identify errors in a roster, correct them, and resubmit the roster rows that contain errors. Rows in a roster that contain an error will not be processed, and the addition, change, or termination will not be updated in our systems.
- **Results Report:** When a roster has the status partially complete or complete, a Results Report will be created for any rosters received on and after June 15, 2024. The Results Report is an Excel file that shows the adds and updates made to your provider group's demographic data based on the information contained in a specific roster.
- **Use the *Roster Submission Guide:*** For Providers and Facilities using the roster upload option, additional information about the Error Report and Results Report can be found in our *Roster Submission Guide*. Find it online at **Availity.com** > Payer Spaces > Select Payer Tile > Resources > Roster Submission Guide using Provider Data Management.

Provider Participation

Credentialing

Credentialing is the process Anthem uses to evaluate healthcare practitioners and health delivery organizations (HDOs) to provide care to Members to help ensure Anthem's Credentialing standards of professional conduct and competence are met.

Anthem's Program Summary includes a complete list of the provider types within Anthem's credentialing scope. The credentials of healthcare practitioners and HDOs are evaluated according to Anthem's criteria, standards, and requirements as set forth in our Program Summary and applicable state and federal laws, regulatory, and accreditation requirements. Anthem retains discretion to amend, change, or suspend any aspect of Anthem's Credentialing Program, and the Program Summary is not intended to create rights on the part of practitioners or HDOs who seek to provide healthcare services to Members. Anthem further retains the right to approve, suspend, or terminate individual practitioners and HDOs in those instances where it has delegated credentialing decision-making.

Anthem's Credentialing Program also includes the recredentialing process, which incorporates re-verification and the identification of changes in the practitioner's or HDO's credentials that may reflect on the practitioner's or HDO's professional conduct and competence. This information is reviewed in order to assess whether practitioners and HDOs continue to meet Anthem credentialing standards. All applicable practitioners and HDOs in Anthem's network within the scope of the Credentialing Program are required to be recredentialed at least every three (3) years unless otherwise required by applicable state contract or state regulations. Additional information regarding Anthem's Credentialing Program can be found in the Program Summary, which includes applicable terms incorporated into this Provider Manual by reference, available on anthem.com. To access the Program Summary, go to the **Anthem Provider Home page**, under the **Provider Enrollment and Maintenance** menu, select **Join our Network**, then select **Program Summary** under the question, **Who do we Credential?.**

Non-Credentialed Providers and Facilities Participation Standards

Anthem contracts with many types of providers that do not require credentialing, as described in the **Credentialing Program Summary** available on the Anthem website. However, to become a Network/Participating Provider or Facility, certain participation standards must still be met. In addition to the insurance requirements listed in the Legal and Administrative Requirements section of this manual, and standards of participation and accreditation requirements outlined in the Provider Agreement, the chart below outlines requirements that must be met in order to be considered for contracting as a Network/Participating Provider or Facility in one of these specialties.

Provider/Facility	Standards of Participation
Ambulance (air and ground)	Medicare Certification/State Licensure
Ambulatory Event Monitoring	Medicare Certification
Convenient Care Centers (CCCs)/ Retail Health Clinics (RHC)	DNV/National Integrated Accreditation for Healthcare Organizations (NIAHO) Urgent Care Association of America (UCAOA) The Joint Commission (TJC)

Provider/Facility	Standards of Participation
Hearing Aid Supplier	State Licensure
Intermediate Care Facilities	The Compliance Team
Immunization Clinic	CDC Certification, Pharmacy License, Medicare Certification
Private Duty Nursing	The Joint Commission (TJC) Community Health Accreditation Partner (CHAP) The Compliance Team Accreditation Commission for Healthcare (ACHC) or DNV/National Integrated Accreditation for Healthcare Organizations (NIAHO)
Urgent Care Center (UCC)	Accreditation Association for Ambulatory Health Care (AAAHC) Institute for Medical Quality (IMQ) National Urgent Care Center Association (NUCCA) The Joint Commission (TJC) Urgent Care Association of America (UCAOA)

For questions about whether a Provider or Facility is subject to the formal credentialing process or the applicable standards of participation, contact Network Management.

Claims Submission

Electronic Claims Submissions

Providers and Facilities are expected to submit Claims electronically whenever possible. Claims must be submitted within the timely filing timeframe specified in the Provider or Facility Agreement. Refer to the Electronic Data Interchange (EDI) Submissions section below for more details about electronic submissions, and to learn more about how EDI can work for Providers and Facilities.

Recommended fields for Electronic 837 Professional (837P) and Institutional (837I) Health Care Claims.

Reference the Transaction Specific Companion Documents available on the **EDI Companion Guide webpage**. Go to the **Anthem Provider Home page**, then under the **Claims** menu, select **Electronic Data Interchange (EDI)**, scroll down to **Review the Guide**, then see the appropriate link under the *Section B – Transaction Specific Companion Documents* heading.

For instructions on connecting and submitting to the Availity Essentials EDI Gateway, review the **Availity Essentials Batch Companion Guide** and the **Availity EDI Connection Guide**.

Electronic Data Interchange (EDI)

Anthem uses Availity as our EDI gateway for managing all electronic data interchange (EDI) transactions. Electronic Data Interchange (EDI), including Electronic Remittance Advice (835), Electronic Funds Transfer (EFT), enables a faster, more efficient, and cost-effective way to work together.

Payer IDs

Payer IDs route EDI transactions to the appropriate payer. The **Availity Essentials Payer ID list** is available on Availity.com. If a provider or facility uses a clearinghouse, billing service, or vendor, work with them directly to determine the payer ID.

Advantages of Electronic Data Interchange (EDI)

- Faster claims processing that allows submissions of corrected claims, primary payment detail, and offers choices for submitting documentation to support your claims.
- Reduce overhead and administrative costs by eliminating paper claim submissions

Use Availity for the following EDI transactions

- Healthcare Claim: Professional (837P)
- Healthcare Claim: Institutional (837I)
- Healthcare Claim: Dental (837D)
- Healthcare Eligibility Benefit Inquiry and Response (270/271)
- Healthcare Services Prior Authorization (278)
- Healthcare Services Inpatient Admission and Discharge Notification (278N)
- Healthcare Claim Payment/Electronic Remittance Advice (835)
- Healthcare Claim Status Request and Response (276/277)

- Medical Attachments (275)

How Providers and Facilities Can Efficiently Use the Availity EDI Gateway

Availity's EDI submission options:

- Availity EDI Clearinghouse for Direct Submitters (requires practice management or revenue cycle software)
- Or use the provider or facility's existing clearinghouse or billing vendor. Requires the vendor to have a connection to the Availity EDI Gateway

Electronic Data Interchange Trading Partner

Trading partners connect with Availity's EDI gateway to send and receive EDI transmissions. An EDI trading partner can be a Provider organization using software to submit direct transmissions, a billing company, or a clearinghouse vendor.

To become an EDI trading partner, visit Availity.com.

Select **Login** if already an Availity Essentials user, choose **My providers** > **Transaction Enrollment,** or choose **Register** if new to Availity Essentials.

EDI Response Reports

Claims submitted electronically will return response reports that may contain rejections. If using a clearinghouse or billing vendor, please work with them to ensure you are receiving all reports.

It's important to review the response reports as rejections will require correction and resubmission. For questions on electronic response reports, contact your clearinghouse or billing vendor, or Availity if you submit directly using your practice management software at **800-AVAILITY (800-282-4548)**.

Electronic Funds Transfer (EFT)

Electronic claims payment through electronic funds transfer (EFT) is a safe, secure, and fast way to receive payment. There is no charge for the deposit, and EFT reduces administrative time related to posting and reconciling payments. EFT deposits are assigned a trace number that is matched to the 835 Electronic Remittance Advice (ERA) for simple payment reconciliation.

To register or manage Electronic Funds Transfer (EFT), use EnrollSafe at **enrollsafe.payeehub.org** to register and manage EFT account changes.

Access EFT enrollment through the website. Go to the **Anthem Provider Home page**, then under the **Claims** menu, select **Electronic Data Interchange (EDI).** Under **EDI Resources**, select the Electronic Funds Transfer tab.

Virtual Credit Cards (VCCs)

For Providers and Facilities who don't enroll in EFT, and in lieu of paper checks, Anthem is shifting some reimbursements to virtual credit cards (VCCs). VCCs allow Providers and Facilities to process payments as credit card transactions. Check with your merchant processor regarding standard transaction fees that will apply. For detailed information, refer to the *Provider and Facility Digital Guidelines* section of this Manual.

Electronic Remittance Advice (ERA) 835

The 835 electronic remittance advice (ERA) eliminates the need for paper remittance reconciliation. Use Availity Essentials to register and manage ERA account changes:

1. Log in to **Availity.com**

2. Select **My Providers**
3. Select **Enrollment Center** and Select **Transaction Enrollment**

Note: *If you use a clearinghouse or vendor, work with them for ERA registration and receiving your ERAs.*

Use EDI to submit corrected claims

For corrected electronic claims, use one of the following frequency codes:

- 7 – Replacement of Prior Claim
- 8 – Void/Cancel Prior Claim

EDI segments required:

- Loop 2300 – CLM – Claim frequency code
- Loop 2300 – REF – Original claim number

Work with your vendor on how to submit corrected claims or contact Availity.

Contact Availity Essentials

Contact Availity Client Services with any questions at 1-800-Availity (282-4548)

Useful EDI Documentation

- **Anthem EDI Webpage** – This webpage contains the payer-specific companion guides and links to the Availity Payer ID list.
- **Availity EDI Connection Service Startup Guide** – This guide includes information to get started with submitting Electronic Data Interchange (EDI) transactions to Availity Essentials, from registration to ongoing support.
- **Availity EDI Companion Guide** – This Availity Essentials EDI Guide supplements the HIPAA TR3s and describes the Availity Essentials Health Information Network environment, interchange requirements, transaction responses, acknowledgments, and reporting for each of the supported transactions as related to Availity Essentials.
- **Availity Essentials Registration Page** – Availity Essentials registration page for users new to Availity Essentials.
- **X12 External Code Listing** – X12 code descriptions used on EDI transactions.

Paper Claims Submission

Digital claim submission, either through the claim submission applications on Availity.com or through EDI, is the preferred method for receiving claims. Filing paper claims can cause delays due to errors associated with using this manual claim submission process. If Providers or Facilities file a paper Claim, failure to submit them on the most current CMS-1500 (Form 1500 (02-12)) or CMS-1450 (UB-04) will cause the Claims to be rejected and returned to the Provider or Facility. More information and the most current forms can be found at **cms.gov.**

- Submit all paper Claims using the current standard RED CMS Form 1500 (02-12) for professional Claims and the UB-04 (CMS-1450) for Facility Claims.
- If Providers or Facilities are submitting a multiple-page Claim, the word “continued” should be noted in the total charge field, with the total charge submitted on the last page of the Claim.

- When submitting a multiple-page document, do not staple over pertinent information.
- Complete all mandatory fields.
- Do not highlight any fields.
- Check the printing of Claims from time to time to help ensure proper alignment and that characters are legible.
- Ensure all characters are inside the appropriate fields and do not overlap.
- Change the printer cartridge regularly and do not use a DOT matrix printer.
- Submit a valid Member identification number, including a three-digit prefix or R+8 numeric for Federal Employee Program® (FEP®) Members on all pages.
- Claims must be submitted with complete provider information, including referring, rendering, and billing NPI; tax identification number; name; and servicing and billing addresses on all pages.

Recommended CMS Form 1500 (02-12)

A sample form and instructions are available on **cms.gov**.

UB-04 (CMS-1450)

A sample form is available on the **CMS website: CMS Forms | CMS, along with instructions on how to complete the paper claim form**

Claim Submission Filing Tips

Eliminate processing delays and unnecessary correspondence with these Claim filing tips.

Ambulatory Surgical Centers

When billing revenue codes, always include the CPT or HCPCS code for the surgery being performed. This code is required to determine the procedure, and including it on the Claim helps Anthem process the Claim correctly and more quickly. Ambulatory surgical Claims must be billed on a CMS-1500 (Form 1500 (02-12)) or CMS-1450 (UB-04), as indicated in the Agreement.

Ancillary Filing Guidelines

Ambulance Claims

- Include the Point of Pickup (POP) ZIP Code for all ambulance (including air ambulance) Claims, both institutional outpatient and professional.
- Ground or independently contracted ambulance Providers should file the Claims to the plan whose service area the Point of Pickup (POP) ZIP Code is located.
- Air Ambulance Providers contracted through a facility and submitting services on UB-04 CMS 1450 (facility claim forms), should file claims to the Plan whose service area matches the facility (local Plan).
- The POP (Point of Pick-up) ZIP Code should be submitted as follows:
    - Professional Claims – for CMS-1500 submitters: the POP ZIP code is reported in field 23 or 54
    - Institutional outpatient Claims – for UB submitters: the Value Code of ‘A0’ (zero), and the related ZIP Code of the geographic location from which the beneficiary was placed on

board the ambulance, should be reported in the Value Code Amount field and billed with the appropriate revenue 54x codes.

Durable/Home Medical Equipment and Supplies

Durable/Home Medical Equipment and Supplies (D/HME) is determined by the provider specialty code in the provider file, not by CPT codes.

- Delivered to the patient's home – File the Claim to the plan in the service area where the item was sent/delivered.
- Purchased at a retail store – File the Claim to the plan in the service area where the retail store is located.

Home Infusion Therapy – Services and Supplies

File the Claim with the plan in the service area where the services are rendered, or the supply was delivered. Examples:

- If services are rendered in a Member's home, Claims should be sent to the plan in the Member's state.
- If Supplies are delivered to the Member's home, Claims should be sent to the plan in the Member's state.

Independent Clinical Laboratory Claims

- File the Claim to the plan in the service area where the specimen was drawn, as determined by the referring Provider's location (based on NPI)
- Independent lab Claims are determined by the place of service 81.
- Unless exempted by state or other legal guidelines, Anthem requires the CLIA number to be included on each Claim billed for laboratory services by any Provider or Facility performing tests covered by CLIA. Anthem requires the CLIA identification number to be submitted based on the applicable method below:
    - ASC X12 837 professional Claim format REF segment as REF02, with qualifier of "X4" in REF01 or
    - Field 23 of the paper CMS-1500

Specialty Pharmacy Claims

- File the Claim to the plan in the service area where the referring provider is located (based on NPI).
- Specialty pharmacy Claims are determined by the provider specialty code in the provider file, not by CPT codes.

CPT Coding

The most current version of the CPT® Professional Edition manual is considered by Anthem as the industry standard for accurate CPT and modifier coding.

Diagnostic Radiopharmaceuticals

Diagnostic radiopharmaceuticals are considered integral components of the diagnostic service. These items are not reimbursed separately to the facility unless reimbursement is explicitly allowed under the applicable drug fee schedule.

Duplicate Claims

Providers and Facilities should refrain from submitting a Claim multiple times to avoid potential duplicate denials. Providers or Facilities can check the status of Claims via Availity Essentials. From the Claims & Payments tab, select Claims Status.

Late Charges

Late charges for Claims previously filed can be submitted electronically. Providers and Facilities must reference the original Claim number when submitting a corrected electronic Claim. If attachments are required, submit them using the *Attachment Face Sheet*. Refer to the **Electronic Data Interchange webpage** on the Anthem website for instructions.

Late charges for previously filed Claims can be submitted on paper. The type of bill should contain a five (5) in the third position of the TOB (ex, 135). A late billing should contain ONLY the additional late charges. Providers and Facilities should also indicate the original Claim number to which the late charges should be added.

Maternity Delivery Claims

Delivery procedure codes reported on a professional Claim (procedure codes: 59612, 59620, 59400, 59410, 59515, 59614, 59622, 59510, 59610, or 59618) are required to submit with the appropriate Z3A diagnosis code indicating the baby's gestational age.

National Drug Codes (NDC)

See the separate subsection titled *National Drug Codes*.

Negative Charges

When filing Claims for procedures with negative charges, don't include these lines on the Claim. Negative charges often result in an out-of-balance Claim that must be returned to the Provider for additional clarification.

Not Otherwise Classified (NOC) Codes

When submitting Not Otherwise Classified (NOC) codes, follow these guidelines to avoid possible Claim processing delays: **Anthem must have a clear description of the item/service billed with a NOC code for review**.

- If the NOC is for a drug, include the drug's name, dosage, NDC number, and number of units. NOC drugs will be priced according to the following methodology if there is no fixed Case Rate on the claim:
  - The WAC (Wholesale Average Cost) + 0%
  - The AWP (Average Wholesale Price) - 16%
- If the NOC is not a drug, include a specific description of the procedure, service, or item.
- If the item is durable medical equipment, include the manufacturer's description, model number, and purchase price if rental equipment.
- If the service is a medical or surgical procedure, include a description on the Claim and submit the medical record/and the operative report (if surgical) that support the use of an NOC and medical necessity for the procedure.
- If the NOC is for a laboratory test, include the specific name of the laboratory test(s) and/or a short descriptor of the test(s)

Note: NOC codes should only be used if there are no appropriate listed codes available for the item or service. Descriptions should be included in the shaded area for item 24 on professional Claim forms, or locator 43 on facility Claim forms.

Occurrence Dates

When billing facility Claims, make sure the surgery date is within the service from and to dates on the Claim. Claims that include a surgical procedure date that falls outside the service from and to dates will be returned to the provider.

Other Insurance Coverage

When filing Claims with other insurance coverage, ensure the following fields are completed and that a legible copy of the Explanation of Benefits (EOB) from the other insurance coverage is attached to the Claim:

- CMS-1500 Fields:
  - Field 9: Other insured's name
  - Field 9a: Other insured's policy or group number
  - Field 9b: Other insured's date of birth
  - Field 9c: Employer's name or school name (not required in EDI)
  - Field 9d: Insurance plan name or program name (not required in EDI)
- UB-04 CMS-1450 Fields:
  - Field 50a-c: Payer Name
  - Field 54a-c: Prior payments (if applicable)

Including Explanation of Medicare Benefits (EOMB) or other payer Explanation of Benefits (EOB):

When submitting a CMS Form 1500 (02-12) or CMS-1450 (UB-04) Claim form with an Explanation of Medicare Benefits (EOMB) attached, the EOMB should indicate Medicare's Assignment. When submitting a CMS Form 1500 (02-12) or CMS-1450 (UB-04) Claim form with an Explanation of Medicare Benefits (EOMB) or other payer Explanation of Benefits (EOB) attached, the EOMB or EOB should match each service line and each service line charge submitted on the CMS Form 1500 (02-12) or CMS-1450 (UB-04).

Preventive Colonoscopy – correct coding

Anthem allows for preventive colonoscopy in accordance with state mandates. Colonoscopies, which are undertaken as a SCREENING colonoscopy, during which a polyp/tumor or other procedure due to an abnormality is discovered, should be covered under benefits for Preventive Services. This has been an area of much confusion in billing by Providers and Facilities for services. Frequently, the Provider or Facility will bill for the CPT code with an ICD-10 diagnosis code corresponding to the pathology found rather than the "Special screening for malignant neoplasms, of the colon".

CMS has issued guidance on correct coding for this situation and states that the ICD-10 diagnosis code Z12.11 (**Encounter for screening for malignant neoplasm of colon**) should be entered as the primary diagnosis and that the ICD-10 diagnosis code for any discovered pathology should be entered as the secondary diagnosis on all subsequent Claim lines.

Anthem endorses this solution for this coding issue as the appropriate method of coding to ensure that the Provider or Facility receives the correct reimbursement for services rendered and that Members receive the correct benefit coverage for this important service.

Type of Billing Codes

When billing facility Claims, ensure the type of bill coincides with the revenue code(s) billed on the Claim. For example, if billing an outpatient revenue code, the type of bill must be for outpatient services.

Claim Inquiry/Adjustment Filing Tips

The different types of Claim inquiries should be handled in separate ways depending on what is being requested. Here are some examples:

- **Claim Inquiry:** A question about a Claim or Claim payment is called an inquiry. Claim Inquiries do not result in changes to Claim payments, but the outcome of the Claim Inquiry may result in the initiation of the Claim Payment Dispute. In other words, once the Provider or Facility receives the answer to the Claim Inquiry, the Provider or Facility may opt to begin the Claim Payment Dispute process.

  Providers and Facilities can chat with Payer or send a Secure Message through the Availity Essentials. If Providers or Facilities are unable to utilize the Availity Essentials for the inquiry, they can call the number on the back of the Member ID Card and select the Claims prompt. Reference the Availity Essentials section for further details on Secure Messaging.

- **Claim Correspondence:** Claim Correspondence is when Anthem requires more information to finalize a Claim. Typically, Anthem makes the request for this information through the Explanation of Payment (EOP). The Claim or part of the Claim may be denied, but it is only because more information is required to process the Claim. Once the information is received, Anthem will use it to finalize the Claim. To upload the requested documentation from Availity.com, select the Claims & Payments tab to access Claims Status. Enter the necessary information to locate the claim and use the Submit Attachments button to upload the requested documentation.

- **Clinical Medical Necessity Appeals:** Information about an appeal regarding a clinical decision denial, such as an authorization or Claim, that has been denied as not medically necessary, experimental/investigational, is located in the *Clinical Appeals* section within the Provider Manual.

- **Claim Payment Disputes:** Refer to the *Claim Payment Dispute* section for further details.

- **Precertification Disputes:** Precertification disputes should be handled via the process detailed in the letter received from the Utilization Management department. If Providers or Facilities disagree with a clinical decision, follow the directions detailed in the letter. A precertification appeal can be submitted through the digital prior authorization application on Availity.com. Select the Patient Registration tab to access Authorizations & Referrals. Sending precertification requests or appeals to the provider correspondence address may delay responses.

- **Corrected Claims:** Submit a corrected Claim only when updating information on the Claim form. Access your claim on Availity.com through the Claims & Payments tab. If the inquiry is about the way the Claim processed, refer to the prior sections. If Providers or Facilities have corrections to the Claim, submit them according to the Corrected Claim Guidance below.

Proof of Timely Filing

Claims must be submitted within the timely filing timeframe specified in the Provider or Facility Agreement.

All additional information reasonably required by Anthem to verify and confirm the services and charges must be provided on request.

Claims submitted after the timely filing period expires will be denied, unless proof of timely filing can be demonstrated according to the guidelines listed below.

Waiver of the timely filing requirement is only permitted when Anthem has received documentation indicating the Member, Provider, or Facility originally submitted the Claim within the applicable timely filing period.

The documentation submitted must indicate that the Claim was originally submitted before the timely filing period expired.

Acceptable documentation includes the following:

1. A copy of the Claim with a computer-printed filing date (a handwritten date isn't acceptable)
2. An original fax confirmation specifying the Claim in question and including the following information: date of service, amount billed, Member name, original date filed with Anthem, and description of the service.
3. The Provider or Facility's billing system printout showing the following information: date of service, amount billed, Member name, original date filed with Anthem, and description of the service.

   If the Provider or Facility doesn't have an electronic billing system, approved documentation is a copy of the Member's chart indicating the billed date and/or a copy of the billing records indicating the billed date, and the information listed above.
4. If the Claim was originally filed electronically, a copy of Anthem's electronic Level 2 or the respective clearinghouse's acceptance/rejection Claims report is required; a copy can be obtained from the Provider or Facility's EDI vendor, EDI representative, or clearinghouse representative. The Provider or Facility also must demonstrate that the Claim and the Member's name are on the original acceptance/rejection report.

   Note: When referencing the acceptance/rejection report, the Claim must show as accepted to qualify for proof of timely filing. Any rejected Claims must be corrected and resubmitted within the timely filing period.
5. A copy of the Anthem letter requesting additional Claim information showing the date information was requested.

Appeals for Claims denied for failing to meet timely filing requirements must be submitted to Anthem **in writing**. Anthem doesn't accept appeals over the phone.

Corrected Claim Guidance

When submitting a correction to a previously submitted Claim, submit the entire Claim as a replacement Claim if Providers or Facilities have omitted charges or changed Claim information (i.e., diagnosis codes, procedure codes, dates of service, etc.), including all previous information and any corrected or additional information. To correct a Claim that was billed to Anthem in error, submit the entire Claim as a void/cancel of the prior Claim. If there is a zero Member, Provider, or Facility liability, then a new Claim is needed versus a corrected Claim.

Regarding paper claims: Claims originally filed on paper are accessible through Availity.com. Submit replacement, void/canceled claims through Availity.com following the instructions below for digital submission. Do not use the paper submission process unless there is a specific reason for filing a paper claim correction.

Type	Professional Claim	Institutional Claim
EDI	**To indicate the Claim is a replacement Claim:** • In element CLM05-3 "Claim Frequency Type Code" • Use Claim Frequency Type 7	**To indicate the Claim is a replacement Claim:** • In element CLM05-3 "Claim Frequency Type Code" • Use Claim Frequency Type 7
	To confirm the Claim which is being replaced: • In Segment "REF – Payer Claim Control Number" • Use F8 in REF)! and list the original payer Claim number is REF02	**To confirm the Claim which is being replaced:** • In Segment "REF – Payer Claim Control Number" • Use F8 in REF)! and list the original payer Claim number is REF02
	To indicate the Claim was billed in error (Void/Cancel): • In element CLM05-3 "Claim Frequency Type Code" • Use Claim Frequency Type 8	**To indicate the Claim was billed in error (Void/Cancel):** • In element CLM05-3 "Claim Frequency Type Code" • Use Claim Frequency Type 8
	To confirm the Claim which is being voided/canceled: • In Segment "REF – Payer Claim Control Number" • Use F8 in REF)! and list the original payer Claim number is REF02	**To confirm the Claim which is being voided/canceled:** • In Segment "REF – Payer Claim Control Number" • Use F8 in REF)! and list the original payer Claim number is REF02
Digital	Submit replacement, void/cancel claims through **Availity.com**	Submit replacement, void/cancel claims through **Availity.com**
	Select the **Claims & Payments tab** and click **Professional Claim**	Select the **Claims & Payments tab** and click **Facility Claim**
	Enter the claim information and set the billing frequency and payer control number as follows: • Select **Replacement of Prior Claim** or **Void/Cancel of Prior Claim** for the **Billing Frequency** (or **Frequency Type**) field in the **Claim Information** • Set the **Payer Control Number (ICN / DCN)** (or **Payer Claim Control Number**) field to the claim number assigned to the claim by the payer. You can obtain this number from the 835, if available.	Enter the claim information and set the billing frequency and payer control number as follows: • Select **Replacement of Prior Claim** or **Void/Cancel of Prior Claim** for the **Billing Frequency** (or **Frequency Type**) field in the **Claim Information** • Set the **Payer Control Number (ICN / DCN)** (or **Payer Claim Control Number**) field to the claim number assigned to the claim by the payer. You can obtain this number from the 835, if available.
Paper	**To indicate the Claim is a replacement Claim:** • In Item Number 22: "Resubmission and/or Original Reference Number" • Use Claim Frequency Type 7 under "Resubmission Code"	**To indicate the Claim is a replacement Claim:** • In Form Locator 04: "Type of Bill" • Use Claim Frequency Type 7

Type	Professional Claim	Institutional Claim
	To confirm the Claim which is being replaced: • In the right-hand side of Item Number 22 under "Original Ref. No." list the original payer Claim number for the resubmitted Claim.	**To confirm the Claim which is being replaced:** • In Form Locator 64: "Document Control Number (DCN)" list the original payer Claim number for the resubmitted Claim.
	To indicate the Claim is a void/cancel of a prior Claim: • In Item Number 22: "Resubmission and/or Original Reference number" • Use Claim Frequency Type 8 under "Resubmission Code"	**To indicate the Claim is a void/cancel of a prior Claim:** • In Form Locator 04: "Type of Bill" • Use Claim Frequency Type 8
	To confirm the Claim which is being voided/canceled: • In the right-hand side of Item Number 22 under "Original Red. No." list the original payer Claim number for the voided/canceled Claim.	**To confirm the Claim which is being voided/canceled:** • In Form Locator 64: "Document Control Number (DCN)" list the original payer Claim number for the voided/canceled Claim.

For additional information on provider disputes and appeals, refer to the *Claim Payment Dispute* and *Clinical Appeals* sections of this Manual.

National Drug Codes (NDC)

All practitioners and providers are required to supply the 11-digit NDC when billing for injections and other drug items on the CMS-1500 and UB-04 Claim forms, as well as on the 837 electronic transactions. Note: These billing requirements will apply to Local Plan and BlueCard Member Claims only, and will exclude Federal Employee Program (FEP) and Coordination of Benefits/ Secondary Claims.

Line items on a Claim regarding drugs administered in a physician's office or outpatient facility setting for all drug categories will deny if they do not include the following:

- Applicable HCPCS code or CPT code
- Number of HCPCS codes or CPT code units
- The valid 11-digit NDC, including the N4 qualifier
- Unit of measure qualifier (F2, GR, ML, UN, ME)
- NDC Units dispensed (must be greater than 0)

Unit of Measurement Requirements

The unit of measurement codes are also required to be submitted. The codes to be used for all Claim forms are:

- F2 – International unit
- GR – Gram
- ML – Milliliter
- UN – Unit
- ME – Milligram

Location of the NDC

The NDC is found on the label of a prescription drug item and must be included on the CMS-1500 or UB-04 Claim form or in 837 electronic transactions. The NDC is a universal number that identifies a drug or related drug item.

NDC Number Section	Description
1 (five digits)	Vendor/distributor identification
2 (four digits)	Generic entity, strength, and dosage information
3 (two digits)	Package code indicating the package size

Correcting Omission of a Leading Zero

Providers and Facilities may encounter NDCs with fewer than 11 digits. In order to submit a Claim, Providers and Facilities will need to convert the NDC to an 11-digit number. Sometimes the NDC is printed on a drug item, and a leading zero has been omitted in one of the segments. Instead of the digits and hyphens being in a 5-4-2 format, the NDC might be printed in a 4-4-1 format (example, 1234-1234-1), a 5-3-2 format (example, 12345-123-12), or a 5-4-1 format (example, 12345-1234-1).

- If this occurs, when entering the NDC on the Claim form, it will be required to add a leading zero to the beginning of the segment(s) that are missing the zero.
- Do not enter any of the hyphens on Claim forms.

See the examples that follow:

If the NDC appears as…	Then the NDC…	And it is reported as …
NDC 12345-1234-12 (5-4-2 format)	Is complete	12345123412
NDC 1234-1234-1 (4-4-1 format)	Needs a leading zero placed at the beginning of the first segment and the last segment	01234123401
NDC 12345-123-12 (5-3-2 format)	Needs a leading zero placed at the beginning of the second segment	12345012312
NDC 12345-1234-1 (5-4-1 format)	Needs a leading zero placed at the beginning of the third segment	12345123401

Process for Multiple NDC numbers for Single HCPC Codes

- If there is more than one NDC within the HCPCs code, Providers and Facilities must submit each applicable NDC as a separate Claim line. Each drug code submitted must have a corresponding NDC on each Claim line.
- If the drug administered is comprised of more than one ingredient (i.e., compound or same drug with different strength, etc.), Providers and Facilities must represent each NDC on a Claim line using the same drug code.
- Standard HCPCs billing accepts the use of modifiers to determine when more than one NDC is billed for a service code. They are:
    - KO – Single drug unit dose formulation

- KP – First drug of a multiple drug unit dose formulation
- KQ – Second or subsequent drug of a multiple drug unit dose formulation
- JW – Drug amount discarded /not administered to the patient

How/Where to Place the NDC on a Claim Form

837 Reporting Fields

Providers and Facilities will need to notify billing or software vendors that the NDC is to be reported in the following fields in the 837 format.

Loop	Segment	Element Name	Information	Sample
2410	**LIN02**	Product or Service ID Qualifier	Enter product or NDC qualifier N4	LIN**__**N4**__*01234567891~
2410	**LIN03**	Product or Service ID	Enter the NDC	LIN**N4***__**01234567891**__~
2410	**CTP04**	Quantity	Enter quantity billed	CTP****__**2**__*UN~
2410	**CTP05-1**	Unit of Basis for Measurement Code	Enter the NDC unit of measurement code: F2: International unit GR: Gram ML: Milliliter UN: Unit ME: Milligram	CTP****2*__**UN**__~
2410	**REF01**	Reference ID Qualifier (used to report Prescription # **or** Link Sequence Number when reporting components for a Compound Drug)	VY: Link Sequence Number XZ: Prescription Number	REF01*__**XZ**__*123456~
2410	**REF02**	Reference Identification	Prescription Number **or** Link Sequence Number	REF01*XZ*__**123456**__~

Digital submission through Availity.com:

- From Availity.com, select the Claims & Payments tab, then select Professional Claim or Facility Claim.
- Enter the NDC code in the NDC Code field that is associated with the procedure code/service line.
- In the NDC Quantity field, you can enter a maximum of thirteen (13) numbers before the decimal point and a maximum of two numbers after the decimal point.
- Convert the NDC to 11 digits following the instructions noted above.

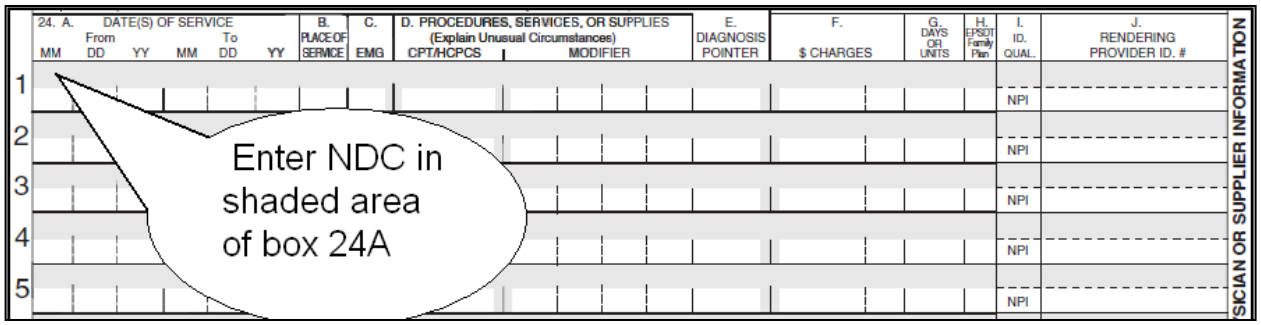
For more information about how to submit an electronic claim, including the NDC Code field using Availity Essentials, log onto Availity.com, select the Help & Training tab, and enter Professional or Facility Claim in the search bar.

CMS-1500 Claim Form:

- Reporting the NDC requires using the upper and lower rows on a Claim line. Be certain to line up information accurately so all characters fall within the proper box and row.
- **DO NOT bill more than one NDC per Claim line.**
- Even though an NDC is entered, a valid HCPCS or CPT code must also be entered in the Claim form.
- If the NDC billed does not have a specific HCPCS or CPT code assigned, the appropriate miscellaneous code should be assigned per Correct Coding Guidelines.
- The unit of service for the HCPCS or CPT code is very important. Units for injections must be billed consistent with the HCPCS or CPT description of the code.

The following table provides elements of a proper NDC entry on a CMS-1500 Claim form.
All Elements are REQUIRED:

How	Example	Where
Enter a valid NDC code including the N4 qualifier	NDC 00054352763 is entered as N400054352763	Beginning at left edge, enter NDC in the **shaded area** of box 24A
Enter one (1) of five (5) units of measure qualifiers; F2 – International Unit GR – Gram ML – Milliliter UN – Units ME – Milligrams and quantity, including a decimal point for correct reporting	GR0.045 ML1.0 UN1.000	In the **shaded area** immediately following the 11-digit NDC, enter three (3) spaces, followed by one (1) of five (5) units of measure qualifiers, followed immediately by the quantity
Enter a valid HCPCS or CPT code	J0610 “Injection Calcium Gluconate, per 10 ml” is billed as one (1) unit for each ten (10) ml ampul used	Non-shaded area of box 24D



UB-04 Claim Form:

- Even though an NDC is entered, a valid HCPCS or CPT code must also be entered in the Claim form.

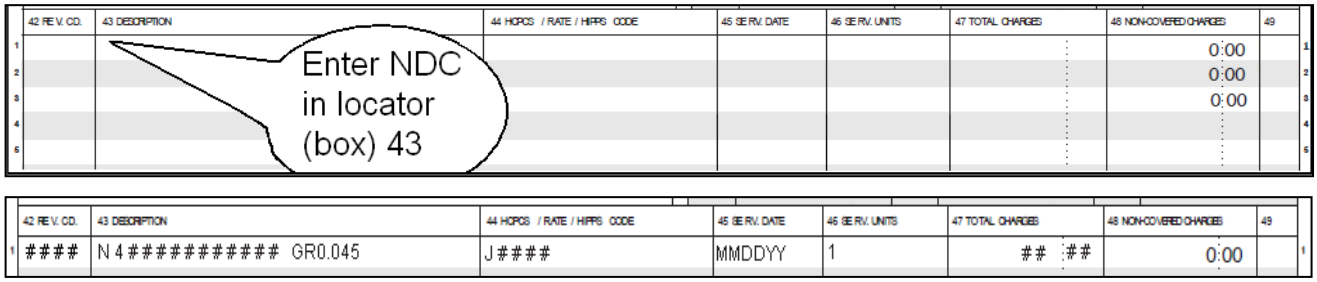
- If the NDC billed does not have a specific HCPCS or CPT code assigned, the appropriate miscellaneous code should be assigned per Correct Coding Guidelines.
- DO NOT bill more than one NDC per Claim line.
- The unit of service for the HCPCS or CPT code is very important. Units for injections must be billed consistent with the HCPCS or CPT description of the code.

The following table provides elements of a proper NDC entry on a UB-04 Claim form.

All Elements are REQUIRED:

How	Example	Where
Enter a valid revenue code	Pharmacy Revenue Code 0252	Form locator (box) 42
Enter 11-digit NDC, including the N4 qualifier	NDC 00054352763 is entered as N400054352763	Beginning at the left edge, enter NDC in the locator (box) 43, currently labeled as “Description”
Enter one (1) of five (5) units of measure qualifiers F2 – International Unit GR – Gram ML – Milliliter UN – Units ME – Milligrams and quantity, including a decimal point for correct reporting	GR0.045 ML1.0 UN1.000	Immediately following the 11-digit NDC, enter three (3) spaces followed by one (1) of five (5) units of measure qualifiers, followed immediately by the quantity.
Enter a valid HCPCS or CPT Code	J0610 “injection Calcium, per 10ML” is billed as one (1) unit for each 10ML ampul used	Form locator (box 44)

Sample Images of the UB04 Claim Form



Medical Records Submission

When submitting documentation in response to Anthem’s request, the recommended method is to submit them electronically via the 275 transaction or digitally through the Attachments Dashboard. To attach requested documentation, navigate to Availity Essentials Claim Status, locate your Claim and use the Send Attachment link to upload your documents.

Always include a copy of the request letter as part of your attachment. The documentation should be formatted as a .tiff, .jpg, or .pdf file. Providers and Facilities should submit medical records

within ten (10) calendar days of Anthem's request, or sooner, depending upon the urgency of the matter and or as required by state or federal law, statute, or regulation. Providers and Facilities can view the status of submitted documentation in Availity Essentials Attachment New.

A Provider or Facility organization's Availity Essentials administrator should complete the following set-up steps to authorize user access to the Medical Attachments New tool:

From **My Providers** select **Enrollments Center > Medical Attachments Setup**, follow the prompts, and complete the following sections:

- Select **Application** > Choose **Medical Attachments Registration**
- Provider Management > Select **Organization** from the drop-down.
  - Add billing NPIs and Tax IDs. (both are recommended)
  - Multiples can be added, separated by spaces or semi-colons.
- Assign user access by checking the box in front of the user's name. Users may be removed by unchecking their name.

If Availity Essentials set-up has not been completed and medical records must be sent via mail or fax, send them to the appropriate department as directed in the notification from Anthem. Do not place a copy of the Claim on top of the records.

If Providers or Facilities are submitting X-rays, pictures, or dental molds, remember to include a valid and complete Member Identification number on page one (1) of the material sent with these items.

Medical Records Submission With Initial Claim

Providers and Facilities can expedite claim processing by sending medical records with the 837 Claim submission or Direct Data Entry.

To determine what medical records or portions of the medical records may be required, refer to the applicable Anthem Medical Policy, Anthem Clinical Guideline, Carelon Clinical Criteria, MCG, at **anthem.com**. Review the Position Statement section of the Anthem Medical Policies, the Clinical Indications section of the applicable Anthem Clinical Guidelines, or the Clinical Criteria section of Carelon to determine what medical records are needed. Refer to the *Medical Policies*, *Clinical Guidelines*, and/or *Carelon Medical Benefits Management* sections of the Provider Manual for details on accessing this information.

When submitting medical records that are not requested by Anthem, include a clear description of the billed code to help ensure prompt processing of the Claim for all miscellaneous, not otherwise classified (NOC), not otherwise specified (NOS), and unlisted HCPCS and CPT codes.

Providers and Facilities can also use the Clinical Documentation Lookup Tool to access information about the documents needed when submitting a claim. Access the Clinical Documentation Lookup Tool from our public website: **clinicaldocumentationtool.anthem.com**.

A Provider or Facility organization's Availity Essentials administrator should complete the set-up steps listed above in the Medical Records Submission section to authorize user access to the Medical Attachments tool.

Submit an EDI 837 (claim) batch, which includes a *PWK segment* containing the attachment control number segment in loops 2300/2400; this detail links the electronic claim and the documentation. The attachment control number can be assigned by the Provider organization or vendor and must be unique. Log in to the Availity Essentials portal

- Select **Claims & Payments** to access **Attachments – New**
- From the **Attachments Dashboard Inbox**, locate the appropriate Claim
- Add files with supporting documentation
- When a PWK segment is submitted with the claim, an intake with the attachment control number will display in the Attachment Dashboard inbox for seven (7) calendar days.
- If the document is not received within the seven (7) calendar day requirement, documentation can be uploaded using Claim Status by locating your Claim and attaching the document.

Digital Request for Additional Information (RFAI)

Providers and Facilities registered for the Medical Attachments application will receive digital notifications when additional documentation is needed to process your Claim. Digital notifications will be posted to your Attachments Dashboard daily when additional documentation is needed. Most Claims will pend for up to thirty (30) days. After the 30-day pend period, the Claim will deny and you will receive the explanation of payment. An additional digital notification will be posted to your Dashboard for an additional forty-five (45) days.

Digital RFAI notifications reduce the amount of time it takes for Anthem to receive the needed documentation to process your Claims. This reduces Claims processing time, and Claims are paid faster.

Visit the Availity, EMR & Digital Solutions webpage on **anthem.com** for more information about Digital RFAI.

Types of Claims Documentation Required

Medical records may be needed to determine the medical necessity of a billed code. The following are examples of the types of records we may need to make the determination. Only submit the records requested for that specific claim, procedure, and date of service. Do not send more records than requested or required:

- History & physical, office visit/clinical notes, treatment records & response
- Chemotherapy regimens, oncology drugs, and records
- Medications list (current and prior)
- Radiology, diagnostic imaging, or diagnostic testing reports
- Therapy/rehabilitation records
- Laboratory reports, pathology reports
- Exact description of NOC/NOS code
- Operative/procedure report
- Inpatient admission summary, history & physical, daily records, discharge summary, physician progress notes, operative/procedure report, CT/MRI report

Anthem May Request Additional Documentation

Some situations may require additional medical records in addition to what was submitted with the Claim. Although these situations may not have specific rules and guidelines, Anthem will make every

effort to make these requests explicit and limited to what is minimally necessary to render a decision. Examples include, but are not limited to, the following situations:

- Medical records requested by a Member's Blue Cross Blue Shield (BlueCard) home plan
- Federal Employee Health Benefits Program (FEP) requirements
- Review and investigation of Claims (e.g., pre-existing conditions [for grandfathered policies of the Affordable Care Act], lifetime benefit exclusions)
- Medical review and evaluation
- Requests for retro authorizations
- Medical management review (utilization review) and evaluation
- Underwriting review and evaluation
- Adjustments
- Appeals
- Quality management (quality of care concerns)
- Records documenting prolonged services
- Provider audits
- Pre-pay review program
- Fraud, waste, and abuse

Medical Record Appeals

When a request for additional information is received in support of the resolution of a grievance or appeal, the Provider or Facility should respond within ten (10) calendar days of the request, or sooner, depending upon the urgency of the matter or as required by state or federal law, statute, or regulation.

HIPAA Privacy Rule – Minimum Necessary

Anthem complies with HIPAA Privacy Rules and will request the minimum necessary information needed to determine benefits and/or coverage associated with Claim processing. Providers and Facilities are also required under the Minimum Necessary rule to submit only those records requested.

Overpayments

Anthem's Payment Integrity department reviews Claims for accuracy and requests refunds if Claims are overpaid or paid in error. Some common reasons for overpayment are:

- Paid the wrong Provider/Member
- Coordination of Benefits
- Allowance overpayments
- Late credits
- Billed in error
- Duplicate

- Non-covered services
- Claims editing
- Terminated Members
- Total charge overpaid
- Paid the wrong Member/ Provider number

Anthem's Payment Integrity department also requests refunds for overpayments identified by other divisions of Anthem, such as Complex and Clinical Audit (CCA) or the Special Investigations Unit (SIU).

Anthem Identified Overpayment (Solicited)

When refunding Anthem on a Claim overpayment that Anthem has requested, use the payment coupon included on the request letter and supply the following information with the check:

- The payment coupon
- Member ID number
- Member's name
- Claim number
- Date of service
- Reason for the refund as indicated in the refund request letter

As indicated in the refund request letter and in accordance with Provider contractual language, and state regulations, Provider overpayment refunds not received and applied within the timeframe indicated will result in Claim recoupment from any Claim the Provider or Facility submits to Anthem.

Providers and Facilities may direct disputes of amounts indicated on an Anthem refund request letter to the address indicated on the letter.

Disputes/Inquiries through Availity

For Providers and Facilities who use the Availity Overpayment Portal, inquiries or disputes must be submitted through Availity.

Provider and Facility Identified Overpayments

If Anthem is due a refund because an overpayment was discovered by a Provider or Facility, refunds can be made in one of the following ways:

- Submit a refund check with supporting documentation outlined below, or
- Submit the request with supporting documentation to have the Claim adjustment/recoupment done off a future remittance advice.

When voluntarily refunding Anthem on a Claim overpayment, include the following information

- All documents supporting the overpayment, including EOBs from Anthem and other carriers as appropriate.
- Member ID number
- Member's name
- Claim number

- Date of service
- Reason for the refund as indicated in the above list of overpayment reasons

Be sure the copy of the Provider remittance advice is legible, and the Member information that relates to the refund is circled. By providing this critical information, Anthem will be able to expedite the process, resulting in improved service and timeliness to Providers and Facilities.

Important Note: *If a Provider or Facility is refunding Anthem due to coordination of benefits and the Provider or Facility believes Anthem is the secondary payer,* ***refund the full amount paid****. Upon receipt and insurance primacy verification, the Claim will be reprocessed and paid appropriately.*

Utilize the correct address noted below to return payment.

Make Check Payable To:	Regular Mailing Address:	Overnight Delivery Address:
Anthem Blue Cross and Blue Shield	Anthem Blue Cross and Blue Shield PO Box 73651 Cleveland, OH 44193-1177	Anthem Blue Cross and Blue Shield Lockbox 73651 4100 West 150th Street Cleveland, OH 44135

Medicare Crossover

Claims Handling for Medicare Crossover

Blue Plans are required to process Medicare crossover Claims for services covered under Medigap and Medicare Supplemental products through the Centers for Medicare & Medicaid Services (CMS). This has resulted in automatic submission of Medicare Claims by Medicare to the Blue secondary payer to eliminate the need for Providers or Facilities or their billing service to submit an additional Claim to the secondary carrier. Additionally, this has also allowed Medicare crossover Claims to be processed in the same manner nationwide.

When a Medicare Claim has crossed over, Providers and Facilities must wait 30 calendar days from the Medicare remittance date before submitting the Claim to the local Plan if the charges have still not been considered by the Member's Blue Plan.

To avoid the submission of duplicate Claims, use the 276/277 healthcare Claims status inquiries to verify Claim and adjudication status prior to resubmission of electronic Claims.

The Claims Providers and Facilities submitted to the Medicare intermediary will be crossed over to the Blue Plan only after they have been processed by the Medicare intermediary. This process may take approximately 14 days to occur. This means that the Medicare intermediary will be releasing the Claim to the Blue Plan for processing about the same time the Provider or Facility receives the Medicare remittance advice. As a result, upon receipt of the remittance advice from Medicare, it may take up to thirty (30) additional calendar days for Provider or Facilities to receive payment or instructions from the Blue Plan.

Providers and Facilities should continue to submit services that are covered by Medicare directly to Medicare. Even if Medicare may exhaust or has exhausted, continue to submit Claims to Medicare to allow for the crossover process to occur and for the Member's benefit policy to be applied.

Medicare primary Claims, including those with Medicare exhaust services, that have crossed over and are received within thirty (30) calendar days of the Medicare remittance date or with no Medicare remittance date, will be rejected by the local Plan.

Anthem will reject Medicare primary provider-submitted Claims with the following conditions:

- Medicare remittance advice remark codes MA18 or N89 that Medicare crossover has occurred
    - MA18 Alert: The Claim information is also being forwarded to the patient's supplemental insurer. Send any questions regarding supplemental benefits to them.
    - N89 Alert: Payment information for this Claim has been forwarded to more than one other payer, but format limitations permit only one of the secondary payers to be identified in this remittance advice.
- Received by Provider or Facility's local Plan within thirty (30) calendar days of Medicare remittance date
- Received by Provider or Facility's local Plan with no Medicare remittance date
- Received with GY modifier on some lines but not all
    - A GY modifier is used by Providers and outpatient Facilities when billing to indicate that an item or service is statutorily excluded and is not covered by Medicare. Examples of statutorily excluded services include hearing aids and home infusion therapy.

When these types of Claims are rejected, Anthem will also remind the Provider or Facility to allow thirty (30) days for the crossover process to occur or instruct the Provider or Facility to submit the Claim with only GY modifier service lines indicating the Claim only contains statutorily excluded services.

Medicare statutorily excluded services – just file once to the local Plan

There are certain types of services that Medicare never or seldom covers, but a secondary payer, such as Anthem, may cover all or a portion of those services. These are statutorily excluded services. For services that Medicare does not allow, such as home infusion, Providers and outpatient Facilities need only file statutorily excluded services directly to their local Plan using the GY modifier and will no longer have to submit to Medicare for consideration. These services must be billed with only statutorily excluded services on the Claim and will not be accepted with some lines containing the GY modifier and some lines without.

For Claims submitted directly to Medicare with a crossover arrangement where Medicare makes no allowance, Providers and Facilities can expect the Member's benefit plan to reject the Claim, advising the Provider or Facility to submit to their local Plan when the services rendered are considered eligible for benefit. These Claims should be resubmitted as a fresh Claim to a Provider or Facility's local Plan with the Explanation of Medicare Benefits (EOMB) to take advantage of Provider or Facility contracts. Since the services are not statutorily excluded as defined by CMS, no GY modifier is required. However, the submission of the Medicare EOMB is required. This will help ensure the Claims process is consistent with the Provider's or Facility's contractual agreement.

- Providers or outpatient Facilities who render statutorily excluded services should indicate these services by using the GY modifier at the service line level of the Claim.
- Providers or Facilities will be required to submit only statutorily excluded service lines on a Claim (cannot combine with other services like Medicare exhaust services or other Medicare-covered services)
- The Provider or outpatient Facility's local Plan will not require Medicare EOMB for statutorily excluded services submitted with a GY Modifier.

If Providers or outpatient Facilities submit combined line Claims (some lines with GY, some without) to their local Plan, the Provider or outpatient Facility's local Plan will deny the Claims, instructing the Provider or outpatient Facility to split the Claim and resubmit.

Original Medicare – The GY modifier *should* be used when service is being rendered to a Medicare primary Member for statutorily excluded service, and the Member has Blue secondary coverage, such as an Anthem Medicare Supplement plan. The value in the SBR01 field should not be "P" to denote primary.

Medicare Advantage – Ensure SBR01 denotes "P" for primary payer within the 837 electronic Claim file. This helps ensure accurate processing of Claims submitted with a GY modifier.

The GY modifier *should* not be used when submitting:

- Federal Employee Program Claims
- Inpatient institutional Claims. Use the appropriate condition code to denote statutorily excluded services.

These processes align Blue Cross and Blue Shield plans with industry standards and will result in less administrative work, accurate payments, and fewer rejected Claims. Because the Claim will process with a consistent application of pricing, Members will also see a decrease in health care costs as the new crossover process eliminates or reduces balance billing to the Member.

Medicare Crossover Claims FAQs

1. **How do Providers and Facilities handle traditional Medicare-related Claims?**
    - When Medicare is the primary payer, submit Claims to the local Medicare intermediary.
    - All Blue Claims are set up to automatically cross over (or forward) to the Member's Blue Plan after being adjudicated by the Medicare intermediary.
2. **How do Providers and Facilities submit Medicare primary/Blue Plan secondary Claims?**
    - For Members with Medicare primary coverage and Blue Plan secondary coverage, submit Claims to the Medicare intermediary and/or Medicare carrier.
    - When submitting the Claim, it is essential that Provider and Facilities enter the correct Blue Plan name as the secondary carrier. This may be different from the local Blue Plan. Check the Member's ID card for additional verification.
    - Be certain to include the three-character prefix as part of the Member identification number. The Member's ID will include the three-character prefix in the first three positions. The three-character prefix is critical for confirming membership and coverage, and key to facilitating prompt payments.

      When Providers and Facilities receive the remittance advice from the Medicare intermediary, look to see if the Claim has been automatically forwarded (crossed over) to the Blue Plan.
    - If the remittance advice indicates that the Claim was crossed over, Medicare has forwarded the Claim on behalf of the Provider or Facility to the appropriate Blue Plan, and the Claim is in process. DO NOT resubmit that Claim to Anthem; duplicate Claims will result in processing and payment delays.
    - If the remittance advice indicates that the Claim was not crossed over, submit the Claim to the local Anthem Plan with the Medicare remittance advice.
    - In some cases, the Member identification card may contain a COBA ID number. If so, be certain to include that number on the Claim.
    - For Claim status inquiries, contact the local Anthem Plan.

3. **Who do Providers and Facilities contact with Claims questions?**

   The local Anthem Plan.

4. **How do Providers and Facilities handle calls from Members and others with Claims questions?**

   - If a Member contacts a Provider or Facility, tell them to contact their Blue Plan. Refer them to the front or back of their ID card for a customer service number.
   - A Member's Blue Plan should not contact Providers or Facilities directly, unless a paper Claim was filed directly with that Blue Plan. If the Member's Blue Plan contacts the Provider or Facility to send another copy of the Member's Claim, refer the Blue Plan to the local Anthem Plan.

5. **Where can Providers and Facilities find more information?**

   For more information, contact the local Anthem Plan.

Claim Payment Disputes

Provider and Facility Claim Payment Dispute Process

If a Provider or Facility disagrees with the outcome of a Claim, the Provider or Facility may begin the Anthem Claim Payment Dispute process. The simplest way to define a Claim Payment Dispute is when the Claim is finalized, but a Provider or Facility disagrees with the outcome. Providers and Facilities must complete the Claim Payment Reconsideration and Claim Payment Appeal processes set forth in this Provider Manual before they can initiate the dispute resolution and arbitration process set forth in your Provider or Facility Agreement.

A Claim Payment Dispute may be submitted for multiple reasons, including:

- Contractual payment issues
- Disagreements over reduced or zero-paid Claims
- Claim code editing issues
- Duplicate Claim issues
- Retro-eligibility issues
- Claim data issues
- Claims that are denied for no authorization when an authorization was obtained, a Claim Payment Dispute may be submitted as long as the authorized services match the Claim details.
- Timely filing issues*
- Disputes of prepayment itemized bill review findings

*Anthem will consider reimbursement of a Claim that has been denied due to failure to meet the timely filing requirements if the Provider or Facility can: 1) provide documentation that the Claim was submitted within the timely filing requirements, or 2) demonstrate good cause exists. See “Timely Filing for Claims” and “Proof of Timely Filing” in the Claims Filing Tips section of the Manual for more information.

Please note: The Claim Payment Dispute process described in this section does not apply to appeals regarding a clinical decision denial, such as a utilization management authorization or a Claim that has been denied as not medically necessary or experimental/investigational. For more information on Clinical/Medical Necessity Appeals, refer to the *Clinical Appeals* section within the Provider Manual.

There are other Claim-related matters that are **not** considered Claim Payment Disputes. To avoid confusion with Claim Payment Disputes, they are defined briefly here:

- **Claim Inquiry:** A question about a Claim or Claim payment is called an inquiry. Claim Inquiries do not result in changes to Claim payments, but the outcome of the Claim Inquiry may result in the initiation of the Claim Payment Dispute. In other words, once the Provider or Facility receives the answer to the Claim Inquiry, the Provider or Facility may opt to begin the Claim Payment Dispute process.

  Providers and Facilities can Chat with Payer or send a Secure Message through the Availity Portal. If Providers or facilities are unable to utilize the Availity Portal for inquiry, they can call

the number on the back of the Member ID Card and select the Claims prompt. For further details on Secure Messaging, reference the Availity Portal section in this Manual.

- **Claim Correspondence:** Claim Correspondence is when Anthem requires more information to finalize a Claim. Typically, Anthem makes the request for this information through the Explanation of Payment (EOP), the Claim, or part of the Claim may be denied, but it is only because more information is required to process the Claim. Once the information is received, Anthem will use it to finalize the Claim.
- **Clinical/Medical Necessity Appeals:** An appeal regarding a clinical decision denial, such as an authorization or Claim that has been denied as not medically necessary, experimental/investigational. For more information on Clinical/Medical Necessity Appeals, refer to the Clinical Appeals section within the Provider Manual.

Reference the *Claims Submission Filing Tips* subsection for additional information.

The Anthem Claim Payment Dispute process consists of two (2) steps**: Claim Payment Reconsideration and Claim Payment Appeal**. Providers and Facilities will **not** be penalized for filing a Claim Payment Dispute, and no action is required by the Member.

Step 1: Claim Payment Reconsideration

The first step in the Anthem Claim Payment Dispute process is called the Claim Payment Reconsideration. It is the Provider or Facility's initial request to investigate the outcome of a finalized Claim. Anthem cannot process a Claim Payment Reconsideration without a finalized Claim on file. Most issues are resolved at the Claim Payment Reconsideration Step.

Claim Payment Reconsiderations can be submitted via phone, Availity Essentials, or in writing. Providers and Facilities have a minimum of two (2) years from the issue date of the EOP, unless otherwise required by State law (or such time period set forth in the Provider or Facility Agreement.

A determination will be made, and the initial adjudication of the Claim will either be upheld or overturned. If the Provider or Facility is satisfied with this determination, the process will end. If the Provider or Facility disagrees with the determination of the Reconsideration, they proceed with *Step 2* below and file a Claim Payment Appeal. Providers and Facilities cannot submit another Claim Payment Reconsideration request.

When submitting Claim Payment Reconsiderations, Providers and Facilities should include as much information as possible to help Anthem understand why the Provider or Facility believes the Claim was not paid as expected. If a Claim Payment Reconsideration requires clinical expertise, it will be reviewed by the appropriate Anthem clinical professionals.

If the decision results in a Claim adjustment, the payment and EOP will be sent separately.

Except in cases where the Provider or Facility presents evidence of an extenuating circumstance, Anthem will not accept Claim Payment Reconsiderations that are not submitted timely, according to the procedures set forth above. If a Provider or Facility submits a request for a Claim Payment Reconsideration more than two (2) years from the issue date of the EOP without evidence of an extenuating circumstance, the request is deemed ineligible, and requests for payment will be denied. In such cases, Providers or Facilities will not be permitted to bill Anthem, Plan, or the Covered Individual for those services for which payment was denied.

Provider and Facilities will be notified of the Claims Payment Reconsideration determination in writing or through an EOP.

Step 2: Claim Payment Appeal

A Claim Payment Appeal is the second step in the Claim Payment Dispute process. If a Provider or Facility is dissatisfied with the outcome of a Claim Payment Reconsideration determination, Providers and Facilities may submit a Claim Payment Appeal through Availity Essentials or in writing. Providers and Facilities must submit a Claim Payment Reconsideration before submitting a Claim Payment Appeal. In addition, Providers and Facilities must submit Claims Payment Appeals within ninety (90) days from the date of the determination of the Claims Payment Reconsideration.

Except in cases where the Provider or Facility presents evidence of an extenuating circumstance, Anthem will not accept Claim Payment Appeals that are not submitted timely, according to the procedures set forth above. If a Provider or Facility submits a request for a Claim Payment Appeal more than ninety (90) calendar days from the date of the Claims Payment Reconsideration determination without evidence of an extenuating circumstance, the request is deemed ineligible, and requests for payment will be denied. In such cases, Providers or Facilities will not be permitted to bill Anthem or the Covered Individual for those services for which payment was denied.

When submitting a Claim Payment Appeal, Providers and Facilities should include as much information as possible to help Anthem understand why the Provider or Facility believes the Claim Payment Reconsideration determination was in error. If a Claim Payment Appeal requires clinical expertise, it will be reviewed by appropriate Anthem clinical professionals.

Provider and Facilities will be notified of the Claims Payment Appeal determination in writing or through an EOP.

If the decision results in a Claim adjustment, the payment and EOP will be sent separately.

Required Documentation for Claims Payment Disputes

Anthem requires the following information when submitting a Claim Payment Dispute (Claim Payment Reconsideration or Claim Payment Appeal):

- The Provider or Facility position statement explaining the nature of the dispute
- Provider or Facility name, address, phone number, email, and either NPI or TIN
- The Member's name and Anthem ID number
- A listing of disputed Claims, which should include the Anthem Claim number and the date(s) of service(s)
- All supporting statements and documentation

How to Submit a Claim Payment Dispute

There are several options to file a Claim Payment Dispute:

- Online through the Availity
- Mail all required documentation, including the Claim Payment Appeal Form or the Reconsideration Form, to:

  Anthem Blue Cross and Blue Shield
  Payment Dispute Unit
  P O. Box 105557
  Atlanta, GA 30348-5557
- Call the number on the back of the Member ID Card

Clinical Appeals

Clinical appeals refer to a situation in which an authorization or Claim for a service was denied as not medically necessary or experimental/investigational. Medical necessity appeals/prior authorization appeals are different than Claim Payment Disputes and should be submitted in accordance with the Clinical appeal process.

For questions regarding non-clinical decisions, refer to the Claim Payment Dispute section. Examples of non-clinical items that fall under Claim Payment Disputes include:

- Contractual payment issues
- Disagreements over reduced or zero-paid Claims
- Claim code editing issues
- Duplicate Claim issues
- Retro-eligibility issues
- Claim data issues
- Claims that are denied for no authorization when an authorization was obtained, a Claim dispute may be submitted as long as the authorized services match the Claim details.
- Timely filing issues
- Disputes of Prepayment Itemized Bill Review Findings

Clinical Appeals

Clinical Appeals can be used if Providers or Facilities disagree with clinical decisions. Clinical Appeals are requests to change decisions based on whether services or supplies are Medically Necessary or experimental/ investigative. UM program Clinical Appeals involve certification decisions, Claims, or predetermination decisions evaluated on these bases. Clinical Appeals can be made through Availity.com using the Authorizations & Referrals application, where available, verbally, or in writing, for appeals regarding prior authorization clinical adverse decisions.

Anthem Members may designate a representative to exercise their complaint and appeal rights. When a Provider or Facility is acting on behalf of a Member as the designated representative, the complaint or appeal may be directed to Provider Customer Service, using the phone number on the back of the Member ID card. These types of issues are reviewed according to Anthem's Member Complaint and Appeal Procedures for each applicable state. Provider Customer Service will help Providers and Facilities determine what action must be taken and if a Designation of an Authorized Representative (DOR) is needed. The **Designation of an Authorized Representative (DOR) form** is available online. To access the form, go to **anthem.com**, then in the **Member Support** menu under the **Using Your Anthem Plan** section, select **Find a Form**. On the **Forms Library** webpage, select **Kentucky**. The DOR form is under the Authorization & Verification section.

Guidelines and Timeframes for Submitting Clinical Appeals

- Providers and Facilities have one hundred eighty (180) calendar days to file a clinical appeal from the date they receive notice of Anthem's initial decision.

- All standard post-service clinical appeals will be resolved within a reasonable period of time appropriate to the medical circumstances, but not later than sixty (60) calendar days from the receipt of the appeal request by Anthem.
- For clinical appeals, there are two (2) types of review: expedited and standard.
    - **Expedited Appeal:** Anthem offers an expedited appeal for decisions meeting the expedited criteria. Requests to handle a review as "expedited" are always handled as a Member appeal. Both standard and expedited appeals are reviewed by a person who did not make the initial decision. Unless the Member, on his or her own behalf, or another Provider or Facility has already filed an expedited appeal on the service at issue in the appeal, a Provider or Facility that requests an expedited appeal will be deemed to be the Member's designated representative for the limited purpose of filing the expedited appeal. As a result, the expedited appeal will be handled pursuant to the Anthem Member Appeal Procedures exclusively.

      When a request for information is received in support of the resolution of a clinical appeal, the provider is required to respond within seven days of the request or sooner, depending upon the clinical urgency of the case, in accordance with the state or federal law, statute, or regulation.
    - **Standard Appeal:** A standard appeal is available following the reconsideration, or initially, if it is formally requested.
- UM decisions are communicated in writing to the Provider or Facility and Member. These letters provide details on appeal rights and the address to use when sending additional information.

Requests for appeal of Pre-Service requests will always be handled as a Member appeal. An expedited appeal is available for cases meeting the expedited criteria. See the instructions detailed in the UM decision letter.

- Appeals should be submitted to Anthem, along with a copy of the response to the original complaint.
    - Provider or facility name, address, phone number, email, and either NPI or TIN
    - The member's name and Anthem ID number
    - Claim, authorization, or reference number and date of service
    - Specific reason(s) for disagreement with decision
    - All supporting statements and documentation (medical records, etc.)
    - A signed DOR (Designation of Representation) is needed if the Provider is appealing on behalf of the member. No DOR is required when the Provider is appealing on their own behalf.
- Send the appeal request to:

  Anthem Blue Cross and Blue Shield
  Attention: Grievances and Appeals
  P.O. Box 105568
  Atlanta, GA 30348-5568

BlueCard® Members

Appeals involving clinical decisions related to Medical Necessity, experimental/investigative, and/or Utilization Management (UM) decisions involving Precertification/Pre-authorization are the responsibility of the Blue Plan, insuring or administering benefits for non-Anthem Members (the Member's Home Plan).

Technically, the Member, not the Provider or Facility, is responsible for obtaining the necessary authorization prior to the delivery of non-inpatient admission services. Providers must obtain the necessary authorization prior to the delivery of inpatient admission services. Failure to obtain the necessary authorization may result in non-payment or penalty reduction to the Provider. Anthem understands that many Providers obtain Pre-certification/Pre-authorization or may wish to dispute these types of denials on behalf of and as a service to their patients.

- If the appeal relates to Pre-certification/Pre-authorization, the Provider or Facility may have received information directly from the Member's Home Plan regarding appeal rights and processes. Follow the directions provided by the Member's Home Plan.
- If the appeal relates to a Claim denial, and the Provider or Facility did not receive this information from the Member's Home Plan and wishes to appeal a Medical Necessity or experimental/investigational Claim denial, the local Anthem Plan is the point of contact. When a Provider or Facility expresses dissatisfaction and wishes to file an appeal as indicated in the description above, a Claim Payment Dispute should be submitted, along with attached supporting documentation, to the local Anthem Plan. Reference the Claim Payment Dispute section for further details.
- Providers submitting an appeal on behalf of the member are required to submit a member Designation of Representation (DOR) authorization form. Home Plans may have a specific DOR authorization form.

Member Quality of Care/Quality of Service Investigations

The Grievances and Appeals department develops, maintains, and implements policies and procedures for identifying, reporting, and evaluating potential quality of care/service (QOC/QOS) concerns or sentinel events involving Anthem Members. This includes cases reviewed as the result of a grievance submitted by a Member and potential quality issues (PQI) reviewed as the result of a referral received from an Anthem clinical associate. All Anthem associates who may encounter clinical care/service concerns or sentinel events are informed of these policies.

Quality of care grievances and PQIs are processed by clinical associates. Medical records and a response from the Provider and or Facility are requested. Requests for information, including medical records, must be returned by Providers and/or Facilities on or before the due date on the request letter so that a determination can be made regarding the severity of the Potential QOC/QOS concern. Failure to return or timely return the requested information may result in escalation of the issue and potential corrective action, up to and including review for termination of contract and removal from the network.

If the clinical associate determines, based on the circumstances and applicable review of records, that the matter is a non-issue with no identifiable quality concern or that the evidence suggests a known or recognized complication, the clinical associate may assign a severity level consistent with such a finding. If the circumstances and/or evidence suggest a QOC concern beyond a known or recognized complication, then the clinical associate will prepare and send a summary to the appropriate Medical Director for review.

Specialty-matched reviewers evaluate the matter, and an appropriate Medical Director makes a determination of the severity of the QOC matter. If the QOC matter was initiated by a Member, the Member is advised that a resolution was reached, but the details and outcome of the review are protected by peer review statutes and will not be provided.

The Provider and/or Facility will also receive a letter advising of the QOC/QOS determination and any associated corrective action.

Significant quality of care issues and/or failure to participate or respond to information requests may be elevated for additional review and appropriate action, including, but not limited to, referrals to the Credentialing Committee.

Providers and Facilities are contractually obligated to actively cooperate with QOC/QOS reviews/investigations.

Allegations of quality concerns regarding the care of our Members require review of relevant materials, including, but not limited to, records of member treatment and internal investigations performed by Providers and Facilities in connection with the allegations received. This information is protected by Peer Review confidentiality, which will be maintained during Anthem’s QOC review.

Corrective Action Plan (CAP)

When corrective action is required, Providers and/or Facilities will be notified of appropriate follow-up interventions which can include one or more of the following: development of a CAP from the Provider and/or Facility to address the reviewed issues of concern, Continuing Medical Education, chart reviews, on-site audits, tracking and trending, Provider and/or Facility counseling, and/or referral to

the appropriate committee for additional action. Providers and Facilities that fail to comply with requests associated with potential QOC/QOS allegations, such as the request for information for investigations, the completion of corrective action plans by the noticed deadline and/or failure to comply with the terms of a corrective action plan will be referred to the Credentialing Committee for further actions, up to and including, termination of contract and removal from the network.

Reporting

G&A leadership reports grievance and PQI rates, categories, and trends to the appropriate Quality Improvement Committee on a bi-annual basis or more often as appropriate. Quality improvement or educational opportunities are reported, and corrective measures are implemented as applicable. Results of corrective actions are reported to the Committee. The Quality Council reviews these trends annually during the process of prioritizing quality improvement activities for the subsequent year.

Reimbursement Policies and Requirements

This section includes reimbursement requirements and policies on how Anthem will reimburse Providers and Facilities for certain services unless otherwise set forth on the Rate Sheet. Anthem reserves the right to review and revise policies when necessary.

Reimbursement policies serve as a guide to assist Providers and Facilities in accurate claim submissions and to outline the basis for reimbursement if the service is covered by a Member's Anthem benefit plan.

Anthem's public provider website is the source for reimbursement policies. To locate the policies online, go to the **Anthem Provider Home page**, then under the **Claims** menu, choose **Reimbursement Policies**.

Services must meet authorization and medical necessity guidelines appropriate to the procedure and diagnosis, as well as to the Member's state of residence. Covered services do not guarantee reimbursement unless specific criteria are met.

Proper billing and submission guidelines must be followed, including using industry-standard, compliant codes on all claim submissions. Services should be billed with Current Procedure Terminology (CPT®) codes, Healthcare Common Procedure Coding System (HCPCS) codes, and/or revenue codes that indicate the services and/or procedures performed. The billed code(s) are required to be fully supported in the medical record and/or office notes.

If appropriate coding/billing guidelines or current reimbursement policies are not followed, Anthem may:

- Reject or deny the claim.
- Recover and/or recoup claim payment.

Anthem reimbursement policies are developed based on nationally accepted industry standards and coding principles. These policies may be superseded by mandates in Provider, state, federal, or CMS contracts and/or requirements. System logic or setup may prevent the loading of policies into the claims platforms in the same manner as described; however, Anthem strives to minimize these variations.

Reimbursement Hierarchy

Claims submitted for payment must meet all aspects of the criteria for reimbursement. The reimbursement hierarchy is the order of payment conditions that must be met for a claim to be reimbursed. Conditions of payment could include benefits coverage, medical necessity/clinical criteria, authorization requirements, and/or stipulations within a reimbursement policy. Neither payment rates nor methodology is considered to be a condition of payments.

Review Schedule and Updates to Reimbursement Policies

Reimbursement policies undergo reviews for updates to state contracts, federal or CMS contracts, and/or requirements. Additionally, updates may be made at any time if we are notified of a mandated change or due to an Anthem business decision. We reserve the right to review and revise our policies when necessary. When there is an update, we will publish the most current policies on our provider website.

Reimbursement Audit and Review Process

The Reimbursement Audit and Review Process is subject to the policy and processes outlined in the Audit and Review section of this Manual and includes the following claim types:

- Claims reimbursed by DRG with an outlier reimbursed at percent of billed charges
- Claims reimbursed at percent of billed charges
- Claims reimbursed partly or fully based on a percent of billed charges

Our vendor-partner(s) or our internal team may review these claims as part of our itemized bill review (IBR) program or hospital bill audit (HBA) program to ensure appropriate reimbursement. Upon completion of the review, documentation, including a summary of adjusted charges, will be provided for each claim. Disputes related to the review may be submitted according to the instructions in the documentation supplied upon completion of the review.

In addition to the services listed below, please refer to all other service-specific sections, which may have more stringent guidelines. There may be multiple sections that apply to any given reimbursable service.

Anthem Rate

The Anthem Rate, including Cost Share amounts, for Facility Covered Services provided to Covered Individuals shall be as set forth on the Rate Sheet, subject to the following. The Commercial Outpatient Prospective Payment System (COPPS) reimbursement logic shall prevail if there are conflicts between any of these provisions and Claims reimbursed using the COPPS payment methodology. Reimbursement for Covered Services provided in an outpatient setting shall not exceed the amount Anthem would have reimbursed had the same service been provided in an inpatient setting. Anthem reserves the right to conduct an Audit or pre-payment review of Claims to ensure coding and/or payment accuracy of Claims submitted or paid under the Agreement, which may include, but shall not be limited to, a review against applicable Policies, the provider manual, state and federal laws, and Coded Service Identifier guidelines and instructions. Such an audit or pre-payment review may result in payment adjustments.

340B Drug Pricing Program

The Facility agrees to bill all drugs purchased through the 340B Drug Pricing Program in accordance with CMS rules. This includes appending modifier TB to the drug codes as applicable. Facility further agrees that all drugs purchased under the 340B Drug Pricing Program shall be administered at the CMS designated site.

Blood, Blood Products, and Administration

Blood and blood products, such as platelets or plasma, are reimbursable. Administration of Blood or Blood Products by nursing/facility personnel is not separately reimbursable on inpatient Claims. Administration of Blood or Blood Products by nursing/facility personnel billed on outpatient Claims is separately reimbursable when submitted without observation/treatment room charges.

Charges for blood storage, transportation, processing, and preparation, such as thawing, splitting, pooling, and irradiation, are also not separately reimbursable. Lab tests such as typing, Rh, matching, etc., are separately reimbursable charges.

Changes During Admission/Continuous Outpatient Encounter

There are elements that could change during an admission/outpatient encounter. The following table shows the scenarios and the date to be used for the entire Claim.

Change	Effective Date
Member's Insurance Coverage	Admission/First day of continuous Outpatient Encounter *(subject to the Member's health benefit plan)*
Facility's Contracted Rate (other than DRG)	Admission/First day of continuous Outpatient Encounter
DRG Base Rate	Admission
DRG Grouper	Discharge
DRG Relative Weights	Discharge
CPT & HCPCS coding changes	Discharge/Last day of continuous Outpatient Encounter

Chargemaster Rate Changes

The Facility will provide written notification regarding any rate changes to Anthem at least thirty (30) days prior to any change in Facility Charges. Including but not limited to room rates, pharmacy, and supplies. The notification will supply all data necessary for Anthem to evaluate the impact of the change in rate on Claims processing for Covered Services. This data should be in a format acceptable to Anthem. Data elements included in electronic submission should be CPT/HCPCS/Revenue Codes, procedure and service description codes, charge number, current price, previous price, departmental code, effective date, and Facility tax ID. These fields are not to be considered all-inclusive. The Facility agrees that Anthem may suspend payment of Claims until such time as the Facility has provided the information required above. If the Plan suspends payment, no interest would be due.

Rate increases that are not approved by Anthem are recoverable through a refund request, a reduction in future Claims payment, or a decrease for those Inpatient and Outpatient Services which are reimbursed based on the Anthem Rate (where the Anthem Rate is determined using a percentage of Facility Charges) to offset the increase in Facility Charges. The outlier amount will be increased by the same weighted average percentage as Inpatient Charges.

Facility agrees that Charges for Covered Services rendered to Members shall not exceed Facility's regular billed Charges to persons other than Member for the same services. If multiple Charge schedules are maintained by the Facility, the schedule that generates the lowest aggregate Charge per case for a service shall be used for Members.

Facility payment levels will be reviewed on an annual basis if requested by the Facility. The following steps are to be followed:

- Contact the network manager for all notifications of rate changes.
- Proposed increases in inpatient and/or outpatient charges will require documentation necessary to verify the amount of increase being implemented within each department of the Facility.

Upon receipt of the Facility's notification of a change in their charges, Anthem's review may include, but is not limited to, the following items:

A. Facility-specific Anthem Rate versus the Anthem Rate in the market (market rate is the weighted average of Anthem Rates for a particular service area).

B. The percentage increase being implemented in the Facility's charges.

C. Contractual maximum.

D. Previously negotiated changes to the Anthem Rate.

E. The length or term of the previous agreement and proposed agreement.

F. The potential impact to premium pricing versus cost of care trends.

G. Reasoning and or documentation behind the specific rate change request.

H. Information detailing how changes are distributed between inpatient and outpatient services, and specifically within each Facility department.

Coding Updates

Coded Service Identifier(s) used to define specific rates are updated from time to time to reflect new, deleted, or replacement codes. Anthem shall use commercially reasonable efforts to update all applicable Coded Service Identifiers within sixty (60) days of release by CMS or other applicable authority. If an update is delayed beyond sixty (60) days, Anthem shall notify the Facility. Any such replacement or new code will be reimbursed at the applicable Anthem Rate as set forth on the Rate Sheet that was in effect prior to the coding update, and Claims processed prior to the implementation of the revised codes shall not be reprocessed.

- CMS Maternity and Newborn DRG Changes.

  With respect to specific changes to the CMS Maternity and Newborn DRGs as published in the Federal Register, the parties agree that their intent hereunder is to keep the economic benefits to each party under the Agreement the same.

  If any such CMS Maternity and Newborn DRG change has a Material Financial Impact (Material Financial Impact" shall mean a change equal to or greater than one half of one percent (0.5%) of the total amount paid by Plan to Facility for Inpatient Services for all commercial lines of business, excluding governmental programs, including but not limited to Medicare and Medicaid), the parties agree to work together in good faith to reach an agreement on new rates such that the agreed upon rate adjustments have the effect of neutralizing any change made to the Maternity and Newborn DRGs. The rates should first be adjusted for any applicable contract year inflator, then shall be neutralized for any additional positive or negative impact of any change in the Maternity and Newborn DRGs.

  If the parties cannot agree that the CMS Maternity and Newborn DRG change(s) have a Material Financial Impact, then the parties may select a mutually agreed upon independent accounting firm or consultant, with experience and expertise in health care analytics (the "Reviewer) to generate a report regarding the auditing party's analysis. The Reviewer shall determine the rate adjustments, if any, to be made under this review process. The report of the Reviewer shall be final and binding upon the parties. For purposes of this provision, CMS Maternity and Newborn DRGs shall include Major Diagnostic Categories (MDC) 14 and 15, or their successor categories set forth by CMS.

- Non-DRG Code Implementation Terms.

  Unless otherwise required by law or applicable regulation, any updates to the Anthem Rates (excluding DRG as DRG Related Changes are described in above paragraph (a), and OP Grouper changes as OP grouper changes are described below in paragraph (c)) tied to any governmental agency, vendor, or other entity shall be effective no later than sixty (60) days after Anthem's receipt of such rate changes. Examples include, but are not limited to, CMS state-specific fee schedules and pharmaceutical rates.

  The parties recognize and agree that Anthem payment systems cannot be modified immediately when the above changes are made, but must instead be modified during a timeframe of sufficient length to allow for the loading of payment system changes and updates,

including changes and updates that allow for the recognition of new fees and codes. Anthem shall use reasonable efforts to load such payment system changes as quickly as practicable following the release date of such changes. Claims processed prior to the implementation of the new Anthem Rates shall not be reprocessed.

- Outpatient Grouper Updates.

  Each year, changes are made on a quarterly basis that may affect outpatient reimbursement under this Agreement, including but not limited to: changes to outpatient groupers, such as Anthem Outpatient Surgical Groupers and APC groupers; AMA CPT-4 code additions, deletions, and revisions; and updates to CMS APC codes and/or CMS APC weights, if applicable. The above changes shall be referred to as "Outpatient Grouper Related Changes", and the point at which Outpatient Grouper Related Changes have been made by the above-referenced organizations shall be referred to as the "Outpatient Grouper Modification Date". The parties recognize and agree that Anthem payment systems cannot be modified immediately when the above Outpatient Grouper Related Changes are made, but must instead be modified during a time-frame of sufficient length to allow for the loading of payment system changes and updates, including changes and updates that allow for the recognition of new CPT-4 or APC codes and Outpatient groupers and/or the application of new relative weights. Anthem shall use commercially reasonable efforts to load such payment system changes as quickly as is practicable following the Outpatient Grouper Modification Date. The point at which all necessary Anthem payment system changes/updates have been loaded and completed shall be referred to as the "System Modification Point". During the period between the Outpatient Grouper Modification Date of each quarter and the System Modification Point of such quarter (the "Update Period), the parties understand and agree to the following, notwithstanding any contrary provision contained herein: Unless otherwise required by law or applicable regulation, any Claim processed by Anthem during each Update Period during the term of this Agreement which Claim has a date of service after the Outpatient Grouper Modification Date and before the System Modification Point, will be processed and paid using the grouper version in effect on the date immediately prior to such Outpatient Grouper Modification Date (the "Processing System Date), will be assigned the Outpatient group or APC code, if applicable, in effect on such Processing System Date, and will be assigned the relative weights (if relative weights are a component of any payment methodology under the terms of this Agreement) in effect on such Processing System Date, and such payment shall satisfy all Anthem payment obligations hereunder (e.g., Facility shall accept such payment as payment in full subject to the other terms of this Agreement). Claims processed during the Update Period shall not be reprocessed by Plan, and unless Claims were incorrectly reimbursed for reasons other than the implementation of a new Outpatient Grouper, Facility agrees that it shall not request an adjustment for Claims processed during the Update Period.

  Prior to the System Modification Point, any new or deleted CPT-4 codes that became effective as of the Outpatient Grouper Modification Date will not be accepted by Anthem. Facility shall make best efforts to hold any Claims containing such CPT-4 code changes until it receives written notification from Anthem that the System Modification Point has been reached. If Claims containing any such CPT-4 diagnosis and procedure code changes are filed prior to the System Modification Point, the Claims will not be accepted by Anthem, and the Facility agrees to hold rejected Claims until it receives written notification from Anthem that the System Modification Point has been reached. Once the System Modification Point has been reached, Claims shall be submitted by the Facility, and processed/administered by Anthem consistent with the Outpatient Grouper Related Changes.

Comprehensive Health Planning

Facility shall not bill Anthem or a Member for Health Services, expanded facilities, capital operating costs, or any other matter of service requiring a certificate of need approval or exemption under existing law, or similar or successor laws that may be adopted from time to time, unless such approval or exemption has been granted in writing.

Courtesy Room

“Courtesy Room” means an area in the Facility where a professional Provider is permitted by the Facility to provide Health Services to Members. Anthem will not reimburse for Courtesy Room charges separately.

Daily Supply or One-Time Charge Fees/Items

Supply fees billed daily or one time, which are unidentified and unsupported by medical records or documentation, are not reimbursable.

Different Settings Charges

If Anthem determines that Facility submits charges differently for the same service performed in a different setting, then Anthem may reimburse at the Anthem Rate for the lesser of the two (2) charges.

Eligibility and Payment

Anthem shall provide methods for identifying a Member either through an issued document or through telephonic, paper, electronic, or digital communication to Provider or Facility. The identification will include information to contact Anthem, but doesn't guarantee the individual’s eligibility at the time of rendering a Health Service. Verification of eligibility doesn't guarantee payment, and lack of identification does not disqualify an individual from being a Member. Eligibility requires more than possession or access to this identification.

Emergency Room Service Charges

The Emergency Room level reimbursement includes all monitoring, equipment, supply and time, and staff charges. Reimbursement for the use of the Emergency Room includes the use of the room and personnel employed for the examination and treatment of patients. This reimbursement does not typically include the cost of physician services. See Facility Personnel Charges and Nursing Procedures sections for additional information.

Evaluation and Management (E&M) Services

Prior to payment, Anthem may review E&M Claims to determine, in accordance with correct coding requirements and/or reimbursement policy as applicable, whether the E&M code level submitted is higher than the E&M code level supported on the Claim. If the E&M code level submitted is higher than the E&M code level supported on the Claim, then Anthem reserves the right to:

- Deny the Claim and request resubmission of the Claim with the appropriate E&M level;
- Pend the Claim and request that the Facility or Provider submit documentation supporting the E&M level billed; and/or
- Adjust reimbursement to reflect the lower E&M level supported by the Claim

Facility Charge Audit

Facility shall provide electronic submission of data to the Plan for evaluation of the Chargemaster annually, or as requested by the Plan, or no later than forty-five (45) days prior to the effective date of any Chargemaster rate changes. This data shall be in a format acceptable to the Plan. Data elements

included in electronic submission should be Coded Service Identifier(s), charge number, current price, previous price, departmental code, effective date, and facility tax identification. These fields are not to be considered all-inclusive. Anthem shall make best efforts to review the Chargemaster within thirty (30) days. After a review of the Chargemaster data the Facility will work with Anthem for any follow-up evaluation and for any applicable recoupment of funds.

Anthem may perform a maximum Charge audit for Charges incurred for an annual fiscal period to be identified by Anthem. Anthem reserves the right to perform such an audit should the Facility agree to total or partial reimbursement using a Percentage Rate reimbursement. The audit will be conducted using electronic datasets produced by the Facility, including revenue and usage (utilization), and Chargemaster data containing date parameters identified above. The foregoing data shall be produced at the Charge code level.

All Facility Charge codes and related Charge revenue will be included in the evaluation of Charge increases.

Facility Charges will be evaluated for the total entity, combining inpatient and outpatient Charge activity. Charges will be combined for total inpatient and outpatient volume regardless of other scheduled pricing mechanisms in force, such as, but not limited to DRG, Ambulatory Patient Group (APG), Ambulatory Patient Classification (APC), and Fee Schedule Rate that may be active for certain groups of services.

The audit will be conducted using inpatient and outpatient utilization for Anthem Members.

Audits will be conducted using the revenue and usage (utilization) data for the fiscal period (annual), subject to audit, as opposed to using fiscal period data from any other period than the audit.

The Charge audit will be permitted to take place, and any resulting assessments should be made, should a violation be identified, even if an amendment or any other contractual change is made during or after the fiscal period of the audit.

Notwithstanding the Entire Agreement section of the Agreement, Facility represents that to the best of Facility's knowledge and belief, all documents provided by Facility to Anthem relating to Facility's Chargemaster increases prior to and upon entering into the Agreement, and Charge increase information submitted during the term of the Agreement (including, without limitation, certifications as to changes in Facility Charges from time to time), do and did contain true and correct information. Facility acknowledges that Anthem has relied on such information in good faith in negotiating the Anthem Rate throughout the term of the Agreement, whether the Agreement was amended or otherwise altered at any time during the contractual period thereof. Should Anthem learn, through the course of any audit or otherwise, that Facility Chargemaster data was not accurately represented, Anthem shall be entitled to a recalculation of the Anthem Rate and reimbursement for the difference between the Anthem Rate determined based upon inaccurate Facility represented information at the time the Anthem Rate went into effect, to the time the audit is completed.

Facility Personnel Charges

Charges for Inpatient Services for Facility personnel are not separately reimbursable, and the reimbursement for such is included in the room and board rate or procedure charge. Examples include, but are not limited to, lactation consultants, dietary consultants, overtime charges, transport fees, nursing functions including IV or PICC line insertion at bedside, therapeutic, prophylactic, or diagnostic intravenous (IV) injections or infusion or IV fluid administration/monitoring, call back charges, nursing increments, therapy increments, bedside respiratory and pulmonary function services, and chemotherapy infusion/administration/monitoring. Outpatient Services for Facility

personnel are also not separately reimbursable. Reimbursement is included in the reimbursement for the procedure or observation charge.

General Industry Standard Language

Per Anthem policy and the Agreement, Provider and Facility will follow industry standards related to billing. Per the UB-04 and CMS1500 (or successor forms) billing manual, referenced as Coded Service Identifier(s).

Instrument Trays

Charges for instrument trays for any procedure are included in the cost of the procedure and are not separately reimbursable. See Operating Room Time and Procedure Charges and Routine Supplies sections for additional information.

Interim Bill Claims

Anthem shall not adjudicate Claims submitted as interim bills for services reimbursed under a DRG methodology.

IV Sedation Local Anesthesia

Charges for IV Sedation and local anesthesia administered by the provider performing the procedure, and/or nursing personnel, are not separately reimbursable and are included as part of the Operating Room (OR) time/procedure reimbursement. Charges for medications-drugs used for sedation and local anesthesia are separately reimbursable.

Lab Charges

The reimbursement of charges for specimen collection is considered Facility personnel charges, and the reimbursement is included in the room and board or procedure/observation charges. Examples include venipuncture, urine/sputum specimen collection, draw fees, phlebotomy, heel sticks, and central line draws.

Processing, handling, and referral fees are considered included in the procedure/lab test performed and are not separately reimbursable.

Labor Care Charges

Anthem will reimburse appropriately billed room and board or labor charges. Payment will not be made on both charges billed concurrently. Facilities reimbursed under DRG will not be reimbursed by Anthem for Outpatient Services rendered prior to the admission.

Medical Care Provided to or by Family Members

Services for any type of medical care rendered by a Provider to him/herself or to an immediate family member (as defined below), who is a Member, are not eligible for coverage and should not be billed to Anthem. In addition, a Provider may not be selected as a Primary Care Physician (PCP) by their immediate family member.

Unless otherwise set forth in a Member's Health Benefit Plan, an immediate family member includes: father, mother, children, spouse, domestic partner, legal guardian, grandparent, grandchild, sibling, step-father, step-mother, step-children, step-grandparent, step-grandchild, and/or step-sibling.

Mother/Baby Claims

Facilities will submit, and Anthem will reimburse maternity Claims as two (2) separate and individual Claims according to the specified compensation schedule. Claims submitted for multiple births will be reimbursed separately for each birth.

Neuromonitoring

Anthem considers neuromonitoring services performed in an operating room setting to be included in the surgical procedure reimbursement.

Therefore, Claims submitted by anyone other than the rendering facility will not be eligible for separate or additional reimbursement. If the rendering facility utilizes a neuromonitoring vendor to perform any services, then it is the rendering facility's responsibility to reimburse the vendor directly. Any Claims submitted to Anthem for these additional services will be denied as they will be considered part of the all-inclusive facility reimbursement.

New High-Cost Therapeutics

New High-Cost Therapeutics (defined below) are not reimbursable.

Notwithstanding the foregoing, Facility may submit the following documentation to Anthem at least sixty (60) days prior to the implementation of any New High-Cost Therapeutic for consideration as a reimbursable service: (1) a description of the New High-Cost Therapeutic; (2) Facility's proposed charge for the New High-Cost Therapeutic and; (3) such other reasonable data and information required by Anthem to evaluate the New High-Cost Therapeutic. If Anthem agrees that a New High-Cost Therapeutic shall be a reimbursable service, then the reimbursement for such New High-Cost Therapeutic is set forth below in paragraph (a). The parties further agree that Facility shall obtain any Covered Individual consent to use and disclose the Covered Individual's personally identifiable information, including PHI if required by Anthem and/or manufacturers (i.e., pharmaceutical, biotechnology, medical device manufacturers, or other parties in the supply chain), and shall release such information pursuant to the consent, upon request to Anthem and/or manufacturers.

New High Cost Therapeutics shall be defined as Food and Drug Administration (FDA)-approved drugs, biologicals, gene and cell therapies to treat rare or serious conditions, with an annual estimated Wholesale Acquisition Cost (WAC) per patient of greater than two hundred fifty thousand dollars ($250,000). New High-Cost Therapeutics shall be subject to applicable Policies. Except for New High Cost Therapeutics that are packaged to a Case Rate or to an APC under COPPS (if COPPS is applicable to the Agreement), and notwithstanding anything to the contrary in the Agreement, the Anthem Rate for New High Cost Therapeutics shall automatically be the lesser of: i) the 340B acquisition cost plus one percent (1%) if Facility is an eligible 340B entity, or ii) the Wholesale Acquisition Cost (WAC) plus one percent (1%) if Facility is unable to acquire the drug or biological at the 340B acquisition cost.

New/Expanded Service or New/Expanded Technology

New/Expanded Service or New/Expanded Technology (defined below) is not reimbursable.

Notwithstanding the foregoing, Facility may submit the following documentation to Anthem at least sixty (60) days prior to the implementation of any New/Expanded service or New/Expanded technology for consideration as a reimbursable service: (1) a description of the New/Expanded Service or New/Expanded Technology; (2) Facility's proposed charge for the New/Expanded Service or New/Expanded Technology, and (3) such other reasonable data and information required by Anthem to evaluate the New/Expanded Service or New/Expanded Technology. If Anthem agrees that the New/Expanded Service or New/Expanded Technology may be considered a reimbursable service, then Anthem shall notify Facility, and both parties agree to negotiate in good faith a new Anthem Rate for the New/Expanded Service or New/Expanded Technology within sixty (60) days of Anthem's notice to Facility. If the parties are unable to reach an agreement on a new Anthem Rate for the New/Expanded Service or New/Expanded Technology before the end of the sixty (60) day period, then such New/Expanded Service or New/Expanded Technology shall not be reimbursed by Anthem,

and the Payment in Full and Hold Harmless provision of the Agreement shall apply. The COPPS reimbursement logic shall prevail if there are conflicts between this provision and Claims reimbursed using the COPPS payment methodology. For the avoidance of doubt, New/Expanded Service or New/Expanded Technology shall not include New High Cost Therapeutics as set forth in this provider manual.

- New /Expanded Service shall be defined as a Health Service: (a) that Facility was not providing to Covered Individuals as of the Effective Date of this Agreement, and (b) for which there is not a specific Anthem Rate as set forth on the Rate Sheet.
- "New/Expanded Technology" shall be defined as a technological advancement in the delivery of a Covered Service which results in a material increase to the cost of such service. New/ Expanded Technology shall not include a new device or implant that merely represents a new model or an improved model of a device or implant used in connection with a service provided by Facility as of the Effective Date of the Agreement.

Non-Covered Services, Supplies, or Treatment

Reimbursement shall not be made for Claims submitted for services, supplies, or treatment related to, or for complications directly related to, a service that is not covered by the Plan. Directly related means that the care took place as a direct result of the non-Covered Service and would not have taken place without the non-Covered Service.

Nursing Procedures

Anthem will not separately reimburse fees associated with nursing procedures or services provided by Facility nursing staff or unlicensed Facility personnel (technicians) performed during an inpatient (IP) admission or outpatient (OP) visit. Examples include, but are not limited, to therapeutic, prophylactic, or diagnostic intravenous (IV) injections or infusion or IV fluid administration/monitoring, intramuscular (IM) injections, subcutaneous (SQ) injections, nasogastric tube (NGT) insertion, urinary catheter insertion, point of care/bedside testing (such as glucose, blood count, arterial blood gas, clotting time, pulse oximetry, etc.) and inpatient blood transfusion administration/monitoring and inpatient chemotherapy infusion/administration/monitoring (with the exception of OP blood administration, OP chemotherapy administration, or OP infusion administration which are submitted without a room charge, observation charges, or procedure charges other than blood, chemotherapy, or infusion administration).

Observation Room/Treatment Room Charges

Anthem considers observation room charges up to twenty-four (24) hours of observation. Observation room allowable charges for Covered Services will be combined and limited to the maximum of the average approved semi-private room rate, discounted by the Facility's negotiated outpatient percent of charge.

Facility shall not bill Anthem and/or Members, and Anthem shall not reimburse Facility for the use or accommodation of treatment rooms for the provision of Health Services to a Member.

Observation Services, Proper Billing of

Facility shall report observation charges under Revenue Code 762 with the appropriate CPT Codes of 99217 through 99220 and 99234 through 99236. Facility shall report the number of hours the patient is in observation status in the units field. Facility shall report any additional services performed while the patient is in observation status under the appropriate Revenue Code for those services.

Operating Room Time and Procedure Charges

The operating room (OR) charge will be reimbursed on a time or procedural basis. When time is the basis for the charge, it should be calculated from the time the patient enters the room until the patient leaves the room, as documented on the O.R. nurse's notes. The Operating Room is defined as surgical suites, major and minor, endoscopy labs, cardiac catheterization (cath) labs, Hybrid Rooms, X-ray, pulmonary, and cardiology procedural rooms. The operating room reimbursement will reflect the cost of:

- The use of the operating room
- The services of qualified professional and technical personnel
- Any supplies, items, equipment, and services that are necessary or otherwise integral to the provision of a specific service and/or the delivery of services. Refer to the *Routine Supplies* section of the manual.

The operating room charge will not reflect the cost of robotic technology and is not eligible for separate reimbursement. Examples of charges that are **not** eligible for separate or additional reimbursement are listed below.

- Increased operating room unit cost charges for the use of the robotic technology
- Charges billed under CPT or HCPCS codes that are specific to robotic-assisted surgery, including, but not limited to, S2900.
- Supplies billed related to the use of robotic technology.
- Reference the **Technology Assisted Surgical Procedures** Reimbursement Policy.

Other Agreements

If Facility currently maintains a separate Agreement(s) with Anthem solely for the provision and payment of home health care services, skilled nursing facility services, ambulatory surgical facility services, or other agreements that Anthem designates (hereinafter collectively "Other Agreement(s)"), then such Other Agreement(s) will remain in effect and control the provision and payment of Covered Services rendered there under.

Personal Care Items

Personal care items used for patient convenience are not reimbursable. Examples include but are not limited to: breast pumps, deodorant, dry bath, dry shampoo, eye lubricants, lotion, non-medical personnel, mouthwash, powder, soap, telephone calls, television, tissues, toothbrush, and toothpaste.

Pharmacy Charges

Pharmacy charges will be reimbursed to include only the cost of the drugs prescribed by the attending physician. Medications furnished to patients shall not include an additional separate charge for administration of drugs, the cost of materials necessary for the preparation and administration of drugs, and the services rendered by registered pharmacists and other pharmacy personnel. Anthem will reimburse at the Anthem Rate for the drug. All other services are included in the Anthem Rate. Examples of pharmacy charges which are not separately reimbursable include, but are not limited to: IV mixture fees, IV diluents such as saline and sterile water, IV Piggyback (IVPB), Heparin and saline flushes to administer IV drugs, and facility staff checking the pharmacy (Rx) cart.

Portable Charges

Portable charges are included in the reimbursement for the procedure, test, or x-ray and are not separately reimbursable.

Pre-Operative Care or Holding Room Charges

Charges for pre-operative care or a holding room used prior to a procedure are included in the reimbursement for the procedure and are not separately reimbursed. In addition, nursing care provided in the pre-operative care area will not be reimbursed separately. Reimbursement for the procedure includes all nursing care provided.

Preparation (Set-Up) Charges

Charges for set-up, equipment, or materials in preparation for procedures or tests are included in the reimbursement for that particular procedure or test.

Provider and Facility Records

Provider and Facility shall prepare and maintain all appropriate medical, financial, administrative and other records as may be needed for Members receiving Health Services. All of the Provider's and Facility's records on Members shall be maintained in accordance with prudent record-keeping procedures and as required by any applicable federal, state, or local laws, rules, or regulations.

Recovery Room Charges

Reimbursement for recovery room services (time or flat fee) includes all used and or available services, equipment, monitoring, and nursing care that is necessary for the patient's welfare and safety during their confinement. This will include, but is not limited to, cardiac monitoring, Dinamap®, pulse oximeter, injection fees, nursing, nursing time, nursing supervision, equipment and supplies (whether disposable or reusable), defibrillator, and oxygen.
Separate reimbursement for these services will not be made.

Recovery Room Services Related to IV Sedation and/or Local Anesthesia

Anthem will not provide reimbursement for a phase I or primary recovery room charged in connection with IV sedation or local anesthesia. Charges will be paid only if billed as a post-procedure room or a phase II recovery (step-down) (e.g., arteriograms). The Anthem Rate shall not exceed the Facility's approved average semi-private room and board rate, less discount as submitted to Anthem.

Reimbursement Methodologies

If reimbursement methodologies are outlined differently in the Rate Sheet or Plan Compensation Schedule, they will supersede those that are outlined below.

<u>Inpatient and Outpatient Services</u>

Anthem Rate: The Anthem Rate, including Cost Share amounts, for Facility Covered Services provided to Members shall be as follows:

I. For Claims paid according to Per Diem Rates or Per Visit Rates, unless a different or additional payment as set forth in the Rate Sheet, Facility shall be paid the lesser of (i) the Anthem Rate(s) as set forth on the Rate Sheet, multiplied by the number of Patient Days or (ii) Facility's total eligible billed Charges for Covered Services for the entire admission or visit;

II. For Claims paid according to Case Rates, Global Case Rates, and/or DRG methodology unless a different or additional payment as set forth in the Rate Sheet, Facility shall be paid the lesser

of (i) the Anthem Rate(s) as set forth on the Rate Sheet or (ii) Facility's total eligible billed Charges for Covered Services for the entire admission or visit;

III. For Claims paid according to a Fee Schedule Rate or Per Unit Rate, Facility shall be paid the lesser of (i) the Anthem Rate as set forth on the Rate Sheet or (ii) Facility's total eligible billed Charges for the Covered Services as set forth on the fee schedule, which shall be calculated on a line-by-line basis. See example below:

Fee Schedule Rate Example:

Drug	Facility's Billed Charge	Anthem Fee Schedule	Anthem Allowed
#1	$25	$50	$25
#2	$30	$20	$20
		TOTAL ALLOWED	$45

Standard Covered Services Reimbursement Hierarchy

Covered Services that are reimbursed under a Case Rate, Per Diem Rate, Per Visit Rate, or similar fixed reimbursement methodology shall always take precedence over non-fixed reimbursement methodologies and are considered all-inclusive except as otherwise set forth on the Rate Sheet. Per Unit Rate or Fee Schedule Rate pricing will not be reimbursable if billed in conjunction with other Covered Services reimbursed at a Case Rate, Per Diem Rate, Per Visit Rate, or similar fixed reimbursement methodology, except as otherwise set forth on the Rate Sheet.

When a procedure qualifies for more than one fixed reimbursement methodology, such as a Case Rate and grouper-level rate, the reimbursement will default to the Case Rate methodology or as otherwise set forth on the Rate Sheet.

When a Claim has multiple procedures that qualify for more than one fixed reimbursement methodology, such as multiple Case Rates and/or grouper-level rates, the reimbursement will default to the highest reimbursed fixed methodology, such as Case Rate or grouper-level, as set forth on the Rate Sheet.

Case Rate Methodology

A Per Case Rate is an all-inclusive level of reimbursement for a specific inpatient or outpatient Covered Service, as specified by DRG, CPT/HCPCS/Revenue Code, as set forth on the Rate Sheet. Unless otherwise specified, no additional reimbursement shall be made for any covered service(s) submitted on the same Claim as a covered service reimbursed under the Per Case Rate methodology.

Exception: If a Pass Through/Per Case Rate has been negotiated for the following outpatient diagnostic services, such services shall be reimbursed at the Pass Through/Per Case Rate as set forth on the Rate Sheet, in addition to any other outpatient reimbursable Covered Service on the Claim:

- MRI
- CT Scan
- Mammogram
- PET Scan

Emergency Room/Urgent Care Claims/Revenue Codes 450, 456, and 459

If a Case Rate has been negotiated for Emergency Room (ER), as set forth on the Rate Sheet, Anthem shall reimburse the Facility the ER Case Rate for the entire Claim unless noted as an exception in the Combination Emergency Room/Observation Services/Outpatient Surgery section.

The Emergency Room Case Rate includes, but is not limited to, all monitoring, equipment, supplies, time, and staff charges.

ER per Case Rate will be applied to Claims under all 45X Revenue Codes. In the absence of CPT Codes 99281-99285, the Per Case Rate for 99281 will be applied, unless the CPT Code for the service provided is specifically negotiated under a separate Per Case Rate (i.e., critical care codes 99291 and 99292).

Exception: If a Pass Through/Per Case Rate has been negotiated for the following outpatient diagnostic services, such services shall be reimbursed at the Pass Through/Per Case Rate as set forth on the Rate Sheet, in addition to any other outpatient reimbursable Covered Service on the Claim:

- MRI
- CT Scan
- Mammogram
- PET Scan

Combination Emergency Room/Observation Services

The following provisions are applicable unless otherwise set forth on the rate sheet.

If a Member is admitted or transferred within a hospital system, operating under the same Facility Agreement, management, and/or tax identification number, through the Emergency Room (ER), Observation (OBS), or following Surgery (SURG), the Plan shall reimburse the Facility according to the diagnosis and Anthem Rate associated with the Inpatient Services. Anthem shall not reimburse the Facility separately for the ER, OBS, and/or SURG.

If a Pass Through Rate/Per Unit Rate, as set forth on the Rate Sheet, for such services as MRI, CT, or Mammogram, then such rate will be reimbursed in addition to any other service or combination outlined below.

If ER, OBS, and/or SURG services are provided during a single outpatient encounter, and if ER, OBS, and/or SURG services are individually reimbursed as a Case Rate as specified on the Rate Sheet, Anthem shall reimburse the Facility as follows:

- ER/OBS: If a Case Rate has been negotiated for ER only, as set forth on the Rate Sheet, Anthem shall reimburse the Facility the ER Case Rate for the entire Claim.
- ER/OBS: If a Case Rate has been negotiated for OBS only, as set forth on the Rate Sheet, Anthem shall reimburse the Facility the OBS Case Rate for the entire Claim.
- ER/OBS: If a Case Rate has been negotiated for both ER and OBS, as set forth on the Rate Sheet, Anthem shall reimburse the Facility the highest Case Rate for the entire Claim.
- ER/SURG: If a Case Rate has been negotiated for surgery, as set forth on the Rate Sheet, Anthem shall reimburse the Facility the SURG Case Rate for the entire Claim.

- SURG/OBS: If a Case Rate has been negotiated for surgery, as set forth on the Rate Sheet, Anthem shall reimburse the Facility the SURG Case Rate for the entire Claim.
- SURG: If a Case Rate has been negotiated for surgery, as set forth on the Rate Sheet, Anthem shall reimburse the Facility the SURG Case rate for the entire Claim.

Follow-up Care

Initial emergency room or urgent care includes full reimbursement for the subsequent follow-up care by Facility employees or subcontractors in the emergency room or urgent care. Plan shall not make any additional payment to the Facility, and Facility shall not seek any payment from the Member in relation to follow-up care, including any payment which would otherwise be due for the emergency room or urgent care visit (e.g., stitch removal, wound care, cast care, or any procedure with a CPT coding guideline for a global period).

Surgery Claims Under Per Case Rate Reimbursement

Claims that include surgical CPT Codes reimbursed under a Per Case Rate shall be reimbursed based on the highest reimbursed CPT Code for which a Per Case Rate is negotiated. No additional reimbursement for multiple surgeries. The assignment of surgical CPT codes into the reimbursement levels is based on CMS's ambulatory surgical groupings or Anthem's ambulatory surgical groupings. If the Claim does not contain a surgical CPT code that is assigned to one of the levels, reimbursement will be based on the outpatient other rate or at a Per Case Rate as specified on the Rate Sheet.

DRG Methodology

For inpatient admissions on or after the Effective Date of the Agreement for which the total of the Facility Charges for Covered Services is less than or equal to the DRG Outlier amount specified on the Rate Sheet, Anthem shall reimburse the Facility the DRG Rates specified in effect on the date of inpatient admission and specified on the Rate Sheet, multiplied by the weight for the assigned DRG in effect on the date of discharge, using the relative weights as established (and amended from time to time) by CMS.

For inpatient admissions on or after the Effective Date of the Agreement for which the total of the Facility Charges for Covered Services is greater than the DRG Outlier amount specified on the Rate Sheet, Anthem shall reimburse the Facility the DRG Outlier Rate percentage as specified on the Rate Sheet.

For inpatient admissions for Members who are transferred out of the Facility to another facility for purposes of providing Inpatient Covered Services, Anthem shall reimburse the Facility the lesser of the DRG Rate or the Outlier Rate as specified on the Rate Sheet. For Members who are transferred within a hospital system, operating under the same Facility Agreement, management and/or tax identification number, for inpatient services, excluding SNF, MH/SA, and/or Hospice, the Facility agrees to bill all services under one Claim.

For those Facilities reimbursed on a methodology other than DRG, for Inpatient Services and Outpatient Services rendered on or after the Effective Date of the Agreement, the applicable Anthem Rate that is in effect on the date of admission is pursuant to the Rate Sheet effective on the date of admission. The Outlier Rate will not apply to services reimbursed in accordance with a methodology other than DRG, unless specifically identified on the Rate Sheet. The Outlier Rate will not apply to coronary services network services reimbursed on a Per Case Rate basis.

Fixed Outpatient Reimbursement Methodology

Fixed Outpatient Reimbursement methodology provides Anthem with the ability to reimburse outpatient Facility Claims utilizing the CPT/HCPCS Codes currently being reported on Claims.

Revenue Codes will require a companion CPT/HCPCS Code when the Facility's Agreement includes Fixed Outpatient Reimbursement.

Outlier Reimbursement

The Outlier Rate percentage will not apply to services reimbursed as a Per Diem Rate, Per Case Rate, or Percentage Rate, unless specifically identified on the rate sheet. The change in outlier threshold will be determined based on the impact of the rate increases for Inpatient Services on the change in Facility Charges, per the following example.

> If the outlier threshold was $70,000, and inpatient service Charges were increased five percent (5%), the new outlier threshold would be $73,500 ($70,000x1.05 = $73,500).
>
> Standard rounding procedures to the nearest whole dollar will be used.

Outpatient Facility Fee Schedules

The Outpatient Facility Fee Schedule methodology provides Anthem with the ability to reimburse specified Outpatient Services based on a fee schedule utilizing the CPT/HCPCS Code.

Outpatient Laboratory, Radiology, and Diagnostic Services

Outpatient laboratory, radiology, and diagnostic services (not included elsewhere) will be reimbursed based upon the Anthem Rate that includes current Coded Service Identifier(s) for services, including, but not limited to, clinical laboratory, pathology, radiology, and other diagnostic tests. The Anthem Rate includes payment for all services rendered in relation to an outpatient visit for laboratory, radiology, or other diagnostic services, including, but not limited to: facility use, equipment, professional services (if applicable), specified supplies, and all other services incidental to the outpatient visit.

Outpatient Pass Through Rate

Outpatient Pass Through Rate is a fixed rate reimbursed in addition to other outpatient fixed rates for those Members who meet Anthem's criteria. Any exceptions noted to a Pass Through Rate shall be set forth on the Rate Sheet.

Outpatient Surgery

For Outpatient Surgery rendered, reimbursement shall be as set forth on the Rate Sheet. Codes for procedures that are incidental to other procedures performed on the same day will not be eligible for separate reimbursement.

Revenue Codes Requiring Additional Coded Service Identifiers

The following is a current listing of services and Revenue Codes that will require a CPT/HCPCS Code when the Facility's reimbursement section of the Agreement includes one or more Outpatient Facility Fee Schedules:

Service	Revenue Code(s)
Ambulatory Surgery Care	49X
Cardiology	48X, 73X
Drugs	25X, 63X
Gastrointestinal	75X
Laboratory	30X, 31X, 39X
Medical/Surgical Supplies and Devices	27X

Service	Revenue Code(s)
PT/OT/Speech	42X, 43X, 44X
Radiology	32X, 33X, 34X, 35X, 40X
Specialty Services	76X
Surgery	36X

Claims that include specified CPT/HCPCS Code(s) will be reimbursed in accordance with the Anthem Rate in the reimbursement section of the Agreement unless such Claim includes a service reimbursed under a Case Rate methodology. Outpatient Facility Fee Schedule services provided in conjunction with a service reimbursed under the Case Rate methodology are considered incidental to the Case Rate service and are not separately payable.

Other Outpatient Services

Anthem will reimburse other Outpatient Services not defined by the fee schedule, Per Diem, Per Case, Per Visit, or Per Unit at the other Outpatient Services percentage as specified in the reimbursement section of the Agreement.

Per Diem Rate

Per Diem Rate is an all-inclusive fixed payment for each full day of covered services or one (1) outpatient encounter. A full day begins at midnight and ends twenty (24) hours later. The day of admission counts as a full day. The day of discharge, death, or transfer shall be excluded. If admission and discharge, death, or transfer occur on the same day, the day is counted as one (1) inpatient full day. Outpatient surgery reimbursed at a Per Diem Rate will apply multiple surgery logic. Any exceptions noted to a Per Diem Rate shall be as set forth on the Rate Sheet.

The subacute Per Diem Rate as set forth on the Rate Sheet shall be used to reimburse those Members who meet Anthem's criteria for subacute admissions, regardless of bed type.

Per Unit Rate

The flat rate payment made to a Participating Provider for all Covered Services rendered to a Covered Person for each unit of service performed within an Outpatient Service Category as indicated on the rate sheet.

Per Visit Rate

Services reimbursed at a Per Visit Rate include but are not limited to physical therapy (PT), occupational therapy (OT), speech therapy (ST), and respiratory therapy (RT). There will be one (1) Per Visit Rate reimbursed per service category, per date of service. Ancillary services reimbursed under a Per Unit or Fee Schedule Rate will be paid in addition to the Per Visit Rate. Visit Rates include, but are not limited to: facility use, therapist/professional services, laboratory, radiology, supplies, equipment, pharmaceuticals, and other services incidental to the visit. Specified Per Visit Rates for PT, OT, ST, and RT, as set forth on the Rate Sheet, shall apply to each outpatient therapy visit only when such outpatient therapy services are not included within another type of service or visit.

Respiratory Services

For percent of charge reimbursement, Mechanical Ventilation / CPAP / BIPAP support and other respiratory and pulmonary function services are considered separately reimbursable.

Routine Supplies

Supplies that are provided to most patients during an inpatient or outpatient admission are not separately reimbursable.

- Mechanical Ventilation / CPAP / BIPAP support and other respiratory and pulmonary function services provided at the bedside are considered facility personnel, equipment, and/or supply charges and not eligible for separate reimbursement.

Special Procedure Room Charge

Charges for a special procedure room, billed in addition to the procedure itself, are included in the reimbursement for the procedure. If the procedure takes place outside of the OR (refer to Operating Room Time and Procedure Charges for OR definition), then OR time will not be reimbursed to cover OR personnel/staff being present in the room. *Example:* procedures performed in the ICU, ER, etc.

Stand-by Charges

Standby equipment and consumable items, such as oxygen, which are on standby, are not reimbursable. Only actual use is covered. Staff on standby is included in the reimbursement for the procedure and is also not separately reimbursable.

Stat Charges

Stat charges are included in the reimbursement for the procedure, test, and/or x-ray. These charges are not separately reimbursable.

Submission of Claim/Encounter Data

Providers and Facilities will submit Claims and Encounter Data to Anthem in a format that is consistent with industry standards and acceptable to Anthem. Claims must be submitted using the CMS-1500, UB-04, or successor forms, according to Coded Service Identifier(s) guidelines using HIPAA-compliant codes. This submission should occur within the time frames and requirements set forth in your Provider or Facility Agreement.

A "Claim" refers to either a uniform Claim form or an electronic form prescribed by Anthem for the purpose of requesting payment for Health Services offered to a Member. Such a Claim needs to contain all the necessary information needed for processing and making a benefit determination.

"Encounter Data" means Claim information and any additional information submitted by a Provider or Facility under capitated or risk-sharing arrangements for Health Services rendered to Members.

Anthem will make best efforts to pay all complete and accurate Claims for Covered Services submitted by Facilities and Providers in accordance with your Provider or Facility Agreement, and applicable state statutes, exclusive of Claims that have been suspended due to the need to determine Medical Necessity, to the extent of Anthem’s payment liability, if any, because of issues such as coordination of benefits, subrogation or verification of coverage.

Supplies and Equipment

Charges for medical equipment, including but not limited to IV pumps, PCA Pumps, isolation carts, mechanical ventilators, continuous positive airway pressure (CPAP)/ bilevel positive airway pressure (BIPAP) machines, and related supplies, are not separately reimbursable. Oxygen charges, including but not limited to oxygen therapy per minute/per hour when billed with room types ICU/CCU/NICU or any Specialty Care area, are not separately reimbursable.

Tech Support Charges

Pharmacy Administrative Fees (including mixing medications), any portable fees for a procedure or service, and patient transportation fees when taking a patient to an area for a procedure or test are not separately reimbursable. Transporting a patient back to their room following surgery, a procedure, or a test is not separately reimbursable.

Telemetry

Telemetry charges in (ICU/CCU/NICU) or telemetry unit are included in the reimbursement for the place of service. Additional monitoring charges are not reimbursable. Separately billed telemetry charges will only be paid if observation (OBS) charges do not exceed the approved average semi-private room and board rate, less discount, as submitted to Anthem.

Test or Procedures Prior to Admission(s) or Outpatient Services

Any fixed rate reimbursement methodology (e.g., DRG Rates, Per Diem Rates, and Case Rates) for Covered Services includes reimbursement for pre-admission testing/pre-surgical testing (PAT/PST) rendered at any time (unless specifically identified on the Rate Sheet) prior to a scheduled inpatient admission or outpatient procedure.

Facility shall not bill, or receive reimbursement for PAT/PST, and shall not bill, or seek payment from, any Member related to the same. Facility acknowledges that any payment made by Anthem for PAT/PST is subject to recovery under Anthem's Policies. Notwithstanding the foregoing, if the inpatient admission or outpatient procedure is canceled, the Facility may bill and receive reimbursement for PAT/PST.

The following diagnostic services, defined by specific Coded Service Identifier(s), are considered part of pre-admission/pre-surgical/pre-operative testing:

- 254 – Drugs incident to other diagnostic services
- 255 – Drugs incident to radiology
- 30X – Laboratory
- 31X – Laboratory pathological
- 32X – Radiology diagnostic
- 341 – Nuclear medicine, diagnostic
- 35X – CT scan
- 371 – Anesthesia incident to Radiology
- 372 – Anesthesia incident to other diagnostic services
- 40X – Other imaging services
- 46X – Pulmonary function
- 471 – Audiology diagnostic
- 48X – Cardiology
- 53X – Osteopathic services
- 61X – MRI
- 62X – Medical/surgical supplies, incident to radiology or other services

- 73X – EKG/ECG
- 74X – EEG
- 918 – Testing – Behavioral Health
- 92X – Other diagnostic services

Non-diagnostic services are also considered part of pre-admission/pre-surgical/pre-operative testing if they are furnished in connection with the principal diagnosis that necessitates the outpatient procedure or the Member's admission as an inpatient.

Time Calculation

- **Operating Room (OR)** – OR time should be calculated from the time the patient enters the room until the patient leaves the room, as documented on the O.R. nurse's notes.
- **Post Recovery Room** – Time charges should be calculated from the time the patient leaves the recovery room until discharge.
- **Recovery Room** – Time should be calculated from the time the patient enters the recovery room until the patient leaves the recovery room, as documented on the post-anesthesia care unit (PACU) record.
- **Hospital/Technical Anesthesia Component** – Time should be calculated from the time the patient enters the operating room (OR) until the patient leaves the room, as documented on the OR nurse's notes. The time the anesthesiologist spends with the patient in pre-op and in the recovery room is not to be included in the hospital anesthesia time calculation.

Undocumented or Unsupported Charges

Per Anthem policy, Anthem will not reimburse charges that are not documented on medical records or supported with documentation.

Video or Digital Equipment Used in Procedures

Charges for video or digital equipment used for visual enhancement during a procedure are included in the reimbursement for the procedure and are not separately reimbursable. Examples include, but are not limited to, Ultrasound and Fluoroscopy guidance (including, but not limited to, for access and/or placement of medical devices, implants, or lines). Charges for batteries, covers, film, anti-fogger solution, tapes, etc., are also not separately reimbursable.

Additional Reimbursement Guidelines for Disallowed Charges

For any Claims that are reimbursed at a percent of charge, only Charges for Covered Services are eligible for reimbursement. The disallowed charges (charges not eligible for reimbursement) include, **but are not limited to**, the following, whether billed under the specified Revenue Code or any other Revenue Code. These Guidelines may be superseded by the specific agreement. Refer to the contractual fee schedule for payment determination.

The tables below illustrate examples of non-reimbursable items/services codes:

Facility Responsibility	
Typically Billed Under This/These Revenue Codes, but not Limited to the Revenue Codes Listed Below	**Description of Excluded Items**
0990 – 0999	Personal Care Items • Courtesy/Hospitality Room • Patient Convenience Items (0990) • Cafeteria, Guest Tray (0991) • Private Linen Service (0992) • Telephone, Telegraph (0993) • TV, Radio (0994) • Non-patient Room Rentals (0995) • Beauty Shop, Barber (0998) • Other Patient Convenience Items (0999)
0369	Preoperative Care or Holding Room Charges
0760 – 0769	Special Procedure Room Charge
0111 – 0119	Private Room* (subject to Member's Benefit)
0221	Admission Charge
0480 – 0489	Stand-by Charges
0220, 0949	Add on Stat Charges
0270 – 0279, 0360	Video Equipment Used in Procedures
0270, 0271, 0272	Supplies and Equipment • Blood Pressure cuffs/Stethoscopes • Thermometers, Temperature Probes, etc. • Pacing Cables/Wires/Probes • Pressure/Pump Transducers • Transducer Kits/Packs • SCD Sleeves/Compression Sleeves/Ted Hose • Oximeter Sensors/Probes/Covers • Electrodes, Electrode Cables/Wires • Oral swabs/toothettes • Wipes (baby, cleansing, etc.) • Bedpans/Urinals • Bed Scales/Alarms • Specialty Beds • Foley/Straight Catheters, Urometers/Leg Bags/Tubing • Specimen traps/containers/kits • Tourniquets • Syringes/Needles/Lancets/Butterflies • Isolation carts/supplies • Dressing Change Trays/Packs/Kits • Dressings/Gauze/Sponges • Kerlix/Tegaderm/OpSite/Telfa • Skin cleansers/preps • Cotton Balls • Band-Aids, Tape, Q-Tips • Diapers/Chucks/Pads/Briefs

Facility Responsibility	
Typically Billed Under This/These Revenue Codes, but not Limited to the Revenue Codes Listed Below	**Description of Excluded Items**
	• Irrigation Solutions • ID/Allergy bracelets • Foley stat lock • Gloves/Gowns/Drapes/Covers/Blankets • Ice Packs/Heating Pads/Water Bottles • Kits/Packs (Gowns, Towels, and Drapes) • Basins/basin sets • Positioning Aides/Wedges/Pillows • Suction Canisters/Tubing/Tips/Catheters/Liners • Enteral/Parenteral Feeding Supplies (tubing/bags/sets, etc.) • Preps/prep trays • Masks (including CPAP and Nasal Cannulas/Prongs) • Bonnets/Hats/Hoods • Smoke Evacuator Tubing • Restraints/Posey Belts • OR Equipment/Supplies (saws, skin staplers, staples & staple removers, sutures, scalpels, blades etc.) • IV supplies (tubing, extensions, angio-caths, stat-locks, blood tubing, start kits, pressure bags, adapters, caps, plugs, fluid warmers, sets, transducers, fluid warmers, etc.)
0220 – 0222, 0229, 0250	Tech Support Charges • Pharmacy Administrative Fee (including mixing meds) • Portable Fee (cannot charge portable fee unless equipment is brought in from another Facility) • Patient transport fees
0223	Utilization Review Service Charges
263	IV Infusion for therapy, prophylaxis (96365, 96366) IV Infusion additional for therapy: • IV Infusion concurrent for therapy (96368); • IV Injection (96374, 96379)
0229, 0760 – 0762, 0769, 0270, 410 – 413, 0419	Other Charges • Observation hours may never exceed the charge of a semiprivate room charge • Oxygen charges while a patient is on a ventilator • Respiratory assessment/vent management charges
0230, 0270 – 0272, 0300 – 0307, 0309, 0390-0392, 0310	Nursing Procedures and 99001 – Handling and/or conveyance of specimen from patient (charge for specimen handling)
0230	Incremental Nursing – General
0231	Nursing Charge – Nursery
0232	Nursing Charge – Obstetrics (OB)
0233	Nursing Charge – Intensive Care Unit (ICU)

Facility Responsibility	
Typically Billed Under This/These Revenue Codes, but not Limited to the Revenue Codes Listed Below	**Description of Excluded Items**
0234	Nursing Charge – Cardiac Care Unit (CCU)
0235	Nursing Charge – Hospice
0239	Nursing Charge – Emergency Room (ER) or Post Anesthesia Care Unit (PACU) or Operating Room (OR)
0250 – 0259, 0636	Pharmacy • Compounding fees • Medication prep • Nonspecific descriptions • Anesthesia Gases – Billed in conjunction with Anesthesia Charges • IV Solutions 250 cc or less • Miscellaneous Descriptions • Non-FDA Approved Medications (subject to UM determination- Medical Policies)
0256	Experimental Drugs (subject to UM determination- Medical Policies)
0270, 0300 – 0307, 0309, 0380 – 0387, 0390 – 0392	Venipuncture (CPT Code 36415, 36416, or G0001) • Specimen collection • Draw fees • Phlebotomy • Heel stick • Blood storage and processing blood administration • Thawing/Pooling/Splitting, etc.
0222, 0270, 0272, 0410, 0460	Portable Charges
0270 – 0279, 0290, 0320, 0410, 0460	Supplies and Equipment (including rentals) • Preparation (Set-up) Charges; Set-up is included in the fee for the procedure and/or the room and board • Oxygen (ICU/CCU/Progressive) O.R., ER and Recovery • Instrument Trays and/or Surgical Packs • Drills/Saws (All power equipment used in O.R.) • Drill Bits • Blades • IV pumps and PCA (Patient Controlled Analgesia) pumps • Isolation supplies • Daily Floor Supply Charges • X-ray Aprons/Shields • Blood Pressure Monitor • Beds/Mattress • Patient Lifts/Slings • Restraints • Transfer Belt • Bair Hugger Machine/Blankets • SCD Pumps • Heel/Elbow Protector

Facility Responsibility	
Typically Billed Under This/These Revenue Codes, but not Limited to the Revenue Codes Listed Below	**Description of Excluded Items**
	• Burrs • Cardiac Monitor • EKG Electrodes • Vent Circuit • Suction Supplies for Vent Patient • Electrocautery Grounding Pad • Bovie Tips/Electrodes • Anesthesia Supplies When Billed with Anesthesia Charges • Case Carts • C-Arm/Fluoroscopic Charge • Wound Vacuum Pump and supplies • Bovie/Electro Cautery Unit • Wall Suction • Retractors • Single Instruments • Oximeter Monitor • CPM Machines • Lasers • DaVinci Machine/Robot
0309 – 0369, 0419, 0619	After Hours – Call-back
0370 – 0379, 0410, 0460, 0480 – 0489	Anesthesia (Specifically, conscious/moderate sedation by the same physician or procedure nurse) • Nursing care • Monitoring • Pre- or Post-evaluation and education • IV sedation and local anesthesia by the same physician or procedure nurse • Intubation/Extubation • CPR
410	Nursing/Respiratory Functions: • Oximetry (94760, 94761, 94762) • Vent Management • Postural Drainage • Suctioning Procedure • Nursing/Respiratory care performed while the patient is on vent
0480 – 0489	Percutaneous Transluminal Coronary Angioplasty (PTCA) stand-by charges
0940 – 0945	Education/Training
0270, 0272, 0300 – 0309	Bedside/Point of Care/Near Patient Testing (such as glucose, blood count, arterial blood gas, clotting time, etc.)

Member Responsibility	
Typically Billed Under This/These Revenue Codes, but not Limited to the Revenue Codes Listed Below	**Description of Excluded Items**
0110 – 0119	Private Room, subject to the Member's Benefit Agreement
0990	Special Charges
0991	Cafeteria, Guest Tray
0992	Private Linen Service
0993	Telephone, Telegraph
0994	TV, Radio
0995	Non-patient Room Rentals
0996	Late Discharge
0998	Beauty Shop, Barber
0999	Other Patient Convenience Items

Medical Policies and Clinical Guidelines

Clinical Practice Guidelines

Anthem considers clinical practice guidelines to be an important component of healthcare. Anthem adopts nationally recognized clinical practice guidelines and encourages physicians to utilize these guidelines to improve the health of Members. Several national organizations, such as the National Heart, Lung, and Blood Institute, the American Diabetes Association, and the American Heart Association, produce guidelines for asthma, diabetes, hypertension, and other conditions. The guidelines, which Anthem uses for quality and disease management programs, are based on reasonable medical evidence. Anthem reviews the guidelines at least every year or when changes are made to national guidelines for content accuracy, current primary sources, new technological advances, and recent medical research.

Providers can access the up-to-date listing of the medical, preventive, and behavioral health guidelines online. To access the guidelines, go to the **Anthem Provider Home page**, then under the **Resources** menu, select **Policies, Guidelines and Manuals**. Scroll to **Clinical Practice Guidelines** and select **Download the Index**.

With respect to the issue of coverage, each Member should review his/her Certificate of Coverage and Schedule of Benefits for details concerning benefits, procedures, and exclusions prior to receiving treatment. The Certificate of Coverage and/or Schedule of Benefits supersede the clinical practice guidelines.

Preventive Health Guidelines

Anthem considers prevention an important component of healthcare. Anthem develops preventive health guidelines in accordance with recommendations made by nationally recognized organizations and societies such as the American Academy of Family Physicians (AAFP), the American Academy of Pediatrics (AAP), the Advisory Committee on Immunization Practices (ACIP), the American College of Obstetrics and Gynecology (ACOG), and the United States Preventive Services Task Force (USPSTF). The above organizations make recommendations based on reasonable medical evidence.

Anthem reviews the guidelines annually for content accuracy, current primary sources, new technological advances, and recent medical research, and makes appropriate changes based on this review of the recommendations and/or preventive health mandates. Anthem encourages physicians to utilize these guidelines to improve the health of Members.

The current guidelines are available online. To access the guidelines, go to the **Anthem Provider Home page**, then under the **Resources** menu, select **Policies, Guidelines and Manuals**. Scroll to **Preventive Health Guidelines** and select **Review the guidelines**.

With respect to the issue of coverage, each Member should review his/her Certificate of Coverage and Schedule of Benefits for details concerning benefits, procedures, and exclusions prior to receiving treatment. The Certificate of Coverage and/or Schedule of Benefits supersede the preventive health guidelines.

Medical Policies and Clinical Utilization Management (UM) Guidelines

The Office of Medical Policy & Technology Assessment (OMPTA) develops medical policy and clinical Utilization Management (UM) guidelines (collectively, “Medical Policy”) for Anthem. The principal

component of the process is the review for development of Medical Necessity and/or investigational position statements, or clinical indications that are objective and based on medical evidence for certain new medical services and/or procedures, or for new uses of existing services and/or procedures. The services, consisting of medical, surgical, and behavioral health treatments, may include, but are not limited to, devices, biologics, specialty pharmaceuticals, gene therapies, and professional health services.

Medical Policies are intended to reflect current scientific data and clinical thinking. While Medical Policy sets forth position statements or clinical indications regarding the medical necessity of individual services and/or procedures, Federal and State law, as well as contract language, including definitions and specific provisions/exclusions, take precedence over Medical Policy and must be considered first in determining eligibility for coverage.

The Medical Policy & Technology Assessment Committee (MPTAC) is a multiple-disciplinary group including physicians from various medical and behavioral health specialties, clinical practice environments, and geographic areas. Voting memberships may include external physicians in clinical practices and participating in networks; external physicians in academic practices and participating in networks, internal medical directors, and Chairs of MPTAC Subcommittees. Non-voting members may include internal legal counsel and internal medical directors.

Additional details regarding the Medical Policy development process, including information about MPTAC and its Subcommittees, are provided in **ADMIN.0001 Medical Policy Formation**.

Medical Policy and Clinical UM Guidelines Distinction

Medical policy and clinical UM guidelines differ in the type of determination being made. Both set forth position statements or clinical indications regarding the medical necessity of individual services and/or procedures. In general, Medical Policy may be developed to address experimental or investigational technologies (including a novel application of an existing technology) and services where there is a significant concern regarding Member safety. Clinical UM guidelines may be developed to address Medical Necessity for technologies or services where sufficient clinical evidence exists to evaluate the clinical appropriateness of the request, goal length of stay (GLOS) place of service and level of care. In addition, Medical Policies are implemented by all Anthem Plans, while clinical UM guidelines are adopted and implemented at the discretion of the local Anthem Plan or line of business discretion.

Accessing Medical Policies and Clinical UM Guidelines

Medical policies and clinical UM guidelines are available on our websites, which provide transparency for Providers, Facilities, Members, and the public in general. Some vendor guidelines used to make coverage determinations are proprietary and are not publicly available on the health plan's website, but are available upon request.

To locate Medical Policy online, go to the **Anthem Provider Home page**, then under the **Resources** menu, select **Medical Policies & Clinical UM Guidelines**. Search for policies and guidelines using a keyword or code, or select **Full List page** to view.

Medical Policy and Clinical UM Guidelines

BlueCard. To locate medical policy and clinical UM guidelines and prior authorization requirements for BlueCard Out-of-area members, go to the **Anthem Provider Home page**, then under the **Resources** menu, select **Prior Authorization**. Scroll to **Helpful Links.** Select **Medical Policy and Prior Authorization for Blue Plans**. Select **Type of information being requested,** enter **Alpha Prefix**, select **Go.** Review external link acknowledgement and select **Continue.** Read the

acknowledgment reference of the general policies of Anthem Blue Cross and Blue Shield and select **Continue**.

Medical Policy and Prior Authorization for Blue Plans

Clinical UM Guidelines

The clinical UM guidelines published on **anthem.com** represent the clinical UM guidelines currently available to all Plans for adoption throughout the organization. Because local practice patterns, Claims systems, and benefit designs vary, a local Plan or line of business may choose whether to implement a particular clinical UM guideline. The link below can be used to confirm whether the local Plan or line of business has adopted the clinical UM guideline(s) in question. Adoption lists are created and maintained solely by each local Plan or line of business.

To view the list of specific clinical UM guidelines adopted by Kentucky, go to the **Anthem Provider Home page**, then under the **Resources** menu, select **Medical Policies & Clinical UM Guidelines**. Scroll to the bottom of the Clinical UM Guidelines section and select the link titled "**Clinical UM Guidelines for Kentucky**".

Other Criteria

In addition to medical policy and clinical UM guidelines maintained for coverage decisions, the health plan may adopt third-party criteria, which are developed and maintained by other organizations. Where the health plan has developed criteria that address a service also described in one of the third party's sets of criteria, the health plan's medical policy supersedes.

To access these other criteria, go to the Anthem Provider Home page, then under the Resources menu, select **Medical Policies & Clinical UM Guidelines**. Scroll to **Other Criteria,** and select the desired criteria:

- MCG
- Carelon Medical Benefits Management
- CarelonRx
- Carelon Post Acute Solutions

Utilization Management

Utilization Management (sometimes referred to as Utilization Review) is our evaluation of clinical information for the purpose of making favorable determinations and adverse determinations to ensure appropriateness of care.

Utilization Management Program

The Utilization Management (UM) Program goal is to have Members receive the appropriate quantity and quality of healthcare services, delivered at the appropriate time, and in a setting consistent with their medical care needs. Providers and Facilities agree to abide by the following UM Program requirements in accordance with the terms of the Agreement and the Member's Health Benefit Plan. Providers and Facilities agree to cooperate with Anthem in the development and implementation of action plans arising under these programs. Providers and Facilities shall comply with all requests for medical information required to complete Anthem's UM review. Providers and Facilities agree to adhere to the following provisions and provide the information as outlined within this Utilization Management section.

Decisions are based on medical necessity and appropriateness of care and service, and the organization does not specifically reward denials of coverage.

UM Definitions

Adverse Determination: means a denial, reduction or failure to make payment (in whole or in part) for a benefit based on a determination that a benefit is experimental, investigational, or not medically necessary or appropriate as defined in the applicable health benefit plan. This may apply to Prospective, Continued Stay, and Retrospective reviews.

Business Day: Monday through Friday, excluding designated company holidays.

Continued Stay Review: (continuation of services)**.** Utilization review that is conducted during a Member's ongoing stay in a Facility or course of treatment. Continued Stay Review includes continuation of services (Urgent Care & Extensions).

Discharge Planning: includes coordination of medical services and supplies, medical personnel, and family to facilitate the Member's timely discharge to a more appropriate level of care following an inpatient admission.

Notification: The telephonic and/or written/electronic communication to the applicable Provider(s), Facility, and the Member documenting the UM determination.

Pre-certification (includes Pre-authorization, Pre-Service, Prospective): List of services that require review by UM prior to service delivery. For UM team to perform reviews, the Provider submits the pertinent information as soon as possible to UM prior to service delivery.

Review Types:

- **Prospective Review:** UM review conducted on a health care service (or supply) that requires pre-certification prior to its delivery to the Member.
- **Continued Stay Review:** UM review conducted during a Member's ongoing stay in a Facility or course of treatment. Continued Stay Review includes Continuation of Services (Urgent Care & Extensions).

- **Retrospective Review:** UM review conducted after the healthcare service (or supply) has been provided to the Member.
- **Urgent Care Review:** request for medical care or services where application of the time frame for making routine or non-life-threatening care determinations:
    a. Could seriously jeopardize the life or health of the Member or the Member's ability to regain maximum function, based on a prudent layperson's judgment, or
    b. Could seriously jeopardize the life, health or safety of the Member or others, due to the Member's psychological state, or
    c. In the opinion of a practitioner who is a licensed or certified professional providing medical care or behavioral healthcare services with knowledge of the Member's medical or behavioral condition, would subject the Member to adverse health consequences without the care or treatment that is the subject of the request.

Program Overview

UM review may be required for Prospective, Continued Stay, or Retrospective services. UM may be conducted via multiple communication paths.

The review may consider such factors as the Medical Necessity of services provided, and whether the service involves cosmetic or experimental/investigative procedures.

Providers and Facilities shall comply with all requests for medical information required to complete UM review up to and including discharge planning coordination. To facilitate the review process, Providers and Facilities shall make best efforts to supply requested information within twenty-four (24) hours of request.

UM will provide electronic or written Notification for all determinations to the Member, Provider, and/or Facility, as applicable.

UM review timeframes follow Federal, State, and accreditation requirements as applicable to the review.

The determination that services are medically necessary is based on the information provided, and is not a guarantee that benefits will be paid. Payments are based on the Member's coverage at the time of service. These terms typically include certain exclusions, limitations, and other conditions. Benefit payment could be limited, for example, when:

- The information submitted with the Claim, or on the medical record, differs from that given for the pre-Claim UM review.
- The service is excluded from coverage.
- The Member is not eligible for coverage when the service is provided.

Inpatient admissions require UM review. UM review for inpatient services may include, but is not limited to, acute hospitalizations, units described as "sub-acute," "step-down," and "skilled nursing facility," designated skilled nursing beds/units; residential treatment facilities, comprehensive outpatient rehabilitation facilities, rehabilitation units, inpatient hospice, and sub-acute rehabilitation facilities or transitional living centers. These services are subject to admission review for the determination of Medical Necessity, site of service, and level of care.

Non-inpatient services may require Pre-certification review.

Pre-certification requirements can be accessed in several ways:

- **Online**: Go to the **Anthem Provider Home page**, then under the **Resources** menu, select **Prior Authorization**. Select **Kentucky** to see the appropriate link depending on the type of Member Plan.
- **In Availity:** Pre-certification requirements may be confirmed by utilizing the Availity portal
- **By Phone:** Contact the appropriate phone number on the back of the Member's ID card.

Providers and Facilities shall verify that the Member's primary care physician has provided a referral as required by certain Health Benefit Plans.

Prospective Review and Continued Stay Review

A. Elective inpatient admission and outpatient procedures require review and to have a decision rendered **before** the service occurs. Information provided to UM shall include demographic and clinical information, including but not limited to primary diagnosis. For information on applicable penalties for non-compliance, see the *Failure to Comply with Utilization Management Program* section.

B. Emergency inpatient admission requires Providers and Facilities to notify UM within forty-eight (48) hours or the first Business Day following admission. If the forty-eight (48) hours expires on a day that is not a Business Day, the timeframe will be extended to include the next Business Day. Information provided to UM shall include demographic information and primary diagnosis. For information on applicable penalties for non-compliance, see the *Failure to Comply with Utilization Management Program* section.

Retrospective Utilization Management

Medical records and pertinent information regarding the Member's care may be reviewed to make a Claim determination for services that require prior authorization after services have been rendered. For information on medical records submission, refer to the "Member Medical Records Submission" located in the Claims Submission section of this manual.

Penalties may result for failing to preauthorize elective inpatient admissions, outpatient procedures, or providing notification within forty-eight (48) hours of an emergency admission, even if records are reviewed retrospectively.

For information on applicable penalties for non-compliance, see the *Failure to Comply with Utilization Management Program Processes* section.

Medical Policies and Clinical UM Guidelines

Refer to the Medical Policies and Clinical Utilization Management (UM) Guidelines section of this manual for additional information about Medical Policy and Clinical UM Guidelines.

On-Site/Electronic Medical Record Review (EMR)

If applicable, the Facility agrees to provide UM with on-site or EMR access through the facility portal or through Epic Payor Platform (EPP) for inpatient admission reviews.

Discharge Planning

Discharge planning includes the coordination of medical services and supplies, medical personnel, and family to facilitate the Member's timely discharge to a more appropriate level of care following an inpatient admission.

Failure to Comply With Utilization Management Program Processes

Providers and Facilities acknowledge that Anthem may apply monetary penalties, such as a reduction in payment, as a result of Provider's or Facility's failure to provide notice of admission or obtain Pre-certification review on specified outpatient procedures, as required under the Agreement, or for Provider's or Facility's failure to fully comply with and participate in any cost management programs and/or UM programs. Members may not be balance billed for penalty amounts.

Penalties include, but are not limited to, the following:

- Pre-certification review is required for elective inpatient admissions and outpatient procedures that require Pre-certification, as specified by Anthem, that are not submitted for review and a decision rendered **BEFORE** the service occurs will be subject to a thirty percent (30%) payment penalty. Providers and Facilities can only dispute the thirty percent (30%) penalty in order to present evidence of extenuating circumstances.
- Payment for emergency inpatient admissions will be subject to a thirty percent (30%) penalty if the notification is not provided within forty-eight (48) hours of admission. Providers and Facilities can only dispute the thirty percent (30%) penalty in order to present evidence of extenuating circumstances. If the forty-eight (48) hours expires on a day that is not a Business Day, the time frame will be extended to include the next Business Day.

Extenuating Circumstances Approval List

- Insurance information was not available from the Member at the time of admission, or incorrect information was received from the Member, due to illness, mental status, or language differences at the time of services. Including primary payer issues (e.g., Medicare, AKA admissions, or VIP member admitted under a false name, etc.).
- Anthem health system problems prevented authorization from being obtained, or Anthem health provides erroneous information (e.g., misinformation about authorization requirements or Member eligibility).
- Admission or services received are court-ordered.
- The need for another covered service was revealed and performed at the time the original authorized service was performed; the newly revealed covered service would not receive a late call penalty.
- The Member presented with an emergency/urgent condition or life-threatening illness/injury/trauma (e.g., intubation or loss of consciousness).
- Routine maternity admissions/newborn admissions – active/Coordination of Benefits membership
- Proof of timely notification of emergency admission was received within forty-eight (48) hours or the first business day following admission. If the forty-eight (48) hours expires on a day that is not a business day, the timeframe will be extended to include the next business day. Substantiation may be requested.
- Provider or Facility was given misinformation about authorization or patient eligibility by an Anthem Health employee or Department of Medical Assistance (DMAS).
- Transition of Care. This includes transfer from one hospital to another or transfer to home.
- The Member was traveling out of the area, and the Provider or Facility had difficulty finding whom to call for the authorization.

- Retro enrollment issues where the member was terminated and then reinstated, but the application was not loaded timely.
- Member's plan reinstated post-admission and retroactive to a date prior to the admission.
- A Provider or Facility system outage extending forty-eight (48) hours beyond the date of service requiring authorization prevented the authorization from being obtained, and the Provider or Facility has provided adequate evidence of the system outage.
- A Member is admitted to observation and then becomes an inpatient.
- Any other Extenuating Circumstances specific to the health plan.

Utilization Statistics Information

On occasion, Anthem may request utilization data. These may include, but are not limited to:

- Member name
- Member identification number
- Date of service or date specimen collected
- Physician name and/or identification number
- HEDIS Measures or any other pertinent information Anthem deems necessary.

This information will be provided by the Provider or Facility at no charge to Anthem.

Inpatient Electronic Data Exchange

For additional information, see the Clinical Data Sharing section of this Manual, located under Legal and Administrative Requirements.

Prior Authorization Determinations and Submissions

Use our digital tools to determine and submit prior authorizations**.**

- **Determinations:** To **determine** prior authorization requirements:
  - **Online:** The preferred method for Providers to verify outpatient prior authorization requirements is by using Availity.com.
    - Log into **Availity.com**
    - Go to the **Payer Spaces** tab
    - Select the applicable plan
    - Click on **Authorization Rules Lookup**
    - Enter required Provider information
    - Select **Next** to enter the required Member information

    Note: There could be infrequent scenarios where the lookup tool indicates a prior authorization is needed, but upon submission of the prior authorization request, it is determined not to be required based on additional information provided.
  - **Website:** The list of prior authorization requirements can also be accessed online:
    - Go to the Anthem Provider Home page
    - Under the Resources heading, select Prior Authorization

- Select the applicable state
- Select the appropriate link depending on the type of Member Plan
- If the Member's home plan is not with Anthem, the applicable home plan will determine prior authorization requirements. The prefix lookup tool within anthem.com will inform the Member's home plan.

  - **Phone:** The prior authorization requirements may be confirmed by calling the appropriate phone number on the back of the Member's ID card.

- **Submissions:** To **submit** prior authorization requests

  Using Availity.com to submit prior authorizations offers a streamlined and efficient experience for Providers and Facilities requesting inpatient and outpatient medical services for members covered by Anthem plans. Providers and Facilities can also use the Availity Essentials Authorization application to check authorization status, regardless of how the authorization was submitted.

  - To submit digital prior authorizations:
    - Log onto Availity.com
    - Select the Patient Registration tab to access Authorizations and Referrals
    - Select Authorization Request
  - Transplant Pre-certification requests should be submitted via telephone, fax, or secured email notification.

Peer-to-Peer Review Process

Upon request from a treating practitioner who is a licensed or certified professional providing medical care or behavioral health services and directly involved in the Member's care/treatment plan. Anthem provides a clinical peer-to-peer conversation when an adverse medical necessity determination will be made or has been made regarding healthcare services for Members. The treating Practitioner may offer additional information and/or further discuss their cases with a peer clinical reviewer.

In compliance with accreditation standards, a Practitioner or their designee may request the peer-to-peer review. Others, such as hospital representatives, employers, and vendors, are not permitted to do so.

Quality of Care Incident

Providers and Facilities will notify Anthem in the event there is a quality of care incident that involves a Member.

For additional information, go to the *Member Quality of Care/Quality of Service Investigations* section of this Manual.

Audits/Records Requests

At any time Anthem may request on-site, electronic, or hard copy medical records, utilization review documentation for the purposes of conducting audits and reviews to determine Medical Necessity, diagnosis, and other coding and documentation of services rendered.

Case Management

Case Management assists Members to optimize the use of their benefits and available community resources to gain access to quality healthcare in all settings.

The Case Management programs help coordinate services for Members with healthcare needs due to serious, complex, and/or chronic health conditions. The programs coordinate benefits and educate Members who agree to take part in the Case Management program to help meet their health-related needs. Case Management programs are confidential and voluntary and are made available at no extra cost. These programs are provided by, or on behalf of, and at the request of, case management staff. These Case Management programs are separate from any Covered Services. If the Member meets program criteria and agrees to take part, the case manager will help the Member meet identified health care needs. This is reached through contact and teamwork with the Member and/or the Member's chosen authorized representative, treating Physician(s), and other Providers.

Assistance may be provided in coordinating care with existing community-based programs and services. This may include giving information about external agencies and community-based programs and services.

Care management services ***may*** be provided by Carelon Health Services, Inc.

Carelon Medical Benefits Management

Carelon Medical Benefits Management (Carelon MBM) provides clinical solutions that drive appropriate, safe, and affordable care. Serving more than 50 million Members across 50 states, Washington, D.C., and U.S. territories, Carelon MBM promotes optimal care using evidence-based clinical guidelines and real-time decision support for both providers and their patients. The Carelon MBM platform delivers significant cost-of-care savings across an expanding set of clinical domains, including cancer care quality, cardiology, genetic testing, musculoskeletal care, medical and radiation oncology, radiology, rehabilitation, sleep medicine, post-acute, and surgical procedures.

Visit **Carelon MBM's program microsite** to find program information, resources, clinical guidelines, interactive tutorials, worksheets & checklists, FAQs, and access to the provider portal.

Pre-certification requests to Carelon MBM

Ordering and servicing Providers and Facilities may submit Pre-certification requests to Carelon MBM in one of the following ways:

- Access the provider portal directly at **providerportal.com.** Online access is available 24/7 to process orders in real-time, and is the fastest and most convenient way to request authorization.
- Call the Carelon Medical Benefits Management Contact Center toll-free number: **1-833-419-1357.**

Provider Connections

Providers and Facilities are encouraged to visit Carelon MBM's **Provider Connections website**, a news blog for prior authorization staff and provider portal users. Visit the site frequently to learn about the best ways to use the Carelon MBM provider portal and how to reduce unnecessary calls by using all available features. Our goal is to provide tips and tricks to make Providers and Facilities days easier and more efficient.

OptiNet Registration

The OptiNet Registration is an important tool that assists ordering Providers and Facilities in real-time decision support information to enable ordering Providers and Facilities to choose high-quality, low-cost imaging and genetic counseling Providers and Facilities for their patients. Servicing Providers and Facilities need to complete the OptiNet Registration online.

To access the OptiNet Registration:

- Access the provider portal directly at **providerportal.com**
    - Once logged into Carelon Medical Benefits Management, from the **My Homepage** screen, choose **Access OptiNet Registration**.
- Select the Registration Type and choose the Access OptiNet Registration button.
- Complete the requested information.

The registration does not need to be completed in one sitting. Data can be saved throughout the registration process. Once the registration has been submitted, a scorecard will be produced for Radiation Solution Facilities. Genetics Testing Facilities will not have a scorecard. The score for the Facility will be presented to the ordering Provider or Facility when the particular Facility is selected as a place of service, which drives Ordering Provider Decision Support.

For technical questions, contact Web Support at **800-252-2021**. For specific OptiNet customer service requests, contact **877-202-6543**. For any other questions, contact Anthem Provider Services.

Quality Improvement Program

Quality Improvement (QI) Program Overview

Anthem Quality Improvement (QI) Program is outlined in the Quality Improvement Program Description (QIPD), which details Anthem's quality infrastructure and activities that support its QI strategies. This document defines the program governance, scope, goals, objectives, structure, and responsibilities, all of which ensure the quality of medical and behavioral health care available to covered members.

Local Presence and Member Needs

Healthcare is local, and Anthem's strong local presence is crucial for understanding and supporting member needs to provide accessible, covered care. Anthem is well-positioned to deliver innovative, choice-based products, distinctive service, simplified transactions, and better access to information for quality care. This local presence, combined with broad expertise, facilitates collaborative programs that reward Providers and Facilities for achieving clinical quality and excellence. Participating Providers and facilities are expected to cooperate with quality activities, which enhance member value and improve health and healthcare costs. Anthem plays a leadership role in addressing key healthcare issues and improving community health.

"Whole Health" Strategy and Digital Solutions

Guided by its "Whole Health" strategy, Anthem employs digital-first solutions to deliver exceptional experiences, affordability, high quality, and broader access to care. Digital solutions are the cornerstone of Anthem's strategy, enabling value creation and responsiveness to societal shifts, market demands, and consumer needs. Anthem focuses on integrating data, analytics, insights, and digital technologies throughout the business.

Commitment to Continuous Quality Improvement

Continuous QI is essential for supporting member health and Anthem's business objectives. This commitment aligns with Anthem's mission statement: "Improving lives and communities. Simplifying healthcare. Expecting more." The pursuit of excellence, driven by this mission, forms the foundation for programs and initiatives that aim to achieve meaningful and measurable quality outcomes for members.

Vision and Core Values

Anthem's vision is to be the most innovative, valuable, and inclusive partner. Anthem is dedicated to enhancing member and provider satisfaction, improving health status and quality of care, delivering value-based products and services, enhancing patient safety, improving care coordination, and promoting access to medical services. The comprehensive and integrated QI program involves departments that support quality initiatives, systematically monitoring and evaluating the safety, appropriateness, and quality of medical and behavioral healthcare services by network Providers and identifying opportunities for continuous improvement.

Core values such as leadership, community, integrity, agility, and diversity provide a solid foundation for achieving success.

QI Program Goals and Objectives

The goals and objectives of the QI program support Anthem's vision and values and respond to the evolving needs of Members, Providers, Facilities, and the healthcare community.

These goals focus on establishing Anthem as a valued health partner across the healthcare continuum. Anthem implements evidence-based interventions from both external and internal sources to deliver the best value to customers.

Goals and Objectives

- Quality of Clinical Care
    - Goal: Improve Member health through the enhancement of clinical quality outcomes
    - Objective 1: Identify, monitor, and report priority HEDIS measures
    - Objective 2: Promote evidence-based practices
- Safety of Clinical Care
    - Goal: Promote patient safety and health equity
    - Objective 1: Promote transparency and collaboration to improve patient safety
    - Objective 2: Identify and analyze health disparities
- Quality of Service
    - Goal: Provide high-quality services to Members
    - Objective 1: Maintain operational excellence through compliance with policies and standards
    - Objective 2: Streamline processes to enhance operational efficiency
- Member Experience
    - Goal: Optimize the Member experience
    - Objective 1: Gather, analyze, and act on member feedback
    - Objective 2: Encourage the use of technology to ease member access

Patient Safety

Patient safety continuous improvement efforts support the mission for improving lives and communities. Areas for monitoring are selected by analyzing data for Members, inherent to the quality of medical and behavioral healthcare delivery and service. Areas of focus include PHM programs that target keeping Members healthy, managing Members with emerging risk, patient safety, or outcomes across settings, and managing multiple chronic illnesses.

Patient safety may be addressed through the following:

- Identification, monitoring, and resolution of complaints and grievances, including those related to potential quality of care and quality of service issues. Implementation of processes to address quality of care and service issues, internally and among the network of Providers and Facilities.
- Integrate and automate reporting of the Member-level quality of care cases from all lines of business at the Provider level.
- Credentialing and recredentialing processes confirm Providers' credentials and qualifications to practice as network Providers and perform services appropriately within their scope of practice. Providers must complete the initial credentialing process prior to caring for Anthem

Members. Ongoing monitoring of license sanctions, Medicare and Medicaid sanctions, complaints, and adverse events occurs between comprehensive recredentialing reviews.

- Provider education via the distribution of evidence-based clinical practice guidelines and preventive health guidelines.
- Educating Members to become actively involved in their own care, as well as providing education around medication safety compliance and targeted “gaps in care”.
- Establish procedures for safety in pharmaceutical prescribing and medication management.
- Patient safety activities/programs designed to promote safe practices by identifying opportunities for improvement, such as PHM and medication adherence.
- Work with regulators and industry leaders such as CMS, NQF, AHRQ, Leapfrog, etc., on updating Seriously Reportable Events list; clarifying and advancing payment policies; and contributing to literature on patient safety, quality improvement, and health equity.

Member Rights and Responsibilities

The delivery of quality healthcare requires cooperation between Members, their Providers and Facilities, and their healthcare benefit plans. One of the first steps is for Members, Providers, and Facilities to understand Member rights and responsibilities. Therefore, Anthem has adopted a Members’ Rights and Responsibilities statement, which can be accessed by going to the **Anthem Provider Home page**, then under the **Resources** menu, select **Policies, Guidelines and Manuals**. Scroll down and select the **Read about member rights** link under the **More Resources/Member Rights and Responsibilities** section. Select **Laws and Rights that Protect You** under the Table of Contents on the left column, then select **What are my rights as a member?**. Members or Providers who do not have access to the website can request copies by contacting Anthem or by calling the number on the back of the Member ID Card.

Continuity and Coordination of Care

Anthem encourages communication between all physicians, including primary care physicians (PCPs), behavioral health practitioners, and medical specialists, as well as other healthcare professionals who are involved in providing care to Anthem Members. Discuss the importance of this communication with each Member and make every reasonable attempt to elicit permission to coordinate care at the time treatment begins. HIPAA allows the exchange of information between Covered Entities for the purposes of Treatment, Payment, and Health Care Operations.

The Anthem Quality Improvement Program is an ongoing, integrative program that features a number of evaluative surveys and improvement activities designed to help ensure the continuity and coordination of care across physician and other health care professional sites, enhancing the quality, safety, and appropriateness of medical and behavioral health services offered by Providers.

Continuity of Care/Transition of Care Program

This program is for Members whose Provider or Facility terminates from the network, and for new Members (meeting certain criteria) who have been participating in active treatment with a provider not within Anthem’s network.

Anthem makes reasonable efforts to notify Members affected by the termination of a Provider or Facility according to contractual, regulatory, and accreditation requirements and prior to the effective termination date. Anthem also helps them select a new Provider or Facility.

Anthem will work to facilitate the Continuity of Care/Transition of Care (COC/TOC) when Members or their covered dependents with qualifying conditions need assistance in transitioning to in-network Providers or Facilities. The goal of this process is to minimize service interruption and to assist in coordinating a safe transition of care. Completion of Covered Services may be allowed at an in-network benefit and reimbursement level with an out-of-network provider for a period of time, according to contractual, regulatory, and accreditation requirements, when necessary to complete a course of treatment and to arrange for a safe transfer to an in-network Provider or Facility.

Completion of Covered Services by a Provider or Facility whose contract has been terminated or not renewed for reasons relating to medical disciplinary cause or reason, fraud, or other criminal activity will not be facilitated.

In addition to the above, due to the requirements of the Federal Consolidated Appropriations Act (CAA), effective January 1, 2022, there are federal continuity of care obligations resulting from (i) the termination of Providers or Facilities from Anthem's network and (ii) the termination of a group health plan from Anthem that results in a loss of benefits provided under such group health plan with respect to Provider or Facility.

Members may contact Customer Care to get information on Continuity of Care/Transition of Care.

Quality-In-Sights®: Hospital Incentive Program (Q-HIP®)

The Quality-In-Sights®: Hospital Incentive Program (Q-HIP®) is Anthem's performance-based reimbursement program for hospitals. The mission of Q-HIP is to help improve patient outcomes in a hospital setting and promote healthcare value by financially rewarding hospitals for practicing evidence-based medicine and implementing best practices. Q-HIP strives to promote improvement in health care quality and to raise the bar by moving the bell-shaped "quality curve" to the right towards high performance.

Q-HIP measures are credible, valid, and reliable because they are based on measures developed and endorsed by national organizations, which may include:

- American College of Cardiology (ACC)
- Center for Medicare and Medicaid Services (CMS)
- Institute for Healthcare Improvement (IHI)
- National Quality Forum (NQF)
- The Joint Commission (JC)
- The Society of Thoracic Surgeons (STS)

In order to align Q-HIP goals with national performance thresholds, the Q-HIP benchmarks and targets are based on national datasets such as the Centers for Medicare and Medicaid Services' Hospital Compare database. The measures can be tracked and compared within and among hospitals for all patient data – regardless of health plan carrier.

Annual meetings are held with participating hospitals from across the country, offering participants an opportunity to share feedback regarding new metrics and initiatives. Additionally, a National Advisory Panel on Value Solutions (NAPVS) was established in 2009 to provide input during the scorecard development process. The NAPVS is made up of patient safety and quality leaders from health systems and academic medical centers from across the country and offers valuable advice and guidance as new measures are evaluated for inclusion in the program.

Participating hospitals are required to provide Anthem with data on measures outlined in the Q-HIP Manual. Q-HIP measures are based on commonly accepted indicators of hospitals' quality of care. Participating hospitals will receive a copy of their individual scorecard, which shows their performance on the Q-HIP measures.

Performance Data

Provider/Facility Performance Data means compliance rates, reports, and other information related to the appropriateness, cost, efficiency, and/or quality of care delivered by an individual healthcare practitioner, such as a physician, or a healthcare organization, such as a hospital. Common examples of performance data would include the Healthcare Effectiveness Data and Information Set (HEDIS) quality of care measures maintained by the National Committee for Quality Assurance (NCQA) and the comprehensive set of measures maintained by the National Quality Forum (NQF). Provider/Facility Performance Data may be used for multiple Plan programs and initiatives, including but not limited to:

- **Reward Programs** – Pay for performance (P4P), pay for value (PFV), and other results-based reimbursement programs that tie Provider or Facility reimbursement to performance against a defined set of compliance metrics. Reimbursement models include, but are not limited to, total cost of care shared savings/risk programs, enhanced fee schedules, and episode bundled payment arrangements.
- **Recognition Programs** – Programs designed to transparently identify high-value Providers and Facilities and make that information available to consumers, employers, peer practitioners, and other healthcare stakeholders.

Overview of HEDIS®

HEDIS *(Healthcare Effectiveness Data and Information Set)* is a set of standardized performance measures used to compare the performance of managed care plans and physicians based on value rather than cost. HEDIS is coordinated and administered by NCQA and is one of the most widely used sets of health care performance measures in the United States. Anthem is responsible for collecting clinical information from Provider offices in accordance with HEDIS specifications. Data is collected in four ways: Administratively, Hybrid, Survey or via Electronic Clinical Data Systems. Currently, HEDIS includes eighty-eight (88)* measures across six (6)* domains:

- Effectiveness of Care
- Access/Availability of Care
- Experience of Care
- Utilization and Risk-Adjusted Utilization
- Health Plan Descriptive Information
- Measures Reported using Electronic Clinical Data Systems

Record requests to Provider offices are a year-round process. Anthem requests that the records be returned within the specified time frame to allow time to abstract the records and request additional information if needed from other Providers. Health plans use HEDIS data to encourage their contracted providers to improve the quality of care and service they provide. Employers and consumers use HEDIS data to help them select the best health plan for their needs.

To simplify staying current with annual HEDIS documentation changes, we've developed a comprehensive HEDIS Resource Library for your use. This library includes tip sheets with coding guidance and measure-specific information, along with other helpful materials to support accurate documentation and claims coding guidance, promoting proper reimbursement and quality reporting.

To access these materials, log into Availity, select Payer, then the Resource tab, or reach out to your plan representative or consultant for assistance.

** Subject to change*

HEDIS® is a registered trademark of the National Committee for Quality Assurance (NCQA).

Overview of CAHPS

CAHPS® (Consumer Assessment of Healthcare Providers and Systems) surveys represent an effort to accurately and reliably capture key information from Health Plan Members about their experiences with Anthem's health plans in the past year. This includes the Member's access to medical care and the quality of the services provided by Anthem's network of Providers. Anthem analyzes this feedback to identify issues causing Member dissatisfaction and works to develop effective interventions to address them. Anthem takes this survey feedback very seriously.

Health Plans report survey results to National Committee for Quality Assurance (NCQA), which uses these survey results for the annual accreditation status determinations and to create National benchmarks for care and service. Health Plans also use CAHPS® survey data for internal quality improvement purposes.

Results of these surveys are shared with Providers annually via the provider newsletter so they have an opportunity to learn how Anthem Members feel about the services provided. Anthem encourages Providers to assess their own practice to identify opportunities to improve patients' access to care and improve interpersonal skills to make the patient care experience a more positive one.

CAHPS® is a registered trademark of the Agency for Healthcare Research and Quality (AHRQ).

Culturally & Linguistically Appropriate Services

Patient panels are increasingly diverse, and needs are becoming more complex. It is important for Providers and Facilities to have the knowledge, resources, and tools to offer culturally competent and linguistically appropriate care. Anthem wants to help work together to achieve health equity.

The U.S. Department of Health and Human Services (HHS) defines cultural competence as the ability to honor and respect the beliefs, languages, interpersonal styles, and behaviors of individuals and families receiving services, as well as staff members who are providing such services. It is a dynamic, ongoing developmental process requiring long-term commitment. The Agency for Healthcare Research and Quality (AHRQ) Patient Safety Network explains that healthcare is defined through a cultural lens for both patients and Providers. A person's cultural affiliations can influence:

- Where and how care is accessed; how symptoms are described,
- Expectations of care and treatment options, and
- Adherence to care recommendations.

Providers and Facilities also bring their own cultural orientations, including the culture of medicine. Offering culturally and linguistically appropriate care incorporates a variety of skills and knowledge, including, but not limited to, the ability to:

- Recognize the cultural factors (norms, values, communication patterns, and world views) that shape personal and professional behavior.
- Develop understanding of others' needs, values, and preferred means of having those needs met.
- Formulate culturally competent treatment plans.
- Understand how and when to use language support services, including formally trained interpreters and auxiliary aids and services, to support effective communication.
- Avoid the use of family members, especially minors, to act as interpreters for limited English proficient patients.
- Understand and adhere to regulations to support the needs of diverse patients, such as the Americans with Disabilities Act (ADA).
- Use culturally appropriate community resources as needed to support patient needs and care.

Anthem ensures Providers and Facilities have access to resources to help support the delivery of culturally and linguistically appropriate services. Anthem encourages Providers and Facilities to access and utilize **MyDiversePatients.com**.

The My Diverse Patient website offers resources, information, and techniques to help Providers and Facilities provide the individualized care every Member deserves, regardless of their diverse backgrounds. The site also includes learning experiences on topics related to cultural competency and disparities that offer free Continuing Medical Education (CME) credit. Current CME offerings include:

- **Caring for Children with ADHD:** Promotes understanding of and adherence to diagnosis and treatment guidelines; use of AAP's Resource Toolkit for Clinicians; awareness of and strategies for addressing disparities.
- **My Inclusive Practice – Improving Care for LGBTQIA+ Patients:** Helps Providers understand the fears and anxieties LGBTQIA+ patients often feel about seeking medical care, learn key health concerns of LGBTQIA+ patients, and develop strategies for providing effective healthcare to LGBTQIA+ patients.
- **Improving the Patient Experience:** Helps Providers identify opportunities and strategies to improve patient experience during a healthcare encounter.
- **Medication Adherence:** Helps Providers identify contributing factors to medication adherence disparities for diverse populations and learn techniques to improve patient-centered communication to support the needs of diverse patients.
- **Moving Toward Equity in Asthma Care:** Helps Providers understand issues often faced by diverse patients with asthma and develop strategies for communicating to enhance patient understanding.
- **Reducing Health Care Stereotype Threat (HCST):** Helps Providers understand HCST and the implications for diverse patients as well as the benefits of reducing HCST to both Providers' patients and practices, and how to do so.

Anthem appreciates the shared commitment by Providers and Facilities to ensure Members receive culturally and linguistically appropriate services to support effective care and improved health outcomes.

Centers of Medical Excellence (CME)

Anthem currently offers access to Centers of Medical Excellence (CME) programs in solid organ and blood/marrow transplants, bariatric surgery, cancer care, cardiac care, maternity care, spine surgery, knee/hip replacement surgery, fertility care, cellular immunotherapy – CAR-T, gene therapy, and substance use treatment and recovery. As much of the demand for CME programs has come from National Accounts, most of Anthem's programs are developed in partnership with the Blue Cross Blue Shield Association (BCBSA) and other Blue plans to ensure adequate geographic coverage. The BCBSA refers to its designated CME providers as Blue Distinction Centers for Specialty Care™. Using objective information and input from the medical community, the BCBSA has designated hospitals, ambulatory surgery centers (ASCs), physicians, and/or clinics as Blue Distinction Centers (BDC) that are proven to outperform their peers in the areas of quality, safety, and, in the case of Blue Distinction Centers+ (BDC+), cost efficiency.

For transplants, cellular immunotherapy, CAR-T, and ventricular assist devices (VAD), Members also have access to the Anthem Centers of Medical Excellence Transplant, Cellular Immunotherapy, and VAD Network. The CME designation is awarded to qualified programs by a panel of national experts currently practicing in the fields of solid organ, bone marrow transplantation, and cardiac surgery representing centers across the country. Each Center must meet Anthem's CME participation requirements and is selected through a rigorous evaluation of clinical data that provides insight into the Facility's structures, processes, and outcomes of care. Current Anthem CME transplant designations include the following transplants: adult and pediatric autologous/allogeneic bone marrow/stem cell, adult and pediatric heart, adult and pediatric lung, adult combination heart/lung, adult and pediatric liver, adult and pediatric kidney, adult simultaneous kidney/pancreas, and adult pancreas.

For both the BDC and Anthem CME programs, selection criteria are designed to evaluate overall quality, providing a comprehensive view of how the Facility delivers specialty care. More information on the programs can be accessed online at **anthem.com**. To view the BDC and Anthem CME program information, **Click Here**.

Transplant

- Blue Distinction Centers for Transplant™ (BDCT) launched in 2006.
- Nearly 104,000 people in the United States were waiting for a lifesaving organ transplant from one of the nation's more than 250 transplant centers in the United States as of December 2022. In the United States, more than 42,800 organ transplants in 2022. In 2022, annual records were set for the total number of kidney, liver, heart, and lung transplants.
- Blue Distinction Centers and Blue Distinction Centers+ for Transplants have demonstrated their commitment to quality care, resulting in better overall outcomes for transplant patients. Each Facility meets stringent clinical criteria, established in collaboration with expert physicians' and medical organizations' recommendations**, including the Center for International Blood and Marrow Transplant Research (CIBMTR), the Scientific Registry of Transplant Recipients (SRTR), and the Foundation for the Accreditation of Cellular Therapy (FACT), and is subject to periodic re-evaluation as criteria continue to evolve. Both Blue Distinction Centers and Blue Distinction Centers+ for Transplants help simplify the administrative process involved in this complex care so that patients, their families, and physicians can focus on the medical issues.

- Hospitals receiving the Blue Distinction Center+ for Transplants designation have met the Blue Distinction Centers' standards for quality while also demonstrating better cost-efficiency relative to their peers.
- The Anthem CME Transplant Network is a wrap-around network to the BDCT program and offers Members access to an additional 60 transplant programs. When BDCT and Anthem CME are combined, Members have access to over 800 transplant-specific programs for adult and pediatric heart, lung, liver, kidney, and bone marrow/stem cell transplant, and adult combined heart/lung, combined liver kidney, pancreas, and combined kidney/pancreas transplant.

Cardiac Care

- Blue Distinction Centers for Cardiac Care® launched in January 2006.
- According to the Centers for Disease Control and Prevention, the number of adults with a diagnosis of heart disease is 30.3 million, and the percent of adults with diagnosed heart disease is 12.1%. Heart Disease is the number one (1) cause of death in the United States. The American Heart Association projects the number of Americans with cardiovascular disease to rise to 131.2 million by 2035.
- Research shows that Blue Distinction Centers and Blue Distinction Centers+ demonstrate better quality and improved outcomes for patients, with lower rates of complications following certain cardiac procedures and lower rates of healthcare-associated infections compared with their peers. Blue Distinction Centers+ (BDC+) are also 21 percent more cost-efficient than non-BDC+ designated hospitals for those same cardiac procedures.
- Blue Distinction Centers and Blue Distinction Centers+ for Cardiac Care focus on elective cardiac procedures, including cardiac valve surgery, coronary artery bypass graft (CABG), and angioplasty (percutaneous coronary intervention (PCI) while providing a full range of cardiac care services, including inpatient cardiac care, cardiac rehabilitation, cardiac catheterization, and cardiac surgery.

Bariatric Surgery

- Blue Distinction Centers for Bariatric Surgery® launched in 2008
- According to the National Center for Health Statistics report released in October 2017, Prevalence of Obesity among Adults and Youth has grown to more than one-third (42.4%) of U.S. adults which have been diagnosed with obesity, and 40% for young adults aged 20-39. Obesity-related conditions include heart disease, stroke, type 2 diabetes, and certain types of cancer, which are some of the leading causes of preventable death.
- Blue Distinction Centers for Bariatric Surgery have demonstrated their commitment to quality care, resulting in better overall outcomes for adult bariatric patients ages 18 and older. Each facility meets stringent clinical criteria, developed in collaboration with expert physicians and medical organizations, including the American Society for Metabolic and Bariatric Surgery (ASMBS) and the American College of Surgeons (ACS), and is subject to periodic re-evaluation as criteria continue to evolve
- The 2020 Blue Distinction Centers for Bariatric Surgery program uses updated Metabolic and Bariatric Surgery Accreditation and Quality Improvement Program (MBSAQIP) accreditation levels, which focus on the site of service. With this design change, each facility can apply to

achieve the BDC or BDC+ designation, as either a Comprehensive Center (including outpatient capability) or an Ambulatory Surgery Center (ASC).

Cancer Care

- Blue Distinction Centers for Cancer Care launched in 2018.
- This national designation program recognizes physicians, physician practices, cancer centers, hospitals, and accountable care organizations (ACOs) for their efforts in coordinating all types of cancer care. This program incorporates patient-centered and data-driven practices to coordinate care better and to improve the quality of care and safety, as well as affordability. Providers in this Program are paid under a provider agreement with their local BCBS Plan that has value-based reimbursement, rather than traditional fee-for-service, so they must perform against both quality and cost outcome targets in order to receive incentives and rewards for better health outcomes.
- Designations will be awarded on an ongoing basis, and the program will continue to expand in the future.

Spine Surgery

- Blue Distinction Centers for Spine Surgery® launched in November 2009.
- Studies confirm that as many as eight (8) out of ten (10) Americans suffer from some sort of back pain. Many ways to treat back pain are available for Providers to work with Members, to guide them toward the most appropriate recommendation for their situation. For those with severe and/or chronic back pain, spine surgery may be a treatment option.
- Research confirms that hospitals designated as Blue Distinction Centers and Blue Distinction Centers+ for Spine Surgery have fewer complications and fewer hospital readmissions than non-designated hospitals. Blue Distinction Centers+ for Spine Surgery also deliver care more efficiently than their peers.
- In 2019, Blue Distinction Specialty Care Program for Spine Surgery expanded to include alternate sites of care, such as ambulatory surgery centers (ASCs) and hospitals without an on-site ICU.
- Blue Distinction Centers and Blue Distinction Centers+ for Spine Surgery provide comprehensive inpatient spine surgery services, including discectomy, cervical and lumbar fusion, cervical laminectomy, lumbar laminectomy/discectomy, and decompression procedures.
- To date, Anthem has designated hospitals in the majority of states across the U.S.

Knee and Hip Replacement

- Blue Distinction Centers for Knee and Hip Replacement™ launched in November 2009.
- In 2019, Blue Distinction Specialty Care Program for Knee and Hip Replacement expanded to include alternate sites of care, such as ambulatory surgery centers (ASCs) and hospitals without an on-site ICU.
- Blue Distinction Centers and Blue Distinction Centers+ for Knee and Hip Replacement provide comprehensive inpatient knee and hip replacement services, including total knee replacement and total hip replacement, and revision surgeries.

Maternity Care

- Blue Distinction Centers and Blue Distinction Centers+ for Maternity Care launched in 2016. The program offers access to healthcare facilities with demonstrated expertise, a commitment to quality care, and safety during the delivery episode of care, which includes both vaginal and cesarean section delivery.
- Recent updates to the program address the goal of reducing racial disparities in maternal health and maternal health crises in the United States. Criteria included recommendations from organizations to enhance outcomes and reduce adverse events. Organizations included the Department of Health and Human Services (HHS), American College of Obstetricians and Gynecologists (ACOG), Alliance for Innovation on Maternal Health (AIM), and the California Maternal Quality Care Collaborative (CMQCC).
- The Maternity Care designation uses publicly available data from Hospital Compare data, which includes the Early Elective Delivery (PC-01), Cesarean Section (PC-02), and selected patient experience measures at the facility level from Hospital Consumer Assessment of Healthcare Providers and Systems (HCAHPS). As well as additional measures to support safe practices in childbirth, prenatal and postpartum care.

Substance Use Treatment and Recovery

- Blue Distinction Centers for Substance Use Treatment and Recovery launched in January 2020 to address the treatment of substance use disorders, including opioid use disorder.
- The program aims to improve patient outcomes and costs by addressing the fragmented delivery of substance use disorder treatment. Designations are awarded based on quality criteria that support delivery of timely, coordinated, multidisciplinary, evidence-based care, with a focus on quality improvement and patient-centered care.
- This includes medication-assisted treatment (MAT) and other evidence-based therapies across care settings. Care settings include residential and inpatient care, intensive outpatient (IOP), and partial hospitalization (PH) treatment. At a minimum, all providers must offer treatment for opioid use disorder.

Ventricular Assist Devices

- Anthem's Centers of Medical Excellence Ventricular Assist Device (VAD) launched in 2017. VADs are implantable pumps that assist the heart by pumping blood in the circulatory system of individuals with end-stage heart failure.
- According to the Centers for Disease Control and Prevention, heart failure reports that about 6.2 million adults in the United States have heart failure, a major public health problem associated with significant hospital admission rates, mortality, and costly health care services.
- Based on registry data, >33,000 left ventricular assist devices (LVADs) were implanted from June 2006 to June 2021. An estimated 3000+ VADs will be implanted worldwide this year, but the volume is expected to increase as newer, smaller devices receive regulatory approval, clinical indications slowly expand, and the continued increase in centers certified to place these devices.

Cellular Immunotherapy (Chimeric Antigen Receptor Therapy – CAR-T)

- The U.S. Food & Drug Administration (FDA) continues to approve new cellular immunotherapy products called Chimeric Antigen Receptor T-cell (CAR-T), which are genetically modified

autologous T cell immunotherapies that provides new treatment options for cancer patients. This treatment involves genetic re-engineering of a patient's white blood cells.

- There are eight (8) Chimeric Antigen Receptor T cell therapies (CAR-T) products, listed below, approved by the FDA. This list continues to grow as new products are approved:
    1. Yescarta® (axicabtagene ciloleucel) for treatment in Adult Patients
    2. Kymriah® (tisangenlecleucel) for treatment in Pediatric and Adult Patients
    3. Tecartus™ (brexucabtagene autoleucel) for treatment in Adult Patients
    4. Abecma® (idecabtagene vicleucel) for treatment in Adult Patients
    5. Breyanzi® (idecabtagene maraleucel) for treatment in Adult Patients
    6. Carvykti® (ciltacabtagene autoleucel) for treatment in Adult Patients
    7. Omisirge (omidubicel) for treatment in Pediatric and Adult Patients
    8. Tecelra (afamitresgene autoleucel) for treatment of Adults Patients with unresectable or metastatic synovial sarcoma
- These procedures can be performed in the Inpatient (IP) or Outpatient (OP) setting, and Care and follow-up continue over the first year.
- These Members are managed by the transplant Case Managers, and Anthem Medical Policy requires the procedure to be performed at a Certified CAR-T center.
- Anthem has a Centers of Medical Excellence Network that continues to expand. These programs are reviewed by our Bone Marrow National Transplant Quality Review Committee. Currently, we have eight (8) contracted CAR-T CME Providers. Until a Provider or Facility is contracted, each referral will require a Letter of Agreement.
- The Blue Cross Blue Shield Association also has a designation, but not a contract requirement for CAR-T Providers in 2020. Providers must be certified by a product manufacturer's certification program to deliver CAR-T therapy.

Gene Therapy

- The U.S. Food & Drug Administration (FDA) continues to approve new gene therapy products, which provide new treatments for various conditions. This treatment involves gene therapy that introduces or is related to the introduction of genetic material into a person, intended to replace or correct faulty or missing genetic material.
    - Zynteglo (Betibeglogene autotemcel) for treatment of adults and children with beta-thalassemia requiring red blood cell transfusions.
    - Skysona (Elivaldogene autotemcel) for treatment of pediatric patients with cerebral adrenoleukodystrophy – early active disease
    - Casgevy (Exagamglogene autotemcel; exa-cel) for treatment of patients two (2) years and older with recurrent vaso-occlusive crises (VOC) or patients twelve (12) years and older with transfusion-dependent beta-thalessemia
    - Lyfgenia (Lovotibeglogene autotemcel; exa-cel) for treatment of patients twelve (12) years and older with sickle cell disease and a history of vaso-occlusive events
    - Lenmeldy (OTL-200; Atidarsagene autotemcel) for treatment of pediatric patients with metachromatic leukodystrophy disease

Audit and Review

This section does not apply to audits or reviews performed by the Special Investigations Unit (SIU). For information on SIU processes, refer to the Fraud, Waste, and Abuse section located in this Manual.

Anthem Claims Audit and Review Policy

All capitalized terms used in this Policy shall have the meaning as set forth in the Provider or Facility Agreement between Anthem and Provider or Facility, unless otherwise defined below for this section.

There may be times when Anthem conducts Claim reviews or audits to confirm that charges for covered healthcare services are accurately reported and reimbursed in compliance with the Provider or Facility Agreement and Anthem's policies and procedures, as well as general industry standard guidelines and regulations.

In order to conduct such reviews and audits, Anthem or its designee may request documentation, most commonly in the form of patient medical records and/or itemized bills. Anthem may accept additional documentation from Provider or Facility that typically might not be included in medical records, such as other documents substantiating the treatment of health services or delivery of supplies.

This policy documents Anthem's guidelines for Claims requiring additional documentation and the Provider's or Facility's compliance with the provision of requested documentation.

Definitions

The following definitions shall apply to this Audit and Review section only:

- **Agreement** means the written contract between Anthem and Provider or Facility that describes the duties and obligations of Anthem and the Provider or Facility, and which contains the terms and conditions upon which Anthem will reimburse Provider or Facility for Health Services rendered by Provider or Facility to Member(s).
- **Audit Appeal** means a written request with supporting documentation to Anthem from a Provider or Facility to reconsider a payment determination.
- **Audit Appeal Response** means Anthem's or its designee's written response to the Appeal after reviewing all Supporting Documentation provided by Provider or Facility.
- **Audit** means a post-payment evaluation of Health Services or documents relating to such Health Services rendered by Provider or Facility, and conducted for the purpose of determining appropriate reimbursement under the terms of the Agreement.
- **Business Associate or Designee** means a third party designated by Anthem to perform an Audit or any related function on behalf of Anthem.
- **Notice of Overpayment** means a document that constitutes notice to the Provider or Facility that Anthem or its designee believes an overpayment has been made by Anthem. The Notice of Overpayment shall contain administrative data relating to the amount of overpayment. Unless otherwise stated in the Agreement between the Provider or Facility and Anthem, Notice of Overpayment shall be sent to the Provider or Facility.

- **Provider Manual** means the proprietary Anthem document available to the Provider and Facility, which outlines Reimbursement Requirements and Policies.
- **Recoupment** means the recovery of an amount paid to Provider or Facility which Anthem has determined constitutes an overpayment not supported by an Agreement between the Provider or Facility and Anthem. In accordance with applicable laws, regulations, and unless an agreement expressly states otherwise, a Recoupment may be performed against a separate Anthem payment unrelated to the service or subject made to the Provider or Facility.
- **Review** means the Claim and supporting documentation will be evaluated prior to payment.
- **Supporting Documentation** means the written material contained in a Member's medical records or other Provider or Facility documentation, Claim details, prior authorization clinical information, and supply invoices supporting the Provider's or Facility's claim.

Documents Reviewed During an Audit or Review:

The following is a description of the documents that may be reviewed by Anthem or its designee, along with a short explanation of the importance of each of the documents in the Audit and Review processes. It is important to note that Providers and Facilities must comply with applicable state and federal record-keeping requirements.

A. Confirm that Health Services were delivered by the Provider or Facility

Auditors/Reviewers will verify that the Provider's or Facility's Claim is corroborated by Supporting Documentation reflecting the Health Services delivered and billed by the Provider or Facility. The Provider or Facility must review, approve, and document all such policies and procedures by any applicable accreditation bodies.

B. Confirm charges were accurately reported on the Claim in compliance with Anthem's Policies as well as general industry standard guidelines and regulations.

Auditors/Reviewers may review Supporting Documentation, including the Member's health record documents. The health record includes the clinical data on diagnoses, treatments, and outcomes. A health record generally includes pertinent information related to care and must support services billed by the Provider or Facility. Auditors/Reviewers may review the Claim itemized Billing for a breakdown of the services billed and supply invoices for pricing determinations.

Auditors/Reviewers may reference the Anthem Reimbursement Policies available on **anthem.com.**

Policy

Upon request from Anthem or its designee, Providers and Facilities are required to submit additional documentation for claims identified for pre-payment review or post-payment audit.

Anthem or its designee will use the following guidelines for additional documentation requests when Claims are identified for pre-payment review or post-payment audit. A request may be made via paper or electronic format.

- A Provider's or Facility's physical or electronic address may be confirmed prior to sending an initial request for supporting documentation.
- When a response is not received within thirty (30) days of the date of the initial request, a second request will be sent.
- When a response is not received within fifteen (15) days of the date of the second request, a final request will be sent.

- When a response is not received within fifteen (15) days of the date of the final request, sixty (60) days total:
  - Anthem or its designee will initiate a Claim denial for Claims identified for pre-payment review or post-payment audit when a Provider or Facility fails to submit the required documentation. The Member shall be held harmless for such payment denials.

    or
  - Anthem or its designee will initiate a full or partial recoupment for Claims identified for post-payment audit when a Provider or Facility fails to submit the required documentation. Anthem or its designee will review all submitted documentation, if any, to make a determination as to whether a full or partial recoupment is appropriate. The Member shall be held harmless for such recoupments.

Anthem or its designee will not be liable for interest or penalties when payment is denied or recouped when Provider or Facility fails to submit required or requested documentation for Claims identified for prepayment review or post-payment audit.

Procedure

- Review of Documents: Anthem or its designee will request in writing any supporting documentation required for audit or review. The Provider or Facility will supply the requested information within the time frame outlined above.
- Desk or Off-site Audits: Anthem or its designee may conduct Audits from its offices. The Facility or Provider will comply with the timeline and specific requested documentation listed in Anthem's request for additional documentation.
- Completion of Desk or Off-site Audit: Upon completion of the Audit, where an overpayment is identified, Anthem will generate a Notice of Overpayment. The Notice of Overpayment will identify the Claim overpayment and include an explanation remark for the overpayment. If the Provider or Facility agrees with the Notice of Overpayment, then the Provider or Facility has thirty (30) calendar days to reimburse Anthem the amount indicated in the form of a refund. Should the Provider or Facility disagree with the Notice of Overpayment, then the Provider or Facility may appeal the Notice of Overpayment. If the Provider or Facility does not submit an Appeal against the Notice of Overpayment and does not reimburse Anthem within the thirty (30) calendar days, then Anthem will initiate recoupment as applicable and determined per Provider or Facility Agreement and state guidelines.
- Provider or Facility Audit Appeals: See Audit Appeal Policy.
- On-site Audits: Anthem or its designee may, but is not required to, conduct Audits on-site at the Provider's or Facility's location. If Anthem or its designee conducts an Audit at a Provider's or Facility's location, Provider or Facility will make available suitable workspace for Anthem or its designee's on-site Audit activities. During the Audit, Anthem or its designee will have complete access to the applicable health records, including ancillary department records and/or invoice detail, without producing a signed Member authorization.

  When conducting a credit balance review, the Provider or Facility will give Anthem or its designee a complete list of credit balances for primary, secondary, and tertiary coverage, when applicable. In addition, Anthem or its designee will have access to the Provider's or Facility's patient accounting system to review payment history, notes, Explanations of Benefits, and insurance information to determine the validity of credit balances. If the Provider or Facility

refuses to allow Anthem or its designee access to the items requested to complete the Audit, Anthem or its designee may opt to complete the Audit based on the information available.

All Audits (to include medical chart audits and diagnosis-related group reviews) shall be conducted free of charge, despite any Provider or Facility policy to the contrary.

- Completion of Audit (On-site Audit only): Upon completion of the Audit, Anthem or its designee will generate and give to Provider or Facility a final Audit Report. This Audit Report may be provided on the day the Audit is completed, or it may be generated after further research is performed. If further research is needed, the final Audit Report will be generated at anytime after the completion of the Audit, but generally within ninety (90) days. Occasionally, the final audit report will be generated at the conclusion of the exit interview, which is performed on the last day of the Audit.

  During the exit interview, Anthem or its designee will discuss with Provider or Facility its Audit findings found in the final Audit Report. This Audit Report may list items such as charges unsupported by adequate documentation, under-billed items, late-billed items, and charges requiring additional supporting documentation.

  If the Provider or Facility agrees with the Audit findings and has no further information to provide to Anthem or its designee, then Provider or Facility may sign the final Audit Report acknowledging agreement with the findings. At that point, Provider or Facility has thirty (30) calendar days to reimburse Anthem the amount indicated in the final Audit Report. Should the Provider or Facility disagree with the final Audit Report generated during the exit interview, then the Provider or Facility may either supply the requested documentation or appeal the Audit findings.

- No Appeal (On-site audit only): If the Provider or Facility does not formally Appeal the findings in the final Audit Report and submit supporting documentation within the (thirty) 30 calendar day timeframe, the initial determination will stand, and Anthem or its designee will process adjustments to recover the amount identified in the final Audit Report.

- Scheduling of Audit. (Hospital Bill Audits Only) After review of the documents submitted, if Anthem or its designee determines an Audit is required, Anthem or its designee will call the Provider or Facility to request a mutually satisfactory time for Anthem or its designee to conduct an Audit; however, the Audit must occur within forty-five (45) calendar days of the request

- Rescheduling of Audit. Should Provider or Facility desire to reschedule an Audit, Provider or Facility must submit its request with a suggested new date to Anthem or its designee in writing at least seven (7) calendar days in advance of the day of the Audit. Provider or Facility, the new date for the Audit must occur within thirty (30) calendar days of the date of the original Audit. Provider or Facility may be responsible for cancellation fees incurred by Anthem or its designee due to Provider's or Facility's rescheduling. While Anthem or its designee prefers to work with the Provider or Facility in finding a mutually convenient time, there may be instances when Anthem or its designee must respond quickly to requests by regulations or its clients. In those circumstances, Anthem or its designee will send a notice to the Provider or Facility to schedule an Audit within the seventy-two (72) hour timeframe.

- Under-billed and Late-billed Claims. During an audit, Provider or Facility may identify Claims for which Provider or Facility under-billed or failed to bill for review by Anthem during the Audit. Under-billed or late-billed Claims not identified by the Provider or Facility before the Audit commences will not be evaluated in the Audit.

Claims Audit Appeal Policy

Purpose

To establish a timeline for responding to Provider or Facility Appeals of Audits. This section does not apply to appeals or reconsideration of Claims denied on pre-payment review. If the Provider or Facility does not agree with the Claim determination for Claims denied on a pre-payment review basis, follow the directions in the Claim Payment Disputes section of this Provider Manual.

Procedure:

- Unless otherwise expressly set forth in an Agreement, Provider or Facility shall have the right to Appeal the findings in the Notice of Overpayment. An Appeal of the Notice of Overpayment must be in writing and received by Anthem or its designee within forty-five (45) calendar days of the date of the Notice of Overpayment unless applicable law expressly indicates otherwise. The Appeal should address the findings from the Notice of Overpayment that Provider or Facility disputes, as well as the basis for the Provider's or Facility's belief that such findings are not accurate. All findings disputed by the Provider or Facility in the Appeal must be accompanied by relevant Supporting Documentation. If the Provider or Facility does not timely appeal, retraction will begin at the expiration of the forty-five (45) calendar days unless expressly prohibited by contractual obligations or applicable law.
- Upon receipt of a timely Appeal, complete with Supporting Documentation as required under this Policy, Anthem or its designee shall issue an Appeal Response to the Provider or Facility. Anthem's or its designee's response shall address each matter contained in the Provider's or Facility's Appeal. If appropriate, Anthem's or its designee's Appeal Response will indicate what adjustments, if any, shall be made to the overpayment amounts outlined in the Notice of Overpayment. Anthem's or its designee's response shall be sent via email, mail, or portal to the Provider or Facility within forty-five (45) calendar days of the date Anthem or its designee received the Provider's or Facility's Appeal and Supporting Documentation.
- The Provider or Facility shall have thirty (30) calendar days from the date of Anthem's or its designee's Appeal Response to respond with additional documentation or, if appropriate in the State, a remittance check to Anthem or its designee. If no Provider or Facility response or remittance check (if applicable) is received within the thirty (30) calendar day timeframe, Anthem or its designee shall begin recoupment of the amount contained in Anthem's or its designee's response, and a confirming recoupment notification will be sent to the Provider or Facility.
- Upon receipt of a timely Provider or Facility appeal response, complete with Supporting Documentation as required under this Policy, Anthem or its designee shall formulate a final Appeal Response. Anthem's or its designee's final Appeal Response shall address each matter contained in the Provider's or Facility's response. Anthem's or its designee's final Appeal Response shall be sent via email, mail, or portal to the Provider or Facility within fifteen (15) calendar days of the date Anthem or its designee received the Provider or Facility response and Supporting Documentation.
- If applicable in the state, the Provider or Facility shall have thirty (30) calendar days from the date of Anthem's or its designee's final Appeal Response to send a remittance check to Anthem or its designee. If no remittance check is received within the thirty (30) calendar day timeframe, Anthem or its designee shall recoup the amount contained in Anthem's or its designee's final Appeal Response.

Fraud, Waste, and Abuse Detection

Anthem is committed to protecting the integrity of Anthem's healthcare programs and the effectiveness of operations by preventing, detecting, and investigating fraud, waste, and abuse (FWA). Combating FWA begins with knowledge and awareness.

- **Fraud**: Any type of intentional deception or misrepresentation made with the knowledge that the deception could result in some unauthorized benefit to the person—or any other person—committing it. This includes any act that constitutes fraud under applicable Federal or State law.
- **Waste:** Includes overusing services or other practices that, directly or indirectly, result in excessive costs. Waste is generally not considered to be driven by intentional actions, but rather occurs when resources are misused.
- **Abuse:** Behaviors that are inconsistent with sound financial, business, and medical practices and result in unnecessary costs and payments for services that are not medically necessary or fail to meet professionally recognized standards for health care. This includes any member actions that result in unnecessary costs.

One of the most important steps to help prevent Member fraud is as simple as reviewing the Member identification card to ensure that the individual seeking services is the same as the Member listed on the card. It is the first line of defense against possible fraud. Learn more at **fighthealthcarefraud.com**.

Reporting Fraud, Waste, and Abuse

If someone suspects any Member (a person who receives benefits) or Provider has committed fraud, waste, or abuse, they have the right to report it. No individual who reports violations or suspected fraud and abuse will be retaliated against for doing so. The name of the person reporting the incident and his or her callback number will be kept in strict confidence by investigators.

Report concerns:

- Visit **anthem.com**, scroll to the bottom footer, and click on "Health Care Fraud Prevention" to be directed to the **Fight Health Care Fraud** education site; at the top of the page, click "Report it" and complete the "**Report Waste, Fraud and Abuse**" form
- Participating Providers can call Provider Solutions
- Non-participating providers can call customer service

Any incident of suspected fraud, waste, or abuse may be reported to Anthem anonymously; however, Anthem's ability to investigate an anonymously reported matter may be limited if Anthem doesn't have enough information. Anthem encourages Providers and Facilities to give as much information as possible when reporting an incident of suspected fraud, waste, or abuse. Anthem appreciates referrals for suspected fraud, waste, or abuse, but be advised that Anthem does not routinely update individuals who make reports, as it may potentially compromise an investigation.

Examples of **Member** Fraud, Waste, and Abuse

- Forging, altering, or selling prescriptions
- Letting someone else use the Member's ID (Identification) card

- Relocating to an out-of-service Plan area and not letting the Plan know
- Using someone else's Member ID card

When reporting concerns involving a **Member,** include:

- The Member's name
- The Member's date of birth, Member ID, or case number if available
- The city where the Member resides
- Specific details describing the suspected fraud, waste, or abuse

Examples of **Provider** Fraud, Waste, and Abuse (FWA):

- Altering medical records to misrepresent actual services provided
- Billing for services not provided
- Billing for medically unnecessary tests or procedures
- Billing professional services performed by untrained or unqualified personnel
- Misrepresentation of diagnosis or services
- Overutilization
- Soliciting, offering, or receiving kickbacks or bribes
- Unbundling – when multiple procedure codes are billed individually for a group of procedures that should be covered by a single comprehensive procedure code
- Upcoding – when a provider bills a health insurance payer using a procedure code for a more expensive service than was actually performed

When reporting concerns involving a **Provider** (a doctor, dentist, counselor, medical supply company, etc.), include:

- Name, address, and phone number of provider
- Name and address of the facility (hospital, nursing home, home health agency, etc.)
- Medicaid number of the provider and facility, if available
- Type of provider (doctor, dentist, therapist, pharmacist, etc.)
- Names and phone numbers of other witnesses who can help in the investigation
- Dates of events
- Summary of what happened

To learn more about health care fraud and how to aid in the prevention of it, visit **fighthealthcarefraud.com.**

Investigation Process

The Special Investigations Unit (SIU) investigates suspected incidents of Provider or Facility—contracted in network or out of network—FWA for all types of services, including member fraud. Anthem may take corrective action with a Provider or Facility, which may include, but is not limited to:

- Written warning and/or education: Anthem sends letters to the Provider or Facility advising the Provider or Facility of the issues and the need for improvement. Letters may include education or may advise of further action.

- Medical record review: Anthem reviews medical records to investigate allegations or validate the appropriateness of Claims submissions. Failure to submit medical records when requested may result in an overpayment determination and/or placement on prepayment review.
- Prepayment Review: Specific to a Provider or Facility under investigation, a certified professional coder in the SIU evaluates Claims prior to payment. Edits in Anthem's Claims processing systems identify these Claims for review to prevent automatic Claims payments in specific situations.
- Recoveries: Anthem recovers overpayments directly from the Provider or Facility. Failure of the Provider or Facility to return the overpayment may result in reduced payment for future Claims, termination from our network, and/or legal action.

If you are working with the SIU, all communication (checks, correspondence) should be sent to:

Anthem Blue Cross and Blue Shield
Special Investigations Unit
740 W Peachtree Street NW
Atlanta, Georgia 30308
Attn: investigator name, #case number

If a Provider or Facility is working with the SIU and sending paper medical records and/or Claims based on an SIU request, that address is supplied in correspondence from the SIU. If you have questions, contact your investigator.

An opportunity to submit Claims and medical records **electronically** is an option if you register for an Availity account. For more information, see the Availity Essentials section of the manual or contact Availity Client Services at 800-AVAILITY (282-4548) for assistance.

Anthem does not accept postdated checks. Any fees incurred for a check returned due to insufficient funds are the responsibility of the Provider or Facility.

SIU Prepayment Review

One method Anthem uses to detect FWA is through prepayment Claim review. Through a variety of means, certain Providers or Facilities, or certain Claims submitted by Providers or Facilities, may come to Anthem's attention for behavior that might be identified as unusual for coding, documentation, and/or billing issues, or Claims activity that indicates the Provider or Facility is an outlier compared to their/its peers.

Once a Claim, or a Provider or Facility, is identified as an outlier or has otherwise come to Anthem's attention for reasons mentioned above, further review may be conducted by the SIU to determine the reason(s) for the outlier status or any appropriate explanation for unusual coding, documentation, and/or billing practices. If the review results in a determination that the Provider's or Facility's actions may involve FWA, unless exigent circumstances exist, the Provider or Facility is notified of their placement on prepayment review and given an opportunity to respond.

When a Provider or Facility is on prepayment review, the Provider or Facility will be required to submit medical records and any other supporting documentation with each Claim so Anthem can review the appropriateness of the services billed, including the accuracy of billing and coding, as well as the sufficiency of the medical records and supporting documentation submitted. Failure to submit medical records and supporting documentation to Anthem in accordance with this requirement will result in a denial of the Claim under review. During the pendency of the prepayment review, if requested, the Provider or Facility will be given the opportunity to discuss their prepayment review status.

Under the prepayment review program, Anthem may review coding, documentation, and other billing issues. In addition, and in applicable markets, Anthem may use one or more clinical utilization management guidelines in the review of Claims submitted by the Provider or Facility, even if those guidelines are not used for all Providers or Facilities delivering services to Plan Members.

The Provider or Facility will remain subject to the prepayment review process until Anthem is satisfied that all inappropriate billing, coding, or documentation activity has been corrected. If the inappropriate activity is not corrected, the Provider or Facility could face corrective measures, up to and including termination from our network.

Providers and Facilities are prohibited from billing a Member for services Anthem has determined are not payable as a result of the prepayment review process, whether due to FWA, any other coding or billing issue, or for failure to submit medical records as set forth above. Providers or Facilities whose Claims are determined to be not payable may make appropriate corrections and resubmit such Claims in accordance with the terms of their Provider and Facility Agreement, proper billing procedures, and state law. Providers or Facilities also may appeal such a determination in accordance with applicable grievance and appeal procedures.

Acting on Investigative Findings

Anthem shall be entitled to offset claims and recoup an amount equal to any overpayments or improper payments made by Anthem to Provider or Facility (“Overpayment Amount”) against any payments due and payable by Anthem or any Affiliate to Provider or Facility with respect to any Health Benefit Plan under the Agreement or under any Agreement between Provider and an Affiliate, regardless of the cause.

Provider or Facility shall voluntarily refund the Overpayment Amount regardless of the cause, including, but not limited to, payments for Claims where the Claim was miscoded, non-compliant with industry standards, or otherwise billed in error, whether or not the billing error was fraudulent, abusive, or wasteful.

Upon determination by Anthem that an Overpayment Amount is due from Provider or Facility, Provider or Facility must refund the Overpayment Amount to Anthem within thirty (30) calendar days of the date of the overpayment refund notice from Anthem to the Provider or Facility. If the Overpayment Amount is not received by Anthem within the thirty (30) calendar days following the date of such notice letter, Anthem shall be entitled to offset the unpaid portion of the Overpayment Amount against other Claims payments due and payable by Anthem or an Affiliate to Provider or Facility under any Health Benefit Plan in accordance with Regulatory Requirements. In such an event, Provider or Facility agrees that all future Claim payments, including Affiliate Claim payments, applied to satisfy Provider’s or Facility’s repayment obligation shall be deemed to have been legally paid to Provider or Facility in full for all purposes, including Affiliates and/or Regulatory Requirements as defined by the Provider or Facility Agreement.

Should Provider or Facility disagree with any determination by Anthem or a Plan that Provider or Facility has received an overpayment or improper payment, Provider or Facility shall have the right to appeal such determination under Anthem’s procedures set forth in the Provider Manual, provided that such appeal shall not suspend Anthem’s right to recoup the Overpayment Amount during the appeal process unless required by Regulatory Requirements. Anthem reserves the right to employ a third-party collection agency in the event of non-payment.

In addition to the previously mentioned actions, Anthem may report suspected criminal activity committed by a Member, Provider, or Facility to the appropriate regulatory and/or law enforcement agencies.

Pharmacy & Prescriber Home Program

Program Scope

The Pharmacy & Prescriber Home Program, referred to operationally as the CarelonRx Provider Home Program, applies to Anthem Members with Anthem prescription drug coverage. The program governs the administration of Schedule II–V controlled substance benefits when a Member demonstrates utilization patterns that present elevated safety risk.

The availability of opioid and other controlled medications has improved access to treatment for acute and chronic conditions. However, fragmented prescribing and dispensing across multiple Providers or Facilities may create significant safety concerns, including harmful drug interactions, polypharmacy, prescription cascading, duplication of therapy, and potential misuse. This program establishes structured oversight and coordinated management of controlled substance utilization.

Program Objective

The objective of the Pharmacy & Prescriber Home Program is to:

- Improve patient safety and care coordination
- Reduce non-medically necessary overutilization of controlled substances
- Mitigate risks associated with multiple prescribers and pharmacies
- Ensure compliance with regulatory expectations and clinical best practices

The program accomplishes these objectives through targeted identification, structured enrollment, centralized prescribing and dispensing, and continuous monitoring.

Program Design and Core Features

Members identified as meeting established enrollment criteria may be placed in the program following required notification and monitoring processes.

Once enrolled, the Member is restricted to one designated participating pharmacy and/or one designated prescriber for all Schedule II–V controlled medications for a minimum of twelve (12) consecutive months.

Key program features include:

- **Designated Pharmacy and/or Prescriber Home**
  The Member selects a participating pharmacy and/or prescriber within the designated timeframe. If no selection is made, one is assigned based on retrospective Drug Utilization Review (DUR) of Claims history, prioritizing recency and frequency of use.
- **Claim Controls**
  Controlled substance Claims will be reimbursed only when:
    - Written by the designated Prescriber Home, and/or
    - Dispensed by the designated Pharmacy Home

Claims written or filled outside the assigned Pharmacy or Prescriber Home will deny, except as outlined below.

- **Urgent or Emergent Overrides**[1]
  Temporary overrides are permitted for urgent or emergent circumstances only, subject to defined policy controls.
- **Mail Order and Specialty Access**
  Members retain access to mail order and specialty pharmacies, in addition to their designated Pharmacy Home, as permitted under program parameters.
- **Provider Notification and Prescription Profile**
  Upon enrollment, all prescribers associated with the Member receive written notification that includes:
    - Enrollment status
    - Assigned Pharmacy and/or Prescriber Home
    - A three (3) month controlled substance prescription profile detailing prescribers, medications, dosages, and quantities

Enrollment Criteria[2]

A Member may be enrolled if retrospective Claims analysis demonstrates one or more of the following within a ninety (90)-day period:

- Five (5) or more controlled substance prescriptions
- Controlled substance prescriptions from three (3) or more prescribers
- Controlled substance prescriptions filled at three (3) or more pharmacies

Clinical review and program thresholds are applied in accordance with established policy.

Member Communications and Due Process

Members who meet criteria receive written notice at least sixty (60) days prior to potential enrollment. If criteria continue to be met during the monitoring period, the Member receives formal notification of enrollment.

Following enrollment notification:

- The Member is provided thirty (30) days to select a Pharmacy and/or Prescriber Home and/or file an appeal.
- If no selection is made, a Pharmacy and/or Prescriber Home is assigned on day thirty-one (31) based on Claims utilization history.
- Written confirmation is issued to both the Member and the assigned Provider.

Requests to change the assigned Pharmacy or Prescriber Home are evaluated for good cause only.

Program Governance

The Pharmacy & Prescriber Home Program operates under structured clinical, operational, and regulatory oversight. Enrollment determinations are based on objective Claims data and defined thresholds. Members are not permitted to self-exclude once enrollment criteria are met; however, appeal rights are preserved in accordance with policy and regulatory requirements.

CarelonRx and Anthem remain committed to equipping Providers with clear communication, actionable data, and coordinated processes to reduce inappropriate controlled substance utilization while preserving medically necessary access to care.

For questions regarding enrollment, Providers may direct Members to contact the Member Services number located on the back of the Member ID card.

[1] Changes to the designated pharmacy and/or prescriber will only be approved if the request meets good cause criteria

[2] Members with a diagnosis of cancer, second degree burns, third degree burns, sickle-cell anemia or those that are in hospice care may be exempt from enrollment in the program. **Note:** Exemptions are determined by both the member's pharmacy and medical claims history.

Federal Employees Health Benefits Program and Postal Service Health Benefits Program

FEHBP and PSHBP Requirements

Providers and Facilities acknowledge and understand that Anthem participates in the Federal Employees Health Benefits Program (FEHBP) and the Postal Health Benefits Program (PSHBP). The Anthem FEHBP and PSHBP encompasses the Blue Cross Blue Shield Association Service Benefit Plan, otherwise known as "Federal Employee Program®" or "FEP®", the health insurance Plan for federal employees.

Providers and Facilities further understand and acknowledge that the FEHBP and PSHBP are federal government programs and the requirements of the program are subject to change at the sole direction and discretion of the United States Office of Personnel Management.

Providers and Facilities agree to abide by the rules, regulations, and/or other requirements of the FEHBP and PSHBP as they exist and as they may be amended or changed from time to time, with or without prior notice. Providers and Facilities further agree that, in the event of a conflict between the Provider or Facility agreement or this Provider Manual and the rules, regulations, and/or other requirements of the FEHBP and PSHBP, the terms of the rules, regulations, and other requirements of the FEHBP and PSHBP shall control.

When a conflict arises between federal and state laws and regulations, the federal laws and regulations supersede and preempt the state or local law (Public Law 105-266). In those instances, FEHBP and PSHBP are exempt from implementing the requirements of state legislation.

In certain circumstances, the plan reserves the right to adjust or modify the format of claims received electronically in order to ensure accurate processing and payment.

Submission of Claims under the FEHBP and PSHBP

All Claims under the FEHBP and PSHBP must be submitted to the Plan for payment within the timeframe listed in the Provider or Facility Agreement. This timeframe applies from the date of discharge or from the date of the primary payer's explanation of benefits.

Providers and Facilities agree to provide to Plan, at no cost to Anthem or Member, all information necessary for Plan to determine its liability, including, without limitation, accurate and complete Claims for Covered Services, utilizing forms consistent with industry standards and approved by Plan or, if available, electronically through a medium approved by Plan.

If Plan is the secondary payer, the timeframe will not begin to run until Provider or Facility receives notification of the primary payer's responsibility. Plan is not obligated to pay Claims received after the timeframe indicated in the Agreement.

Except where the Member did not provide Plan identification, Provider and Facility shall not bill, collect, or attempt to collect from Member for Claims Plan receives after the applicable period, regardless of whether Plan pays such Claims.

Omnibus Budget Reconciliation Act of 1993 (OBRA '93)

As a result of the Omnibus Budget Reconciliation Act of 1993 (OBRA '93) legislation, all FEHBP and PSHBP fee-for-service carriers are required to price certain Claims per the Medicare Part B equivalent amount. This legislative change became effective on January 1, 1995. OBRA '93 applies

the Medicare Part B equivalent amount to Claims for physicians' services to retirees and annuitants enrolled in the FEHBP and PSHBP who are 65 years of age and older and who do not participate in Medicare Part B. The Office of Personnel Management (OPM) has defined the individuals to whom the law applies as those who are enrolled in an FEHBP and PSHBP Program and are annuitants or former spouses. In addition, the law also applies to family Members covered by a family enrollment of an annuitant or former spouse.

The covered Member must:

- Not be employed in a position that confers FEHBP and PSHBP coverage
- Be age 65 or older
- Not be covered by Medicare Part B

Erroneous or Duplicate Claim Payments under the FEHBP and PSHBP

For erroneous or duplicate Claim payments under the FEHBP and PSHBP, either party shall refund or adjust, as applicable, all such duplicate or erroneous Claim payments regardless of the cause. Such refund or adjustment may be made within five (5) years from the end of the calendar year in which the erroneous or duplicate Claim was submitted. In lieu of a refund, Plan may offset future Claim payments.

Coordination of Benefits for FEHBP and PSHBP

In certain circumstances, when the FEHBP and PSHBP are the secondary payer, and there is no adverse effect on the Member, the FEHBP and PSHBP pay the local Plan allowable minus the Primary payment. The combined payments from both the primary payer and the FEHBP and PSHBP as the secondary payer might not equal the entire amount billed by the Provider or Facility for covered services.

FEHBP and PSHBP Waiver requirements

- Notice must identify the proposed services.
- Inform the Member that services may be deemed not medically necessary or experimental/investigational by the Plan
- Provide an estimate of the cost for services
- Member must agree in writing to be financially responsible in advance of receiving the services; otherwise, the Provider or Facility will be responsible for the cost of services denied

FEHBP and PSHBP Member Reconsiderations and Appeals

There are specific procedures for reviewing disputed Claims under the FEHBP and PSHBP. The process has two steps, starting with a review by the local Plan (reconsideration), which may lead to a review by the Office of Personnel Management (OPM).

The review procedures are designed to provide Members with a way to resolve Claim disputes as an alternative to legal actions.

The review procedures are intended to serve both contract holders and Members. The local Plan and OPM do not accept requests for review from Providers or Facilities, except on behalf of, and with the written consent of, the contract holder or Member.

Providers and Facilities are required to demonstrate that the contract holder or Member has assigned all rights to the Provider or Facility for that particular Claim or Claims.

When a Claim or request for Health Services, drugs, or supplies – including a request for precertification or prior approval – is denied, whether in full or partially, the local Plan that denied the Claim reviews the benefit determination upon receiving a written request for review. This request must come from the Member, contract holder, or their authorized representative. The request for review must be received within six (6) months of the date of the Plan's final decision. If the request for review is on a specific Claim(s), the Member must be financially liable in order to be eligible for the disputed Claims process.

The local Plan must respond to the request in writing, affirming the benefits denial, paying the Claim, or requesting the additional information necessary to make a benefit determination, within thirty (30) calendar days of receiving the request for review. If not previously requested, the local Plan is required to obtain all necessary medical information, such as operative reports, medical records, and nurses' notes, related to the Claim. If the additional information is not received within sixty (60) calendar days, the Plan will make its decision based on the information available. Appropriate medical review will also be done at this time. If the Plan does not completely satisfy the Member's request, the Plan will advise the Member of their right to appeal to OPM.

Providers or Facilities may not submit appeals to the OPM without authorization from the Member. Only the Member or contract holder may do so, as outlined in the Blue Cross and Blue Shield Service Benefit Plan brochure.

FEHBP and PSHBP Formal Provider and Facility Appeals

Providers and Facilities are entitled to pursue disputes of their **pre–service request** (this includes pre-certification or prior approval) or their **post-service Claim** (represents a request for reimbursement of benefits for medical services that have already been performed), by following a formal dispute resolution process.

A formal Provider or Facility appeal is a written request from the rendering Provider or Facility to their local Plan to have the local Plan re-evaluate its contractual benefit determination of their post-service Claim or to reconsider an adverse benefit determination of a pre-service request. The request must be from a Provider or Facility and must be submitted in writing within one hundred eighty (180) days of the denial or benefit limitation. In most cases, this will be the date appearing on the Explanation of Benefits/Remittance sent by the Plan. For pre-service request denials, the date will be the date appearing on the Plan's notification letter.

The request for review may involve the Provider or Facility's disagreement with the local Plan's decision about any of the ***clinical issues*** listed below, where the Providers or Facilities are *not* held harmless. Local Plans should note that this list is not all-inclusive.

1. Not medically necessary (NMN)
2. Experimental/investigational (E/I)
3. Denial of benefits, in total or in part, based on clinical rationale (NMN or E/I)
4. Precertification of hospital admissions
5. Prior approval (for a service requiring prior approval under FEHBP and PSHBP).

Not all benefit decisions made by local Plans are subject to the formal Provider and Facility appeal process. The formal Provider and Facility appeal process does not apply to any non-clinical case.

When a Claim or request for services, drugs, or supplies – including a request for precertification or prior approval – is denied, whether in full or partially, the local Plan that denied the Claim reviews the benefit determination upon receiving a written request for review. This request must come from the

rendering/requesting Provider or Facility. The request for review must be received within six (6) months of the date of the local Plan's final decision. If the request for review is on a specific Claim(s), the Provider or Facility must be financially liable in order to be eligible for the formal Provider and Facility appeal process.

The local Plans must respond to the request in writing, affirming the benefits denial, paying the Claim, or requesting the additional information necessary to make a benefit determination, within thirty (30) calendar days of receiving the request for review. If not previously requested, the local Plan is required to obtain all necessary medical information, such as operative reports, medical records, and nurses' notes, related to the Claim. If the additional information is not received within sixty (60) calendar days, the local Plan will make its decision based on the information available. Appropriate medical review will also be done at this time. Even if the local Plan does not completely satisfy the Provider or Facility's request, the formal Provider and Facility appeal process is complete; no additional appeal rights are available.

FEHBP and PSHBP Inpatient Skilled Nursing Facility Care

Please see the Blue Cross® and Blue Shield® Service Benefit Plan brochure at **fepblue.org** for the skilled nursing benefit.

Online information for FEHBP and PSHBP

Refer to the benefits and services on the FEHBP and PSHBP website **fepblue.org** for additional information.

Health Insurance Marketplace® (Exchanges)

The Affordable Care Act (ACA) authorized the creation of Health Insurance Marketplaces (commonly referred to as Exchanges) to help individuals and small employers shop for, select, and enroll in high-quality, affordable private health plans.

Anthem offers qualified health plans on the individual or Small Business health Options Program (SHOP) Exchange in many states, as well as health plans not purchased on public exchanges. Qualified health plans on the Individual and SHOP Exchange follow the same policies and protocols within this Provider Manual, unless otherwise stated in the Provider or Facility Agreement.

Updates about Anthem's ACA-compliant health plans and the networks supporting these plans are published in Anthem's provider newsletter and sent via Anthem's email service.

To access the newsletter, go to the ***Provider News* website**. The option to sign up for all email communications is also on this webpage.

Important reminder

Providers and Facilities can confirm their participation status across different networks using the **Find Care** tool. See the *Online Provider Directory and Demographic Data Integrity* section for more details.

BlueCard Program Overview

BlueCard is a national program that enables Members of one Blue Plan to obtain healthcare service benefits while traveling or living in another Blue Plan's service area. The program links participating healthcare Providers and Facilities with the independent Blue Plans across the country and in more than 200 countries and territories worldwide through a single electronic network for Claims processing and reimbursement.

For more information about the BlueCard Program, Providers and Facilities can access the BlueCard Provider Manual online. Go to the **Anthem Provider Home page**, then under the **Resources** menu, select **Policies, Guidelines and Manuals**, scroll down to the **Provider Manual** section and select **Download the Manual**. Scroll to the **Provider Manual Library** section and choose the **BlueCard Provider Manual**.

Medicare Advantage

Refer to the Medicare Advantage website online for additional information at **anthem.com/ky/provider/individual-commercial/medicare-advantage.**

The Medicare Advantage Provider Manual is available on the Anthem website. Go to the **Anthem Provider Home page**, then under the **Resources** menu, select **Policies, Guidelines and Manuals**, scroll down to the **Provider Manual** section, and select **Download the Manual**. Scroll to the **Provider Manual Library** section and choose **Medicare Advantage Provider Manual**.

Medicare Advantage Provider Guidebook

Workers' Compensation

This section of the Kentucky Provider Manual applies to those Providers who participate in the Anthem Workers' Compensation Network.

Provider Guidelines

The Provider should ask the injured worker who is seeking medical treatment the nature of the illness or injury to confirm that the illness or injury is work-related. Provider should further determine whether the injured worker's illness or injury is:

- **Non-emergency:** Instruct the injured worker to obtain authorization from the employer before treatment is rendered.
- **Emergency:** If an injured worker requires emergency treatment, care must be provided to the injured person. Determining workers' compensation coverage should be made within the next seventy-two (72) hours.

When an injured worker seeks non-emergency services for a work-related illness or injury, the Injured Worker will be directed to a participating Provider. In the event the participating Provider is unable to treat the Injured Worker, the Provider shall make every effort to refer or transfer the injured worker to a healthcare professional that participates in the Anthem Workers' Compensation Network. If the Provider elects to treat the injured worker, the Provider must complete a Medical Report, as defined in the state regulations. As payment for such medical services rendered, the Provider agrees to accept, as payment in full, compensation in accordance with the reimbursement set forth in the Workers' Compensation Plan Compensation Schedule Participation Attachment or Agreement.

All Anthem Workers' Compensation Network-related correspondence should be sent to:

- **Shared Mailbox Address: WCContractingServices@anthem.com**
- **Mailing Address:**
  Anthem Workers' Compensation
  Attn: Network Management
  P.O. Box 4210
  Woodland Hills, CA 91365-4210

For questions regarding the Anthem Workers' Compensation Network Participation Attachment or Agreement, or other workers' compensation issues, email WCContractingServices@anthem.com.

For questions regarding the Anthem Workers' Compensation reimbursement rate, email WCContractingServices@anthem.com.

Utilization Management Guidelines

Utilization Management Guidelines are defined by the other payer, state regulation, or processes internal to the Anthem Workers' Compensation Network (if applicable).

The Workers' Compensation Act of Kentucky Standards

The Workers' Compensation Act of Kentucky has established standards for injured workers to access care and guidelines to improve the quality of medical care for occupational injuries. Participating Providers must adhere to the guidelines below.

- Refer or transfer within the Anthem Workers' Compensation Network. To find a participating Anthem Workers' Compensation Network Provider, visit the website at **https://www.enlyte.com/workers-compensation-provider-finder**.
- Services obtained outside of the network may not be paid. Contact the Claims adjuster for authorization for any medical care outside the network.
- Maintain medical records, records of accounts and payments, reports, and other documents that were necessary for the delivery of the medical services as required by state regulations. All such records, including records pertaining to the cost to the healthcare practitioner for delivery of the medical care and the payments received by the healthcare practitioner for such care, will be made available to the network, on request, during regular business hours.
- Submit bills to the appropriate workers' compensation or other payer within the number of days required by state regulation following the delivery of healthcare services. The Explanation of Review will indicate that rates are in accordance with the Anthem Workers Compensation Participation Attachment or Agreement.
- The workers' compensation other payer will ensure payment for authorized medical services rendered while a Claim is under investigation, until such time as a denial of the Claim is made by the other payer.

Other Payers Accessing the Anthem Workers' Compensation Network

A list of participating Other Payers may be accessed by accessing the Availity web portal or by requesting a list by emailing WCContractingServices@anthem.com.

To register for the Availity web portal, go to **Availity.com** and click on **Register**.

Once you are registered, the "Anthem Workers' Compensation Network Leasing List" can be accessed by selecting the state and "Payer Spaces", then "Anthem". From the Anthem Payer Space Page, select tab "Resources", then, "Anthem Workers' Compensation Network Leasing List".

Rules for Calculating Permanent Disability

The calculation of permanent disability is to be in accordance with State Regulations.

If Providers or Facilities are unable to calculate permanent disability in accordance with State Regulation, contact the Claims examiner to refer the patient to another participating Anthem Workers' Compensation Network provider who is able to calculate disability in accordance with State Regulations.

Grievances

Anthem participating Providers with a complaint or grievance may email WCContractingServices@anthem.com**.**

Additional Information:

For additional information regarding the obligations of the treating physician for Workers' Compensation, visit the Kentucky Department of Workers' Claims website at **https://elc.ky.gov/workers-compensation.**

Important Links

BlueCard Provider Manual

https://www.anthem.com/content/dam/digital/docs/provider/commercial/manuals/PM_KY_00006.pdf

Centers of Medical Excellence/Blue Distinction

https://www.anthem.com/member-resources/getting-care

Federal Employee Program

http://www.fepblue.org/

Medicare Advantage Provider Information

https://www.anthem.com/ky/provider/individual-commercial/medicare-advantage

Provider Communications Email Sign-up Form

https://providernews.anthem.com/kentucky/signup?utm_source=providernews&utm_medium=button&utm_campaign=subscribe

Provider Maintenance

https://www.anthem.com/ky/provider/individual-commercial/maintenance-demographic-updates